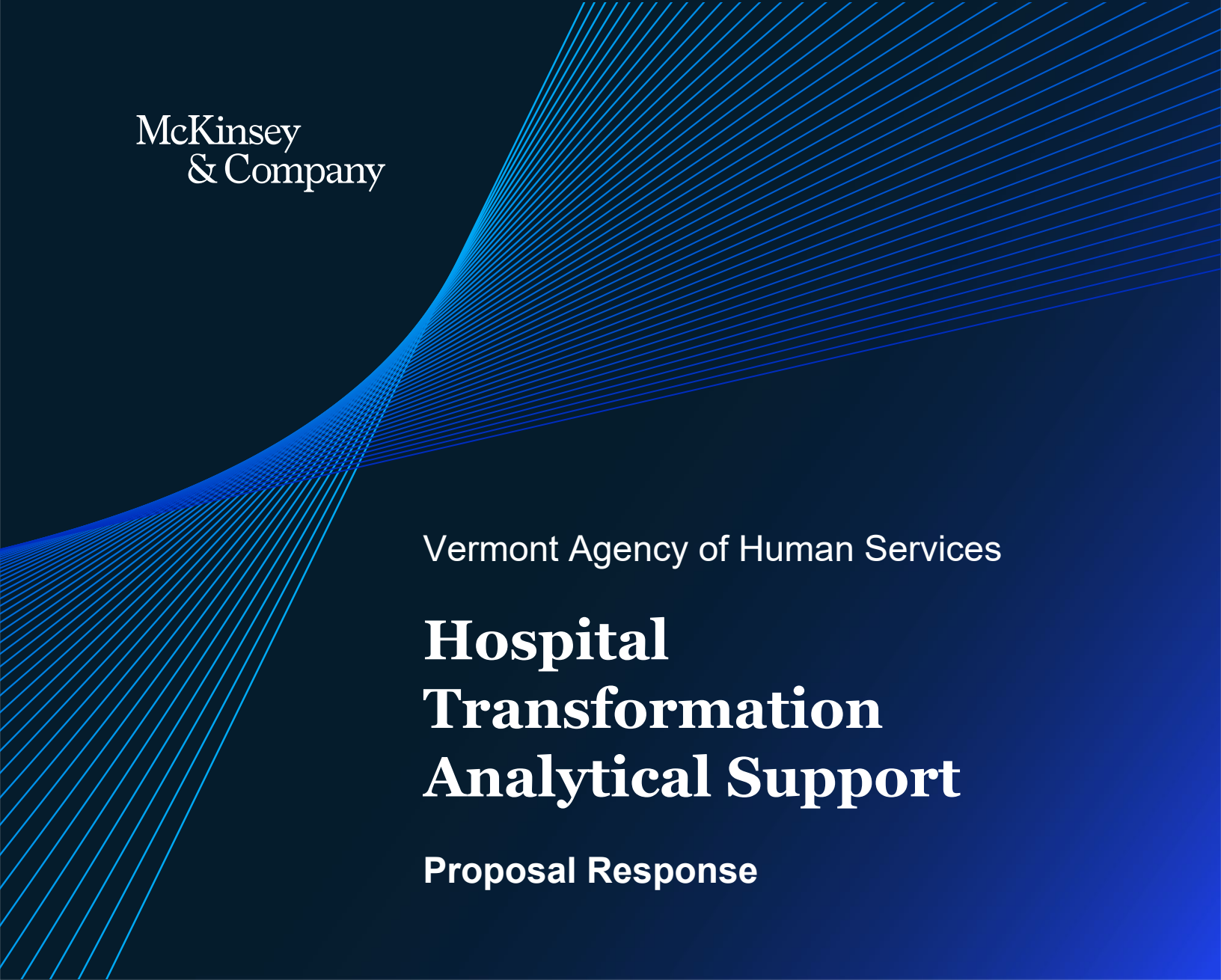


The background of the top half of the page is a dark blue gradient. On the left side, there are several thin, light blue lines that curve upwards and to the right, creating a sense of motion and depth. The McKinsey & Company logo is positioned in the upper left corner.

McKinsey
& Company

Vermont Agency of Human Services

Hospital Transformation Analytical Support

Proposal Response

October 28, 2025

Submitted to:

Vermont Agency of Human Services

Office of Health Care Reform

Matthew Rockwell, Administrative Services Director

Submitted via email to matthew.rockwell@vermont.gov

1.0 Cover Letter (4.6)

October 28, 2025

State of Vermont
Agency of Human Services
Office of Health Care Reform
Matthew Rockwell, Administrative Services Director
Submitted via email to matthew.rockwell@vermont.gov

Subject: Vermont Agency of Human Services Hospital Transformation Analytical Support

Dear Mr. Rockwell:

Please find attached McKinsey & Company, Inc. Washington D.C.'s (McKinsey)'s response to Vermont's Hospital Transformation Analytical Support Request for Proposal (RFP).

If you have any questions about our proposal response, please do not hesitate to contact me at (312) 753-8686 or david_nuzum@mckinsey.com. On behalf of our accountable partner leadership team, we look forward to the potential to serve the State of Vermont on this important endeavor to provide analytic support for regional health care transformation planning.

For contractual questions, please contact Maha Shah, Contracts Specialist, at (771) 241-2062 or Maha_Shah@mckinsey.com and mckinsey_contracts@mckinsey.com.

Sincerely,



David Nuzum,
Senior Partner, signatory on behalf of the McKinsey US Government Practice

Accountable Leadership Team Partners for the State of Vermont:



Sarun Charumilind,
Partner



Thomas Pellathy,
Senior Partner



Michael Elliott, Senior
Partner



Nikhil Sahni,
Partner

Table of Contents

1.0	Cover Letter (4.6)	1
2.0	Technical Response (4.7)	1
2.1	Our Understanding	1
2.2	Why McKinsey	1
2.3	Qualifications and Experience	4
2.3.1	Data capabilities	4
2.3.2	Capabilities of the Healthcare Capacity Planning and Optimization engine	9
2.3.3	People	13
2.3.4	Similar experience in the last 5 years	20
2.4	Technical Proposal of Workplan	37
2.4.1	Deliverable #1: Data Integration & Analysis Plan	38
2.4.2	Deliverable #2: Hospital & Regional Access Modeling	39
2.4.3	Deliverable #3: Market & Financial Analysis	41
2.4.4	Deliverable #4: Evidence-Based Benchmarking Evaluation	42
2.4.5	Deliverable #5: Transformation Scenario Model	43
2.4.6	Deliverable #6: Final Regional and Hospital Transformation Analytics Report	46
2.5	Capacity to Conduct Engagement	47
2.5.1	Our approach to staffing this engagement	47
2.5.2	Timeline to accomplish the scope of work	50
3.0	References (4.8)	51
4.0	Reporting Requirements (4.9)	51
5.0	Price Schedule (4.10)	52
5.1	Assumptions and Disclosures	53
6.0	Certificate of Compliance (4.11)	56
Appendix A:	Resumes	A-1
Appendix B:	Certificate of Compliance Form	B-1

Table of Exhibits

Exhibit 1:	Capability #1, high-performing provider footprint design	10
Exhibit 2:	Capability #2, future healthcare utilization projection	11
Exhibit 3:	Capability #3, healthcare site and capacity planning	12
Exhibit 4:	Our proposed team's proven experience and expertise	14
Exhibit 5:	Relevant client engagement examples in the last 5 years	21
Exhibit 6:	McKinsey's organizational structure	47
Exhibit 7:	Proposed timeline to accomplish the scope of work	50

2.0 Technical Response (4.7)

2.1 Our Understanding

The State of Vermont is undertaking a comprehensive effort to transform its hospital and regional health system to ensure long-term financial sustainability, protect access to essential services, and improve health outcomes for the citizens of Vermont. Rising healthcare costs, workforce constraints, and uneven access to care, particularly in rural areas, are creating structural pressures that cannot be addressed through incremental change. In response, the Agency of Human Services is seeking analytical support to inform data-driven decision making and guide transformational pathways that rebalance cost, quality, access, and sustainability across individual hospitals, regions, and the statewide system to best serve the needs of Vermonters.

To support this effort, the State of Vermont (“State”) requires a partner with a deep understanding of provider financials and operations, network and footprint design, and multi-sector healthcare datasets coupled with advanced analytic modeling capabilities. Our proposed approach would integrate State and hospital data, assess current delivery patterns, and model future scenarios. The focus is on evaluating how specific service line changes, regionalization strategies, and innovative care models would affect overall system performance across multiple dimensions including cost to payers and patients, workforce viability, utilization patterns, travel times, health outcomes, and compliance with state and federal standards. The resulting insights will inform policy and regulatory decisions, enable transparent stakeholder engagement, and help Vermont identify the most viable and equitable pathways for system transformation over the coming years.

McKinsey proposes addressing these questions using our proven integrated *Healthcare Capacity Planning and Optimization* engine, which combines advanced AI modeling and network design capabilities into a single analytic framework. This platform models demand, capacity, workforce, capital requirements, and operating cost trajectories, and then supports optimization of provider footprints and access by generating and evaluating network scenarios against cost, quality, and access standards. Together, these capabilities will enable a consistent, transparent, end-to-end analytic flow for Vermont, allowing State leaders to facilitate transformation of the care delivery network to best support the needs of Vermonters.

2.2 Why McKinsey

McKinsey’s healthcare practice is one of the largest in the world, with more than 2,000 healthcare consultants and over 300 physicians and other clinicians. We have served hospital systems and healthcare agencies across the country for more than 70 years, with experience supporting nearly 40 US states, bringing expertise in provider financial sustainability, health system transformation, and population health improvement. This work is supported by the McKinsey Health Institute, which is dedicated to improving public health and longevity, including topics such as rural health.

Our public sector practice is purpose-built to help governments address their most complex health challenges. We have worked with more than 350 public sector clients, including nearly 40 state governments as well as CMS, HRSA, and other federal agencies from all 15 federal executive cabinet departments, delivering over 1,200 US government engagements in the past five years alone. Our team includes more than 2,900 public sector professionals and 4,000 technology experts who bring capabilities across data analytics, hospital operations, Medicaid financing, workforce modeling, and rural care delivery innovation.

Where healthcare and government intersect, this combination of scale, clinical expertise, public sector expertise, and thought leadership allows us to bring Vermont the full strength of a national perspective while tailoring our approach to Vermont's vision of a sustainable and equitable hospital system that improves affordability while preserving access to care. Our Healthcare Capacity Planning and Optimization engine, a proprietary AI-powered analytic solution, was designed for precisely this type of demand and capacity modeling, and has been deployed regionally, nationally, and internationally across numerous clients, taking into account factors such as provider capacity, demographic shifts, and care delivery innovation. We have extensive experience translating these insights into action: we know how to convene stakeholders, translate analytical models into strategic choices, support regulatory decision-making, and strengthen our clients' capacity and capabilities.

This is what the Agency of Human Services and the Green Mountain Care Board would get from partnering with McKinsey on this effort:

1. McKinsey has delivered similar work successfully across the US and globally, in public and private sectors, and will tailor our approach to Vermont's specific context

McKinsey has helped leading public and private health systems across the globe design and implement care delivery networks that improve access, quality, efficiency, and affordability, enhance financial performance, and meet the evolving needs of their populations. Crucially, our work focuses not just on the end-state design, but what it will take to get there – the transition and transformation needed to realize the end-state vision and the value that it will create for patients and communities. Our experience ranges from national health systems (like in the United Kingdom and Australia) to large integrated delivery networks and rural health providers in the US and gives us insights into what works across diverse market structures, regulatory environments, and demographic realities. These experiences, supported by proprietary data assets and advanced analytics, allow us to tailor these proven approaches to each client's unique context rather than applying a one-size-fits-all model. For Vermont, this means we will bring a national and global perspective on data-driven care delivery transformation, while designing solutions that reflect the State's rural geography, community-based care traditions, and commitment to cost savings, risk reduction, equitable access, and transparent decision-making that promotes sustainable hospital and regional transformation.

2. McKinsey has extensive experience serving HHS agencies in nearly 40 states; we know what ideas, designs, and plans work

Our experience across federal and state government HHS agencies gives us a detailed understanding of what drives successful implementation in a state environment, from navigating legislative cycles and federal requirements to engaging local stakeholders and operationalizing reforms across diverse delivery systems. We have a proven track record of translating our recommendations into impact for the public sector; our references stand by ready to speak to their experiences in partnering with us. Our agile delivery model, grounded in AI, enables us to update assumptions, re-run scenarios, and adapt recommendations in real time based on evolving policy perspectives or new data-driven insights and predictive analytics. Through rapid-cycle analytics, we help states test the implications of policy options, financial models, workforce dynamics, and access requirements before decisions are made, allowing the State of Vermont to proactively assess tradeoffs, reduce operational and financial risks, and advance cost-effective, evidence-based reforms for the State's health system footprint that improve access and sustainability, and build stakeholder confidence. Because we have seen firsthand what accelerates progress and what creates barriers in peer states, we will be able to guide the State towards solutions that are both ambitious and practical within the operational realities of state

government, promoting transparency, accountability, and alignment with the State's hospital and regional transformation goals.

3. We bring deep, hands-on expertise across hospital strategy, finance, and operations, and will help Vermont and its hospitals achieve sustainable performance improvement through our provider-centric change management approach

While other consulting firms support healthcare policy with little or no direct experience in the care delivery systems, McKinsey works directly with hospitals and other providers across strategy, finance, operations, and clinical improvement efforts. We have partnered with more than 250 health systems in the United States, as well as 42 of the nation's top 50 academic medical centers, and over 60% of U.S. News & World Report Honor Roll institutions. This broad exposure gives us a clear view into how leading health systems balance access, quality, affordability, and long-term financial sustainability. Our recent engagements include more than 600 end-to-end transformation initiatives, ranging from medical group and network redesign to clinical operations improvement and care model innovation, providing direct insight into what drives productivity, physician engagement, cost savings, and patient trust.

Our experience spans the full spectrum of the healthcare ecosystem, enabling us to approach challenges through the perspectives of healthcare policymakers, health system CEOs, and hospital COOs alike. This breadth of insight is directly relevant to Vermont's mix of independent, critical access, and regional referral hospitals. Having partnered extensively with clients facing aging populations, workforce constraints, and volume pressures, we design solutions grounded in hospital financial and operational realities. Our provider-centric change management approach, developed with boards, executives, and clinical leaders, ensures that proposed changes are practical, compliant, locally credible, and financially sound, strengthening health systems for the long term.

Importantly, we have also partnered with States and private payers to design data-driven, value-based funding models, global budgets, and sustainable incentive structures that enable transformation not only within individual hospitals but across entire delivery systems. This triple vantage point – combining policymaker, payer, and provider experience – gives us a “top-to-bottom” understanding of how to align financial incentives, regulatory levers, and operational models to achieve system-wide efficiency and sustainability.

4. McKinsey brings flexible data and analytics tools that will be paired with existing Vermont datasets and dynamically adapted to Vermont's specific needs

Other consulting firms try to market analytic software tools that have been designed for a single purpose with limited flexibility to integrate new datasets or adapt analytic outputs. Their goal is often to achieve margins by maximizing scale and minimizing customization. In contrast, McKinsey's focus is to leverage robust and proven analytic approaches but then to develop flexible data and analytic capabilities that grow and evolve with each client deployment, across diverse contexts and applications.

For this specific effort, we will deploy our Healthcare Capacity Planning and Optimization engine. This is a modular, AI-based analytic system built across numerous efforts that integrate demand, capacity, workforce, cost, and access modeling into a single end-to-end workflow. This tool is well-tested and proven and has been effectively deployed across dozens of client situations to drive impact. Using this platform will allow us to support Vermont's statewide network design by simulating alternative configurations, evaluating cost and quality implications, and identifying underserved markets for targeted access expansion. It is flexible and dynamic – capable of incorporating new data, evolving assumptions, and updated policy or demographic inputs to maintain real-time decision readiness. With more than 150 transformation and

analytics experts to draw from, and an existing Vermont-specific data environment already established, we can immediately layer in national benchmarks, Medicare cost reports, hospital financials, social determinants data, and migration trends to begin producing insights within weeks, not months.

In addition, as needed, we can integrate our national datasets that cover more than 200 million lives across Medicare, Medicaid, and commercial payers, paired with a comprehensive provider directory, as well as cost and utilization benchmarks across all major service lines and payment models. Further, for the projections of Vermont's residents, our predictive analytics platforms model billions of medical and pharmacy claims annually, applying CMS-HCC, HHS-HCC, and CDPS methodologies to normalize for risk and ensure equitable comparisons across rural and urban populations. Machine-learning forecasting engines simulate demand, site-of-care and mode-of-care shifts, and the impact of alternative payment methodologies – including total cost of care, global budgets and other value-based models. Our workforce simulators (already used by peer states) forecast future physician, nursing, and behavioral health needs under multiple demographic and regulatory scenarios, supporting the design of sustainable regional staffing models. Through our unified Healthcare Capacity Planning and Optimization engine and our data assets, we will be able to develop comprehensive health system footprint scenarios in a short timeframe allowing for multiple rounds of iterations to ensure the State has all the information needed to guide policy, regulation, facility planning, and long-term transformation.

2.3 Qualifications and Experience

For the State of Vermont's needs, McKinsey will deploy its Healthcare Capacity Planning and Optimization engine, leveraging experience from working with nearly 40 State Medicaid and Health and Human services clients as well as hundreds of hospitals and private payers. This platform combines our robust data ecosystem, proprietary tools, and team of more than 150 analytics and transformation experts to support AHS and GMCB in managing transformation, responding to federal incentives, and maintaining regulatory readiness. A set of core capabilities, described below, will be the focal point for this effort. In addition, we will bring a number of data capabilities and tools to enhance this platform. Along with this, we have built a team of people that combine expertise in these tools, experience working with state and federal governments to build scenarios, and with knowledge of Vermont. Our goal in combining these data and tools, capabilities, and people is to provide transparent, decision-ready insights that will help Vermont achieve its Act 167 goals and long-term healthcare transformation objectives.

While other consulting firms often try to market analytic software tools that have been designed for a single purpose with limited flexibility to integrate new datasets or adapt analytic outputs, our focus with the Healthcare Capacity Planning and Optimization engine has been to develop flexible data and analytic capabilities that grow and evolve with each deployment. This will allow us to immediately start considering potential scenarios using McKinsey's pre-loaded data and then build rapidly from there with the integration of Vermont's data.

2.3.1 Data capabilities

While the *Healthcare Capacity Planning and Optimization* engine will be the core asset for this effort, it is supported by a number of data capabilities and tools:

- 1) First, McKinsey's integrated data assets span national claims, EHR, provider, financial, and demographic sources.
- 2) Second, our data asset creation and modeling methodology (built on a four-step framework of Build, Simulate, Co-design, and Implement) will integrate Vermont-specific datasets to model demand, capacity, workforce, cost, and access.

- 3) Third, our advanced analytics and AI enable predictive modeling and real-time scenario testing to forecast utilization, identify inefficiencies, and optimize care delivery across three tiers (community, regional, and statewide) while embedding access and quality benchmarks.
- 4) Fourth, McKinsey's workforce, site-of-care, and digital readiness models will be tailored for Vermont's regional context, accounting for cross-border patient flows, fixed-cost pressures, and rural hospital sustainability.
- 5) Fifth, our HIPAA-compliant and HITRUST-certified *Nebula* platform securely manages and enriches healthcare data at scale, ensuring analytic flexibility far beyond single-purpose tools offered by other firms.
- 6) Finally, McKinsey prioritizes rigorous data protection, applying ISO 27001-aligned governance, strong encryption, multi-factor authentication, and continuous oversight to ensure Vermont's data remains secure.

1) We have unique data to support the State of Vermont.

McKinsey's integrated data ecosystem combines comprehensive national coverage of claims and EHR data with provider, financial, and demographic sources. This enables deep insights into utilization, cost, quality, and access across all payer segments, which will enable us to have a fast start with the State of Vermont. We break down our data assets into four buckets: (a) Claims and EHR coverage; (b) Non-claims and provider landscape data; (c) Proprietary McKinsey databases; and (d) Additional Public and Government Sources

(a) Claims and EHR Coverage

McKinsey has access to an extensive set of claims and electronic health record (EHR) data sources covering over 200 million patients across all major US payer segments. This includes both closed and open claims as well as clinical EHR data, offering a comprehensive view of healthcare utilization and outcomes.

- **Medicare FFS & MA Data (VRDC):** Full national coverage of Medicare lives (100%), including provider and payer identifiers, and detailed financial information (allowed and paid amounts).
- **Medicaid FFS & MCO Data (VRDC):** Full national coverage of Medicaid lives (100%), similarly detailed and identifiable.
- **Closed Claims:** About 35% of all U.S. commercial lives, sourced from multiple vendors.
- **Open Claims:** Roughly 40% of all U.S. claims across Medicare, Medicaid, Commercial, and VA/Tricare segments.
- **EHR Data:** Around 40 million lives, including clinical details (labs, vitals, allergies) not typically captured in claims.

(b) Non-Claims and Provider Landscape Data

McKinsey complements claims and EHR data with rich non-claims datasets that profile providers, facilities, and local healthcare systems, specifically for Vermont and neighboring states.

- **American Hospital Association (AHA) Survey:** Over 1,300 hospital-level fields on services, staffing, and capacity (2018–2023).
- **Definitive Healthcare Databases – Facility Database:** Details on over 9,700 hospital, ambulatory, and post-acute facilities, including affiliations and capacities (2020–2025).
- **Definitive Healthcare Databases – Physician Group Database:** Data on 350,000+ physician offices, including specialties and affiliations.

- **Medicare Cost Reports:** 140+ metrics on hospital finances, volumes, and operations since 2006, linked to health systems.

(c) Proprietary McKinsey databases

McKinsey maintains several custom-built analytic databases to support forecasting, market analysis, and financial benchmarking which will be key for this effort:

- **Enrollment Projection Tool:** County-level coverage forecasts through 2032.
- **Triangulated Enrollment:** Current and historical insurance enrollment by payer and coverage type through 2024.
- **Payer Financial Database:** Financial performance (e.g., revenue, MLR) from regulatory filings.
- **Claims Predictive Volume:** Forecasts utilization trends by state, coverage type, and clinical service line through 2031.

(d) Additional Public and Government Sources

Finally, to enhance these other datasets, McKinsey integrates publicly available datasets, including:

- CMS Provider Data Catalog (quality metrics)
- HRSA Data Warehouse (workforce and shortage areas)
- U.S. Census data (demographics)
- Vermont-specific health and regulatory reports
- CMS/state network adequacy requirements

2) Our data asset creation methodology enables our modeling framework.

Our Healthcare Capacity Planning and Optimization engine takes a four-part modeling approach: Build, Simulate, Co-design, Implement. In our experience with other organizations, this allows for stress-testing of future scenarios, evaluating trade-offs, and optimizing configurations for adequacy, quality, cost, and sustainability under each scenario.

From the various data sources listed above, we would integrate them with any specific Vermont datasets, such as the All-Payer Claims Database and hospital clinical, financial, and administrative data, to create a unified data asset.

This unified data asset would then support demand, capacity, workforce, cost, and access modeling aligned with national best-practices and CMS guardrails, allowing Vermont to evaluate current performance and test future scenarios with accuracy and transparency. This modeling would integrate demographic and disease patterns to anticipate where demand will exceed or fall below capacity and to inform tiered regionalization strategies that maintain access while improving financial sustainability. In addition, this modeling would be built at the service line level using our hospital data discussed above which tracks operating margins, payer mix, and volume trends.

3) Our data asset and modeling framework are enabled by advanced analytics and AI to bring speed and scale.

All of these data assets and modeling frameworks are enabled by AI. Our AI-enabled predictive models forecast utilization and cost trends by incorporating demographic and health risk shifts, payer mix changes, and migration patterns. This would allow Vermont to anticipate rural hospital distress, identify unsustainable service lines, and test interventions such as regionalization, telehealth expansion, and specialty care consolidation.

Further, we model billions of medical and pharmacy claims across Medicaid, Medicare, and commercial plans each year, using machine learning to generate forecasts under multiple policy and demographic conditions. Our models isolate inefficiencies, simulate site-of-care shifts, and assess the effects of different payment models, including total cost of care, global budgets, and value-based reimbursement. Using CMS-HCC, HHS-HCC, and CDPS risk adjustment, we ensure fair comparisons across populations. The platform supports real-time scenario testing, for example, modeling the financial and access impacts of converting inpatient hospitals to outpatient centers.

4) We have workforce and care delivery models ready to customize to Vermont.

Along with our data assets, we would bring forward three additional models to this effort: (1) workforce demand, (2) site-of-care optimization, and (3) digital readiness.

McKinsey already forecasts long-term workforce demand for physicians, APPs, nurses, EMS, and behavioral health providers under different care delivery models. Our analysis includes productivity, retirements, telehealth, and AI-assisted care. Drawing on experience in other states, we design regional staffing models that stabilize rural systems through shared service lines and collaborative staffing, which we will bring forward for this effort.

In addition, we use our extensive claims database to identify opportunities for site-of-care optimization across the population. Taking into account factors such as regulatory guidance from the ASC CPL, patient health status, and procedure duration, we identify opportunities to increase patient access, lower total cost of care, and help ensure all patients receive the right care in the right place.

Finally, we would also assess Vermont's digital readiness, measuring factors such as telehealth use, remote monitoring, and reimbursement alignment to plan for increased adoption of innovative care delivery models. Our simulations demonstrate how digital-first models can reduce workforce shortages, improve access, and lower capital costs. We have supported states in launching virtual behavioral health and hospital-at-home programs that expand care access for rural and underserved communities.

5) Our “Nebula” environment is HIPAA-compliant and HITRUST-certified.

All our data and tools are enabled by and hosted on McKinsey's big data and analytics cloud platform “Nebula.” Nebula is a HIPAA-compliant and HITRUST-certified solution designed to securely manage healthcare data and deliver actionable insights. It allows us to securely and smoothly ingest and transform healthcare datasets, integrate them with our extensive claims and non-claims data (mentioned above), and deploy complex analytics tools like the Healthcare Capacity Planning and Optimization engine (described below). Here are the key capabilities in the Nebula platform:

- **Environment Setup:** A client-specific AWS S3 bucket is created to store raw data securely
- **Data transfer:** McKinsey offers a variety of safe data transfer options for data exchange with clients, including S3 Cross-Account Replication, McKinsey hosted SFTP, client hosted SFTP, Delta Sharing via Databricks, and Snowflake connect
- **Data Ingestion:** Data is securely loaded into a client-specific S3 location and processed through the Bronze Load tool integrated to our data platform, which converts it into Databricks Unity Catalog Delta tables
 - **Bronze Load Process:** Each data file is ingested after applying content-specific data encryption and transformation rules based on the file's structure and data types. This ensures compliance and prepares the data for further processing

- Supported File Formats: The platform can ingest a wide range of file types, including .csv, .txt, parquet, .zip/.gz, .JSON, and XML
- **Data Transformation:** Raw data undergoes de-duplication, data conversion, and PHI/PII encryption to ensure compliance and quality, data is minimally cleansed and transformed into a format suitable for analysis
 - McKinsey has built a standard and comprehensive data model that allows for conforming and joining key payor data on member eligibility, medical and pharmacy claims, and provider data. With our experience working with multiple healthcare clients, our data model is compatible with various types of claims datasets including APCD and enables automated and extensive quality assessment, data enrichment, and ultimately supporting analytics use cases
- **Data Profiling:** Comprehensive profiling is conducted to identify duplicates, assess field completeness, perform typecasting, and generate descriptive statistics, this ensures the data is structured and ready for advanced analytics
- **Data Storage and Hosting:** Transformed data is stored in a secure AWS environment and organized in a client-specific Operational Data Store (ODS), the ODS serves as the foundation for analysis and insights generation
- **Access and Permissions:** Strict access controls and role-based permissions are implemented to protect sensitive information and ensure compliance with regulatory standards
- **Data enrichments:** Experience performing other industry standard, proprietary, or client-specific transformations and enrichment on claims and other relevant datasets, including:
 - Designing and applying quality metrics to claims data, including for readmission rates, post-trigger acute event rates, complication rates, preventable admission rates, and 7- and 30-day follow-up rates
 - Index on an event (e.g., TAVR) and then map the patient journey prior to and after receiving a given procedure or diagnosis, which allows us to develop a perspective on the appropriate complement of acute, ancillary, and ambulatory assets needed to serve a given population
 - Encounters bundling that identifies claims for a single medical event (one setting, one patient, for one reason) and groups them into one record to enable analytics of multiple claims for the same event as a whole
 - Episodes that identify all relevant claims for a specific care journey across 200+ acute and chronic conditions
 - Chronic Condition Flags (CCWs) that applies member-level flags matching with CCW definitions
 - Hierarchical Condition Category (HCC) member risk adjustment model defined by CMS for Medicare Advantage members and ACA members

Our dedicated team of experts, solution practitioners, and data engineers offer end-to-end support from engagement scoping to data onboarding to deliver impactful insights quickly and reliably. Expertise includes working with healthcare administrative data, such as claims, encounters, eligibility, and provider directory information, sourced from a wide range of payers. These include commercial health plans, Medicare, and self-funded employer- or trust-sponsored plans that voluntarily participate.

6) We prioritize protecting the State of Vermont's data in this effort.

McKinsey recognizes the importance of protecting the data asset and simulations that would be part of this effort. We prioritize the protection of information, including personal and protected health data, by implementing robust technical, organizational, and physical security measures to

safeguard confidentiality, integrity, and availability. Our governance framework aligns with ISO/IEC 27001:2013 standards, industry best practices, and additional compliance requirements for sensitive data, such as HIPAA and HITRUST. Annual third-party audits, strategic oversight by an advisory committee, and leadership by the Chief Information Security Officer ensure continuous improvement. Employees undergo mandatory cybersecurity training, adhere to a data classification matrix with stringent handling controls, and follow an Acceptable Use of Technology Policy. Risk-based procedures monitor IT resources to prevent and mitigate security threats, with strict controls on email use, software, and encrypted devices. Access to data is governed by principles of "default to deny," "need to know," and "least privilege," supported by multi-factor authentication, password policies, and session timeout configurations. Infrastructure includes resilient Tier III data centers with ISO 27001 certifications, SOC2 Type 2 reports, and strong encryption protocols (e.g., AES256) for data storage and transmission. These measures collectively ensure that the State of Vermont's data will be handled securely, as we have done in other states.

2.3.2 Capabilities of the Healthcare Capacity Planning and Optimization engine

For this effort, we will bring our well-tested proprietary analytics engine, the Healthcare Capacity Planning and Optimization engine, which has been effectively deployed across 20+ public and private sector healthcare clients. Available from day one of the project using our own datasets, this platform integrates advanced analytics and AI to enhance efficiency and precision in modeling, simulation, and the development of actionable recommendations. It encompasses three key capabilities that will be core to the State's goals in this effort.

- **Capability #1: high-performing provider footprint design** capability integrates network adequacy and provider performance assessments with AI-based modeling to develop optimized healthcare networks that balance cost, quality, and access measures. This allows rapid testing of multiple configurations and benchmarking to identify opportunities for improvement, e.g., cost reduction and expanded access in underserved areas.
- **Capability #2: future healthcare utilization projection** capability leverages demographic, technological, and policy data to forecast care demand and utilization over 5-10 years, offering transparent, scenario-based insights into how system changes will influence future needs and provider performance.
- **Capability #3: healthcare site and capacity planning** capability uses geospatial analytics and scenario modeling to identify and address service gaps, guide site development or expansion, and evaluate footprint options across dimensions such as quality, affordability, capital cost, and workforce requirements.

Together, these capabilities form a comprehensive, AI-enabled foundation to help the State make data-driven, strategic decisions that deliver measurable improvements in healthcare system performance when redesigning the health system footprint.

Capability #1: Design high-performing provider footprint with integrated network adequacy assessment and provider performance assessment

The Healthcare Capacity Planning and Optimization engine integrates a network adequacy assessment, a provider performance assessment, and an AI-based network design into one end-to-end automated process. It is designed to help organizations create and optimize cost-efficient, high-quality health system footprints. By using advanced machine learning, it generates thousands of footprint scenarios, offers insights into cost efficiency, access, and quality, and provides foundations for strategic decisions. The tool will use the data asset developed for this effort using the methods and sources described above.

What distinguishes our Healthcare Capacity Planning and Optimization engine is its ability to integrate provider performance measures beyond cost and to rapidly test multiple scenarios. This allows users to compare key metrics across different scenarios, addressing areas of need based on the ideal optimization of cost, quality, access, and other factors. This capability has been used to designing high-performing networks with select anchor providers or health systems, conducting network benchmarking to assess network performance across the product portfolio, and optimizing provider footprints to reduce costs and improve patient access to care. For example, we supported a regional health plan creating a high quality and higher performance network product, in which the improvement in quality and efficiency enabled the health plan to reduce premiums by 5-7% while ensuring effective access. In another scenario, the Healthcare Capacity Planning and Optimization engine helped a dialysis provider identify 100+ potential markets where access to care was lagging as potential expansion targets, providing better access for patients in underserved areas.

Applied to Vermont: This capability could be used in Vermont's healthcare transformation in at least two ways. The first is a provider cost efficiency assessment. The advanced machine learning algorithm would deliver a uniform assessment of provider cost efficiency by evaluating both unit cost variation and utilization efficiency across the care journey. These insights would inform footprint optimization and scenario evaluation to guide data-driven decision-making. The second is network footprint optimization. Using linear regression optimization, this asset would generate several footprint scenarios with various footprint breadth, cost efficiency, and access to care that meet pre-set minimal requirements, allowing Vermont to develop an 'ideal state.'

Integrate network adequacy, provider performance assessment and machine-based design in one

Illustrative example

1 Portfolio Performance

Real-time network performance monitoring; with flexibility to set parameters and explore networks, markets and providers nationally

2 Network Access & Adequacy

Machine-surfaced provider insights with built-in adequacy standard logic surfacing any gaps

3 Network Build & Optimization

Flexibly build and manage scenario comparisons; user-configurable machine-learning scenarios to assess impact of changes

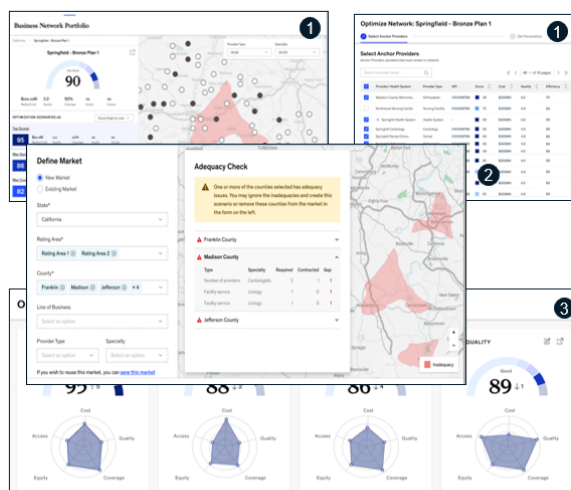


Exhibit 1: Capability #1, high-performing provider footprint design

Capability #2: Project future healthcare utilization with various demographic, technological, and other factors

Integrated into the Healthcare Capacity Planning and Optimization engine is the ability to generate county-by-service-line forecasts of care volumes and shifts in site-of-care utilization

over 5-10 years, drawing on claims, demographic, and macroeconomic data described above and other sources provided by the State. The model establishes a robust, data-driven baseline that captures local utilization patterns and service-line dynamics. Beyond that baseline, it integrates policy and market assumptions, such as public health, technology adoption, workforce availability, and payment or regulatory changes, through structured, transparent scenario settings applied at a fine-grained county and service-line level, reflecting how real-world changes emerge and stabilize over time. This design keeps forecasts analytically rigorous yet intuitively explainable, producing realistic, context-sensitive projections for how utilization may evolve across settings and regions.

What makes this approach distinctive is its two-layer design: AI models generate statistically grounded growth trajectories, while a mathematically disciplined calibration layer translates expert and policy insight into granular, auditable adjustments with built-in guardrails that keep projections plausible, stable, interpretable, and causally grounded in county-level micro-details. The result is a transparent, flexible forecasting system that can test multiple futures side-by-side. For example, it could evaluate how site-neutral payments or workforce shortages could reshape demand and access and provides states and clients with a quantitative foundation to plan future healthcare capacity, inform budgets, and design sustainable service networks.

Applied to Vermont: This capability could be used in Vermont's healthcare transformation in a few ways. The first is to develop the future utilization forecast. This capability will be used to project future healthcare demand of Vermonters across priority services, considering various factors, including site-of-care shifts, demographic mix, population growth, historical utilization trends, and baseline urban/rural mix, among others. Further, this could be used to support the provider performance assessment. As patient and utilization mix shifts with demographic changes, provider performance in cost efficiency and quality will also evolve. By leveraging forecasts of future utilization, we will be able to provide not only providers' current performance but also their future performance assessment to support forward-looking decision-making.

Healthcare utilization forecast model informs how volumes are growing in various sites of care

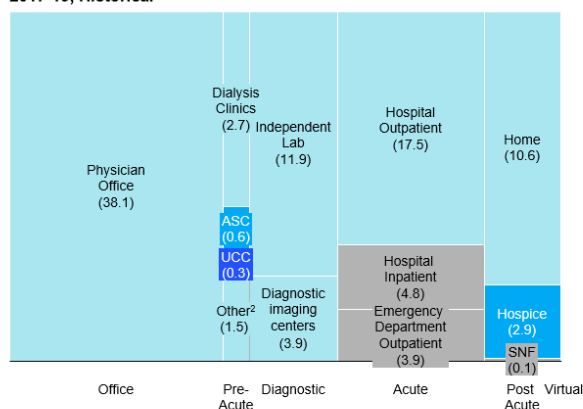
Medicare FFS business

Sample Outputs

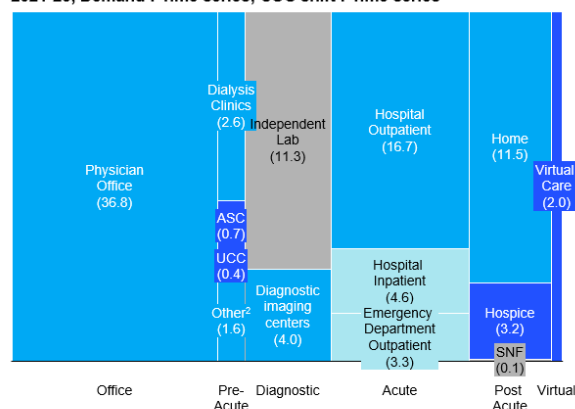
Utilization CAGR █ <0% █ 0-2% █ 2-5% █ >5%

Utilization¹ distribution across sites³ of care for xx service line (market share as a % of total utilization)

2017-19, Historical



2021-25, Demand : Time series; SOC shift : Time series



1. Utilization refers to count of procedures for all sites of care, except hospital IP & SNF which are based on count of admits
2. Includes other pre-acute sites of care like retail clinics, onsite clinics, infusion centers, OP behavioral health, etc.
3. Excludes sites which are not available or cannot be identified accurately in the claims data like long term acute care hospitals, dental clinics, etc.

Source: McKinsey proprietary time-series model, CMS LDS dataset, expert inputs

McKinsey & Company 7

Exhibit 2: Capability #2, future healthcare utilization projection

Capability #3: Plan and optimize healthcare sites and capacity to resolve service lines and areas with gaps in healthcare supply

This Healthcare Capacity Planning and Optimization engine is also designed to forecast future healthcare demand and needs in terms of capacity, capability, and costs at a population segment and clinical disease level. The analytics engine simulates the impact of demographic changes, clinical risk and disease changes, the impact of new models of care and other strategic choices, and improvement measures. This capability has been applied in various healthcare systems worldwide, from small countries like Iceland to large ones like the UK.

This capability includes an integrated visualization dashboard that runs on a secure data platform, building off the parameters that matter most to Vermont. For example, the analytics engine can provide insights into drive time to understand how access to care varies across different seasons and different times of the day.

Applied to Vermont: This capability could be used in Vermont's healthcare transformation in several ways. First, it could examine seasonal driving time and traffic patterns. Second, in the development of potential new sites, conversions, and expansions in the 'ideal state'. Leveraging advanced geospatial analytics, these changes in the footprint could aid in resolving any gaps in overall footprint based upon future demand. When gaps are identified, this asset will integrate any potential sites with capacity for conversion and/or expansion and identify a set of scenarios that will support the State in achieving its strategic priorities. Finally, this capability enables the evaluation of multiple scenarios in a short timeframe. Based on our experience using for 10+ public sector clients across the globe, this tool is enhanced with a well-developed framework to evaluate scenarios from all dimensions, including quality of care, access to care, affordability of care, capital cost and transition cost, and workforce and staffing needs.

Modelling population flows can help understand access and patient volume changes under new site configuration options

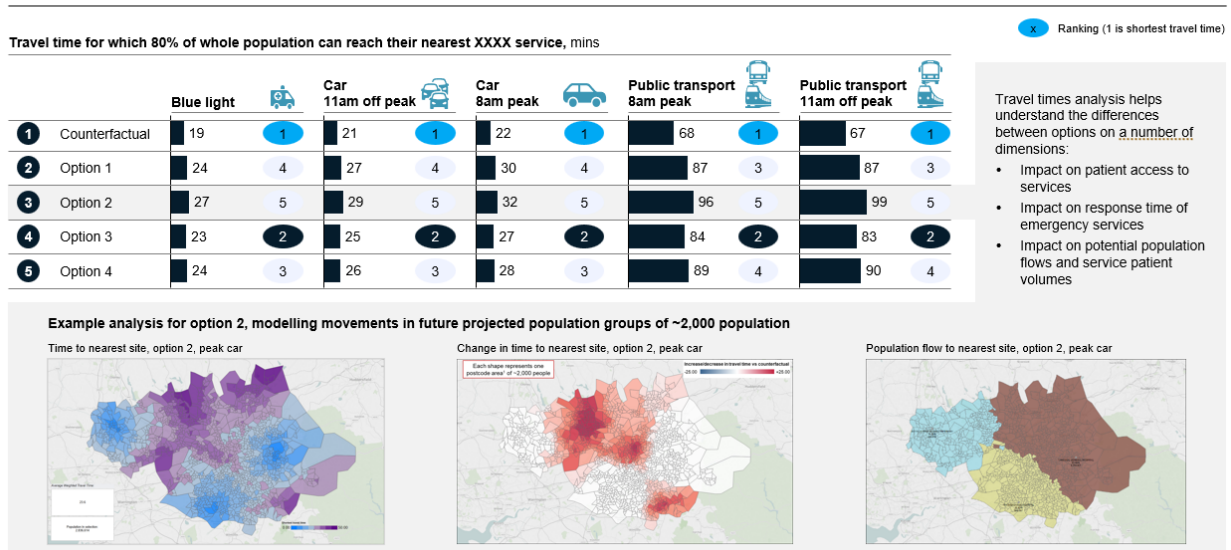


Exhibit 3: Capability #3, healthcare site and capacity planning

2.3.3 People

We have assembled a hand-selected team of practitioners and experts whose collective experience and professional passion directly align with Vermont's goals for a data-driven hospital and regional transformation. This team brings together a diverse range of perspectives including clinicians, data scientists, policy advisors, operators, and Medicaid transformation specialists ensuring a deep understanding of every dimension of this work. Together, our team will combine national best practices with local insight to deliver transparent, evidence-based, and actionable results from day one.

Our proposed accountable core leadership team **Sarun Charumilind, M.D. (Project Manager)**, **Thomas Pellathy (Accountable Senior Partner)**, and **Ronnie Thompson (Accountable Associate Partner)**, as well as our extended leadership team and core subject matter experts, bring experience and expertise in the topics that matter most critical for this scope of work, namely: experience supporting US states; hospital or healthcare provider transformation; regional provider network transformation; innovative care model innovation, redesign, or implementation; healthcare reform initiative design; and/or experience leading advanced data and analytics efforts for healthcare capacity, network, or transformation planning.

Exhibit 4 on the following page provides an overview of our proposed team, and the tables that follow provide summarized biographies for each. Detailed resumes for each named team member are included in **Appendix A: Resumes**.

Category	Team Member	Proven experience areas						
		Supporting US States HHS agencies	Supporting hospital or healthcare provider transformation	Supporting regional provider network transformation	Supporting care model, innovation, redesign or implementation	Supporting healthcare reform initiative design	Leading advanced data and analytics efforts for healthcare capacity, network, or transformation planning	Total years of relevant experience
Accountable McKinsey Leadership Team	Sarun Charumilind, M.D. (Medicaid / states / network) <i>Project Manager</i>	●	●	●	●	●	●	15+
	Thomas Pellathy (network design / value-based care) <i>Accountable Senior Partner</i>	●	●	●	●	●	●	20+
	Ronnie Thompson (alternative sites of care) <i>Accountable Associate Partner</i>		●	●	●		●	5+
	Nikhil Sahnii (hospital / care delivery)	●	●	●	●	●	●	15+
	Michael Elliott (provider transformation)	●	●	●	●	●	●	14+
	David Nuzum (Medicaid / states)	●	●	●	●	●	●	25+
Subject Matter Experts	Kirk Fischer (payor network)		●	●	●	●	●	30+
	Page West, RN (hospital operations)		●		●			30+
	Will Taylor (care delivery footprint)	●	●	●	●	●	●	15+
	Yenli Wong (workforce)		●	●	●	●	●	10+
	Michael Rubin (data and analytics)	●	●	●	●	●	●	15+
	Jenny Rost (rural healthcare, value-based care)	●	●	●	●	●	●	15+
	Aarthi Sridhar (hospital finance)		●		●	●		6+
	Amit Shah, M.D. (healthcare agency strategy)	●	●	●	●	●	●	15+
	Andrew Odden, M.D. (provider and hospitals)		●	●	●		●	22+
	Yuqi Shi (network tools and capabilities)	●	●	●	●	●	●	8+
	Ankit Jain (data, claims forecasting)		●	●	●		●	14+
	Brian Latko (Medicaid / states)	●	●	●	●	●	●	17+

Exhibit 4: Our proposed team's proven experience and expertise

Accountable McKinsey Core Leadership Team

Team member	Experience and key qualifications
 Sarun Charumilind, M.D. <i>Project Manager</i> <i>Medicaid / states / network</i>	Sarun is a Partner with over 15 years of experience leading healthcare transformation and payer–provider network strategy initiatives. He has served six State Medicaid agencies and multiple Managed Medicaid and Medicare Advantage payers on affordability, access, and quality improvement, and co-leads McKinsey’s Healthcare Affordability & Outcomes team of more than 50 experts, analytics, and data science practitioners. Sarun brings deep expertise in network optimization, care model redesign, and hospital ecosystem transformation, combining analytic rigor with operational insight. Trained in primary care and internal medicine at Massachusetts General Hospital, he integrates clinical experience with strategic and analytic capabilities to help organizations improve sustainability and health outcomes.
 Thomas Pellathy <i>Accountable Senior Partner</i> <i>Network design / value-based care</i>	Tom is a Senior Partner with more than 20 years of US and global healthcare experience in system reform, quality and population health, value-based care, and advanced analytics. He leads our center of excellence in healthcare value (working with health systems to improve quality and total cost of care) and co-leads McKinsey’s Global Health System Practice, working across multiple countries supporting national and regional publicly funded health systems enhance care quality, access, and financial sustainability. Previously, as Executive Vice President at Highmark Health, Tom oversaw product design, quality and population health, network management and value-based contracting, and digital transformation for a \$29B enterprise. His work integrates payment innovation, analytics, and multi-stakeholder collaboration to drive measurable outcomes. Tom’s experience spans commercial and government programs, emphasizing evidence-based improvement, operational excellence, and long-term system sustainability, including deep work across topics of rural health transformation.
 Ronnie Thompson <i>Accountable Associate Partner</i> <i>Alternative sites of care</i>	Ronnie Thompson is an Associate Partner with deep experience partnering with not-for-profit and regional health systems on growth strategy and transformation, with an emphasis on site of care optimization and ambulatory strategy. She co-leads McKinsey’s Ambulatory Diversification Service Line and has extensive experience shaping data-driven ambulatory strategies that increase patient access and lower total cost of care. She has led enterprise-wide growth transformations, service-line strategies, and partnership development initiatives, delivering measurable impact through improved patient access, enhanced provider alignment, and operational efficiency. Her work helps healthcare leaders reimagine where and how care is delivered – driving performance improvement, financial sustainability, and improved patient access.

Extended McKinsey Leadership Team

Team member	Experience and key qualifications
 Nikhil Sahni <i>Hospital / care delivery</i>	Nikhil is a Partner with nearly two decades of healthcare consulting experience advising providers, payers, and innovators across the value chain. He leads McKinsey's Center for US Healthcare Improvement, driving data-driven initiatives to enhance productivity, outcomes, and affordability. A recognized healthcare thought leader, Nikhil has shaped state and national reform efforts, including as Policy Director at the Massachusetts Health Policy Commission, where he oversaw cost trends and delivery system innovation. His expertise spans health system transformation, payment model redesign, and administrative simplification, with a focus on leveraging digital and AI-enabled solutions to improve operational efficiency and patient outcomes nationwide.
 Michael Elliott, Ph.D. <i>Provider transformation</i>	Michael is a Senior Partner and leader of McKinsey's Healthcare Transformation Center of Excellence with over 15 years of experience advising hospitals and health systems on cost efficiency, quality improvement, and growth strategy. He has led turnarounds, restructuring efforts, and performance-improvement programs across academic, community, and pediatric settings, both for-profit and not-for-profit. Michael specializes in physician alignment, labor and supply optimization, and revenue cycle improvement, helping organizations achieve sustainable operational and financial performance. His holistic, data-driven approach enables providers to deliver high-quality, affordable care while strengthening organizational resilience and long-term stability.
 David Nuzum <i>Medicaid / states</i>	David is a Senior Partner with over 25 years of experience leading healthcare transformation across hospital systems, private payers, State Medicaid programs and other health and human services programs. He leads McKinsey's State & Local Government Practice, which has supported nearly 40 states across health and human services and other agencies. He co-founded McKinsey's Healthcare Innovation domain and McKinsey's Healthcare Analytics capabilities, supporting advancing payment reform, provider network design, and care model redesign across hospitals, primary care, and community health settings. He has led McKinsey support to Medicaid clients in 17 states including Massachusetts, New York, Connecticut, Maine, and New Jersey, among others, helping shape strategies to improve population health, access, and chronic care management. His expertise bridges policy design, analytics, and implementation, ensuring lasting impact for public and private sector systems.

Core Subject Matter Experts

Expert	Experience and key qualifications
 Kirk Fischer <i>Payor network</i>	Kirk Fischer is a Senior Advisor with over 30 years of healthcare leadership experience in health plan strategy, provider network optimization, and value-based care. He has a deep track record guiding Medicaid, Medicare, and commercial payers through large-scale network transformations that integrate strategic analytics and stakeholder engagement to improve access, quality, and cost efficiency. As a former senior vice president at major national insurers (Centene, Florida Blue), Kirk led network design and contracting teams (400+ staff) to optimize provider footprints and align incentives for quality and cost outcomes. He has orchestrated statewide delivery system redesigns, including footprint optimization and shared-savings programs, delivering significant cost savings and enhanced access to care. Kirk's expertise in data-driven network planning and provider alignment informs regional transformation strategies critical to hospital sustainability.
 Page West, RN <i>Hospital operations</i>	Page is a healthcare executive with over 30 years of clinical and leadership experience, including serving as Senior Vice President and System Chief Nurse Executive for Dignity Health (oversight of nursing across 39 hospitals). She is an expert in workforce optimization and hospital operations, with a track record of improving staff efficiency, care quality, and cost management across large hospital networks. As a McKinsey Senior Advisor, Page has supported major hospital transformation efforts across the country, guiding the implementation of new care delivery models with strong stakeholder engagement to ensure sustainable operational improvements.
 Will Taylor <i>Care delivery footprint</i>	Will Taylor is an Expert Associate Partner with extensive experience leading health system transformation efforts across the UK, Europe, North America, and the Middle East. He has partnered with payors, providers, and governments on initiatives including hospital capital deployment planning, care model redesign, and financial and clinical service strategy. Will has led more than 10 public health system strategy programs, covering populations ranging from 1 million to 9 million. These have involved hospital and regional provider network transformation, care model redesign and implementation as well as the design of healthcare reform initiatives at the regional and statewide levels. He has also led advanced data and analytics efforts for capacity and demand forecasting, network planning, and capital expenditure optimization.
 Yenli Wong <i>Workforce</i>	Yenli Wong is a Partner in McKinsey & Company's Los Angeles office and a leader in the firm's Healthcare Systems and Services Practice. She specializes in workforce excellence and transformation, clinical operations, and the future of care delivery. Yenli is passionate about integrating clinical operations, digital innovation, organizational design, and culture and change management to help healthcare organizations balance cost management with delivering high-quality patient care. With a focus on creating engaged and empowered workforces, Yenli brings a holistic perspective to solving some of the most pressing challenges in workforce today.

Expert	Experience and key qualifications
 Michael Rubin <i>Data and analytics</i>	Michael is a Senior Asset Leader within McKinsey's Healthcare & Public sector Tech practice, based in New York. He brings over 15 years of experience helping public- and private-sector healthcare organizations design, build, and scale digital and data-driven solutions that enhance system performance, quality, and affordability. He has led large-scale innovation programs with national health agencies, public payers, and technology partners, integrating policy expertise with advanced analytics and operational insight. His experience spans data integration, predictive modeling, and visualization, enabling decision-quality outputs that inform policy, regulatory design, and healthcare transformation.
 Jenny Rost <i>Rural healthcare and value-based care</i>	Jenny is a Partner with more than 14 years of experience leading large-scale healthcare transformation and rural system redesign initiatives. She led McKinsey's support for Pennsylvania's CMS-approved Rural Health Model, driving multi-payer transformation across hospital systems and state agencies. She has guided multiple states in payment reform, delivery innovation, and Medicaid modernization, developing deep expertise in regulatory design, stakeholder engagement, and policy execution. Jenny co-leads McKinsey's Value-Based Care & Delivery Innovation Service Line, leveraging analytics to improve cost efficiency, access, and quality outcomes, with a focus on sustainable improvement for rural and community-based systems.
 Aarthi Sridhar <i>Hospital finance</i>	Aarthi Sridhar is an Associate Partner in McKinsey's Bay Area office, specializing in health system transformations, with a focus on financial sustainability and growth. With a background in structural engineering and analytics, she brings a rigorous, systems-based approach to solving complex healthcare challenges. She has led large-scale transformation programs that deliver measurable impact—spanning cost optimization, growth acceleration and institutionalizing financial and operating infrastructure that improve rigor and accountability. Her work on growth focuses on service-line strategy, access and network optimization through evidence and data-backed strategies. She believes deeply in partnership and capability building—working alongside leadership teams to strengthen internal processes, develop lasting capabilities, and embed a culture of continuous improvement.
 Amit Shah, M.D. <i>Healthcare agency strategy</i>	Amit is a Senior Partner and co-leads McKinsey's Healthcare Analytics and NextGen Care Management practices as well as our healthcare affordability practice. He is a physician by training with 15+ years of experience advising state Medicaid agencies and public health systems; he is an expert in data strategy, payment reform, and performance management. He has led statewide initiatives in hospital care, primary care, behavioral health, LTSS, and HCBS, helping clients align policy, analytics, and operations to improve outcomes and cost efficiency. His work spans care management redesign, and utilization optimization, and he has helped states develop 1115 waivers and State Plan Amendments that align financing and delivery systems with CMS priorities.

Expert	Experience and key qualifications
 Andrew Odden, M.D. <i>Provider and hospitals</i>	Andrew is an Associate Partner in McKinsey's Healthcare Provider Transformation Practice with more than 15 years of experience in clinical leadership and hospital operations. A former Vice Chair of Medicine at Washington University in St. Louis and leader in the VA health system, he brings deep expertise in strategic planning, service line growth, and operational improvement. He has led major hospital transformations improving throughput, access, and physician engagement, and developed multi-year digital and site-of-care strategies for regional health systems. Andrew specializes in geospatial analytics, network optimization, and stakeholder alignment to strengthen hospital performance and care delivery.
 Yuqi Shi <i>Network tools and capabilities</i>	Yuqi is a Solution Manager with more than nine years of healthcare consulting experience across public and private payers, providers, and analytics organizations. She has deep expertise in medical cost management, value-based payment design, and healthcare data analytics, developing data-driven strategies to improve cost, access, and quality. Yuqi has led cross-functional teams to deliver care delivery transformation and financial performance improvements for state agencies and health systems. Recognized for her analytical rigor and collaborative approach, she excels at translating complex data insights into actionable strategies that drive measurable impact.
 Ankit Jain <i>Data, claims forecasting</i>	Ankit is an Associate Partner (Solutions) with more than 14 years of experience in healthcare analytics, data strategy, and financial modeling. He specializes in claims analytics, cost reduction, and growth strategy, supporting insurers, providers, and private equity clients. Ankit has led cross-functional teams developing advanced data solutions for M&A, network optimization, and clinical service modeling, blending actuarial expertise with strategic insight. His work emphasizes operational excellence and innovation, helping organizations translate analytics into actionable decisions that improve financial performance, efficiency, and care delivery across large-scale healthcare systems.
 Brian Latko <i>Medicaid / states</i>	Brian is a Partner and leader of McKinsey's Medicaid Domain, with 17+ years of experience in Medicaid, public health and human services. He has worked with 15+ states on Medicaid Program Performance topics including managed care strategy, procurement, and performance management, driving improvements in Medicaid, CHIP, LTSS, and dual-eligible programs. Brian's expertise spans MCO strategy, procurement design, data transformation, and analytics capability building. He brings deep experience in modernizing oversight processes and embedding data-driven performance tracking, enabling states to enhance program integrity, access, and value across complex healthcare delivery systems.

2.3.4 Similar experience in the last 5 years

In the following section we have highlighted nearly 20 examples of recent, relevant experience on similar consulting engagements in the past 5 years. Of these examples, 15 are with State HHS or Medicaid agencies and 4 examples are with international or private sector clients. These examples demonstrate our qualification as a bidder for this scope based on our track record of work and impact on:

- **Hospital or healthcare provider transformation** (14 client engagement examples)
- **Regional provider network transformation** (14 client engagement examples)
- **Care model innovation, redesign, or implementation** to improve affordability, utilization patterns, capacity, access, hospital/provider financial sustainability, quality, and/or population health (15 client engagement examples)
- **Healthcare reform initiative design** at the hospital, regional, or statewide level (18 client engagement examples)
- **Advanced data and analytics for healthcare capacity, network, or transformation planning** (all client engagement examples)

The select catalog of client engagement examples highlighted demonstrate our breadth across varied contexts including public and private sector, US and global, densely populated and rural geographies, and therefore our ability to tailor proven approaches to new contexts. In all cases, we have enabled or accelerated analytics-powered impact on the above themes. While we have not served the State of Vermont government directly, we have supported health systems in Vermont.

Exhibit 4 on the following page includes a summary of the highlighted client engagement examples and the themes relevant to this Vermont scope for which they demonstrate our qualifications. Detailed case studies for each client engagement example are included in the section that follows.

#	State or Geography	Agency	Period of Performance	Topic	Hospital or healthcare provider transformation	Regional provider network transformation	Care model innovation, redesign, or implementation	Healthcare reform initiative design	Advanced data and analytics for healthcare capacity, network, or transformation planning
1	Georgia	Dept. of Community Health & Healthcare Workforce Commission	Sep 2022 - Jan 2023	Workforce capacity diagnostic, strategy, and planning	•		•	•	•
2	Western US state	Regional non-profit health insurer	Jul 2025 - Oct 2025	Regional provider and network transformation analytics and strategy	•	•	•		•
3	United Kingdom (national)	National Health Service	Jan 2023 - Mar 2024	National new hospital analytics, capacity planning, and capital strategy	•	•	•	•	•
4	New York	Office of Health Insurance Programs (Medicaid)	May 2024 - Jun 2025	Regional hospital transformation, access, and value-based care planning	•	•	•	•	•
5	Multi-state	Large non-profit health insurer	Jan 2025 - Oct 2025	Statewide provider network analytics, value-base care, and planning	•	•	•	•	•
6	Iceland (national)	Ministry of Health	Aug 2021 - Dec 2021	National hospital supply and demand analytics, strategy, and capacity planning	•	•	•	•	•
7	Missouri, Montana, West Virginia	Depts. of Health and Human Services	Oct 2025	Rural health transformation analytics and strategies	•	•	•	•	•
8	US (national)	West Health, nonprofit focused on successful aging	Aug 2024 - Oct 2025	National primary care capacity transformation model		•	•	•	•
9	California	Dept. of Health Care Services	Jan 2022 - Jun 2025	Statewide behavioral health analytics and access expansion	•	•	•	•	•
10	Ohio	Dept. of Aging, with Depts. of Medicaid and Health	Apr 2024 - Sep 2024	PACE program, quality, and service expansion	•	•	•	•	•
11	Ohio	Dept. of Aging, with Depts. of Medicaid and Health	Oct 2023 - Sep 2024	Statewide nursing home quality and capacity optimization	•	•	•	•	•
12	New York	Office of Health Insurance Programs (Medicaid)	May 2024 - Oct 2025	Social Care Network program and services design and launch		•	•	•	•
13	Virginia	Office of the Secretary of Health & Human Resources	Nov 2022 - Apr 2023	Statewide behavioral health access and service transformation	•	•	•	•	•
14	Kansas	Dept. of Aging & Disability Services	May 2024 - Jun 2025	Psychiatric care system access and service transformation	•	•	•	•	•
15	Oklahoma	Dept. of Mental Health & Substance Abuse Services	Nov 2023 - Mar 2024	Behavioral health transformation analytics and diagnostic	•	•		•	•
16	Indiana	Family & Social Services Administration	Jan 2022 - Jul 2022	LTSS data surveillance plan design and implementation			•	•	•
17	New Jersey	Dept. of Health	Nov 2020 - Jul 2023	COVID-19 response and public health data modernization				•	•
18	Massachusetts	Executive Office of Health & Human Services	Nov 2020 - Dec 2022	COVID-19 long-term care response and system stabilization				•	•
19	Nebraska	CyncHealth & Dept. of Health & Human Services	Aug 2021 – Aug 2023	Digital Health Hub and immunization platform development				•	•

Exhibit 5: Relevant client engagement examples in the last 5 years

Example 1:

Georgia Department of Community Health – Healthcare Workforce Commission workforce capacity diagnostic, strategy, and planning	
Dates of engagement	September 2022 – January 2023
Relevance	
Conducted a comprehensive assessment of healthcare workforce capacity and designed data-driven strategies to improve access, quality, and sustainability across the care delivery system. Quantified current and projected workforce shortages by region and specialty, identified structural gaps impacting care delivery, and identified policy levers to address long-term system sustainability. Combined analytics with stakeholder engagement to develop transformation recommendations that aligned with state policy goals, regulatory considerations, and CMS-aligned initiatives.	
Description	
Partnered with Georgia’s Healthcare Workforce Commission and the Department of Community Health, including the Medicaid agency, in response to growing concerns about shortages across critical healthcare professions that were impacting the State’s ability to ensure access, maintain quality, and support workforce sustainability. Built a comprehensive fact base, integrating labor force data, provider licensing data, educational pipeline statistics, and healthcare utilization forecasts to estimate current and projected workforce shortages across regions and specialties. Benchmarked Georgia’s performance against peer states to identify competitive risks and opportunities. Conducted structured interviews with leaders from health systems, medical schools, nursing programs, and workforce boards to uncover barriers to training, recruitment, and retention. Evaluated the impact of different interventions on long-term workforce supply and access to care through scenario modeling. Facilitated working sessions with Commission members to align on root causes and prioritize strategic levers for action (e.g., expanding scope of practice, accelerating training pathways, increasing retention incentives, and creating public-private partnerships). Refined recommendations based on stakeholder input, ensuring final outputs were pragmatic, implementable, and aligned with state and federal policy environments.	
Impact of our work	
- Produced a comprehensive set of data-driven recommendations to strengthen Georgia’s healthcare workforce pipeline and improve long-term access to care - Supported the advancement of statewide initiatives including loan forgiveness programs tied to in-state retention of graduates - Enabled the development of pilot programs such as “Quick Start for Healthcare” to expand rapid-entry training pathways - Informed policy proposals that expanded scope of practice for qualified professionals to alleviate provider shortages - Established a collaborative action framework adopted by state agencies, educational institutions, and healthcare employers to coordinate workforce implementation efforts	

Example 2:

Western U.S. state regional nonprofit health insurer – Provider and network transformation analytics and strategy	
Dates of engagement	July 2025–October 2025
Relevance	
Focused strategies and approach for regional network transformation, programs to enable hospital and primary care transformation, and required advanced data and analytics approaches to assess healthcare demand, capacity, and potential network configurations to achieve affordability, quality, and access goals.	
Description	

Western U.S. state regional nonprofit health insurer – Provider and network transformation analytics and strategy

Supported a Western U.S. State to design its network and payment model strategy across commercial, Medicare Advantage, and Medicaid lines of business. Required ingestion of claims, Stars, quality, and other data, and integration with third-party data sources (e.g., national provider directory, Ribbon Health payer network data), to establish a fact base for hospital, primary care, and specialist performance on cost efficiency, quality, and access. Also required assessment of performance and pain points of existing value-based payment models. Facilitated a strategic design process to define network goals across not only affordability, quality, access but also provider sustainability and potential consumer attractiveness of future insurance products. Deployed the Healthcare Capacity Planning and Optimization engine to generate potential network scenarios and understand potential benefits and trade-offs of each scenario. Modeled potential financial impacts to the insurer and also to hospitals and independent providers. Supported provider and other stakeholder communications and contract negotiation strategies, and supported provider enablement strategy, initiative development, and investment prioritization.

Impact of our work

- Established an integrated dataset on provider performance on cost efficiency, quality, and access
- Supported development of overall network and provider strategy
- Conducted advanced network analytics to generate potential network scenarios that could generate 3-5% medical cost savings while improving quality, Stars performance, and maintaining network access for members
- Supported executive leaders to develop change story, communications approach, and negotiation strategy

Example 3:

UK National Health System – National new hospital analytics, capacity planning, and capital strategy

Dates of engagement January 2023 – March 2024

Relevance

Used the Healthcare Capacity Planning and Optimization engine to integrate utilization, demographic, future changes to digital technology and models of care, and cost data to model demand, capacity, workforce, capital requirements, and operational cost trajectories across more than 40 hospitals, including cross-network patient flows and considerations around service rationalization. Enabled central authorities and hospital operators to run future-state scenarios that directly informed national investment decisions. Recommendations addressed escalating capital costs, uneven access to care across regions (including rural and underserved communities), aging infrastructure, and the need to align hospital capacity with long-term demand trends.

Description

NHS New Hospital Program aimed to redesign and modernize hospital infrastructure at scale, improve clinical sustainability, reduce costs, and ensure equitable access across regions. Used Healthcare Capacity Planning and Optimization engine to ingest national datasets covering activity, demographic trends, clinical standards, capital costs, and workforce requirements. Enabled both rapid needs assessment and deeper feasibility modeling for individual projects, all within a consistent national framework.

Established a standardized “minimum viable hospital” model that incorporated required clinical services from a regional network perspective, government safety standards, sustainability targets, digital capabilities, and operational efficiency design principles. Allowed users to test different design and capacity configurations, quantify capital and operating impacts, and model the trade-offs between cost and access. Generated business-case-ready outputs and ran cross-hospital benchmarking and portfolio-level optimization assessments. Provided the government with a structured, technology-

UK National Health System – National new hospital analytics, capacity planning, and capital strategy	
enabled approach to determine hospital size, service mix, and investment scale; ensured capital decisions were aligned with long-term demand forecasts and value for money.	
Impact of our work	
- Delivered a national business case that secured the largest central government capital allocation in more than 75 years, enabling full funding of the hospital program - Identified 25% in capital cost savings through standardization, modular design, optimization of existing infrastructure within the network, and scenario-based optimization around digital and new models of care across more than 40 hospitals - Accelerated the national approvals process by introducing a digital, pre-compliant business case generation tool that aligned local plans with central criteria - Enabled right-sizing of individual hospitals by modeling future bed demand, operating requirements, and cost implications, ensuring each facility met projected capacity needs while minimizing unnecessary expenditure - Established a unified digital asset for ongoing forecasting and scenario evaluation, allowing national leaders to continuously assess the impact of policy decisions and transformation initiatives on capacity, capital budgets, and operational sustainability	

Example 4:

New York State Office of Health Insurance Programs (Medicaid) - Regional hospital transformation, access, and value-based care planning	
Dates of engagement	May 2024 – June 2025
Relevance	
Combined multi-sector data, conducted market and financial assessments, modeled service line sustainability, and developed analytics to inform decisions on cost, quality, access, and regional transformation planning. Supported development of recommendations aligned with CMS requirements, prioritized transformation strategies for safety-net providers, and designed governance structures to coordinate stakeholders and ensure transparency. Work emphasized rural health transformation by identifying financially vulnerable hospitals and modeling interventions that protect access to essential services.	
Description	
Partnered with New York State's Medicaid agency to prepare its application to the CMS AHEAD program and to design a statewide, multi-payer hospital global budget model intended to transform financial sustainability and care delivery across regions. Integrated multiple state and federal data sources, including hospital financial and quality reporting systems and Medicare and Medicaid claims from the CMS Virtual Research Data Center (VRDC), to create a comprehensive analytic foundation for transformation modeling. Leveraged state hospital cost reports, quality metrics, and multi-year Medicare and Medicaid claims from the CMS Virtual Research Data Center to assess care delivery across inpatient, outpatient, primary care, specialty care, behavioral health, digital care, and telehealth modalities. Conducted detailed market and financial analyses to evaluate current hospital performance, including medical cost trends, payer mix, quality outcomes, access to services, and financial vulnerability, particularly for safety-net and rural hospitals. Assessed system performance and identified opportunities for care model redesign, such as shifting care from inpatient to ambulatory or home-based settings, expanding telehealth and hospital-at-home programs, integrating preventative services, and addressing chronic disease management. Performed geospatial modeling to assess regional access to services, travel times, and rural capacity constraints. Evaluated how patient populations could be attributed to hospitals under different budget scenarios, ensuring adequate access while aligning incentives for efficiency and quality improvement. Supported design of proposed payment model, including attribution methodologies, budget-setting processes, quality incentive structures, and multi-payer participation strategies. Benchmarked model elements against national value-based care programs and CMS guidelines to ensure viability and alignment with federal	

New York State Office of Health Insurance Programs (Medicaid) - Regional hospital transformation, access, and value-based care planning	
expectations. Developed New York's hospital transformation support model, including a structured framework for hospital transformation plans, implementation roadmaps, and performance monitoring. Facilitated decision-making across Medicaid, Finance, Quality, and IT agencies to ensure operational readiness and supported stakeholder engagement with hospitals, associations, and other partners. Supported the State in establishing a governance and operating model to align Medicaid, commercial payers, hospital associations, and community stakeholders in coordinating the rollout of payment and delivery reform statewide. Final deliverables included a comprehensive report and CMS-ready documentation of all model design, data analytics, transformation strategies, and governance decisions.	
Impact of our work	
- Supported State's application for a CMS AHEAD grant (Q4 2024) - Completed Medicaid global budget model design, including detailed attribution, payment, quality parameters, and sustainability mechanisms - Produced detailed analyses to evaluate hospital and regional performance across cost, quality, access, chronic care management, and financial viability - Conducted operational readiness assessment across Medicaid, Finance, Quality, and IT agencies to support implementation - Developed hospital transformation roadmaps aligned with Medicaid waiver goals and state health equity, population health, and rural access objectives - Facilitated multi-stakeholder engagement across state agencies, payers, providers, and associations to coordinate implementation and policy decision-making	

Example 5:

Large non-profit health insurer – Statewide provider network analytics, value-base care, and planning	
Dates of engagement	January 2025 - October 2025
Relevance	
Modeled cost, access, and quality outcomes at the system and regional level, used geospatial analytics to evaluate provider network adequacy, forecasted financial sustainability under multiple scenarios, and structured payment models that balanced hospital viability with cost containment for payers and communities. Focused on ways to align value-based reimbursement, quality incentives, and population health to improve access, quality, and cost sustainability.	
Description	
Partnered with a large non-profit Blue in Midwest to develop and implement a three-phase provider network strategy aimed at improving member outcomes, strengthening the provider network in its evolution to value-based care, and ensuring financial sustainability. Developed a comprehensive fact base on provider and payer dynamics, patient flows, and quality and cost performance. Employed geospatial network and access modeling to assess healthcare accessibility, identify coverage gaps, and forecast the out-of-network impact on both providers and payers. Led the development of practical, concrete models of collaboration between the payor and provider, including defining areas of shared value creation. Developed a sustainable payer-provider agreement benefiting members, the insurer, and the health system. Transitioned full ownership to payer through repeatable analytics, hands-on coaching, structured playbooks, and strategic frameworks, empowering the organization to drive the strategy independently.	
Impact of our work	
- Shifted network to more value-based incentives leading to an estimated ~\$1-1.5B in savings for patients, customers and the community - Strengthened long-term provider partnerships and aligned incentives toward shared healthcare outcomes and areas of shared value creation	

Large non-profit health insurer – Statewide provider network analytics, value-base care, and planning

- Institutionalized capabilities within the payer, equipping the client teams with data-driven planning, execution discipline, and strategic independence for future partnerships

Example 6:

Iceland Ministry of Health – National hospital supply and demand analytics, strategy, and capacity planning

Dates of engagement August 2021 – December 2021

Relevance

Developed plan to: align acute care capacity with evolving demographic trends; control healthcare cost growth; evolve provider capacity to best meet the future needs of the population; protect access in rural settings; make evidence-based decisions about service regionalization and investment. Brought together national clinical activity, financial data, and demographic projections and deployed the Healthcare Capacity Planning and Optimization engine to generate demand forecasts (including impact of digital and models of care shifts), acute bed and other site of care needs, workforce requirements, and operating costs. Tested the impact of different transformation scenarios on access, quality, and sustainability. Provided transparent, benchmarked insights tied to internationally accepted standards.

Description

Developed a national hospital strategy in response to rising healthcare costs, growing demand pressures driven by an aging population, and concerns about long-term system financial sustainability. Deployed Healthcare Capacity Planning and Optimization engine, which integrates claims, clinical, financial, and workforce data. Built a baseline forecast of future demand, bed requirements, operating room and outpatient capacity, workforce needs by role, and cost trajectories under current-state assumptions.

Worked with key stakeholders to identify future drivers of changes to demand (including new technologies and emerging models of care) and to test multiple transformation scenarios. Evaluated the impact of shifting care to lower-acuity settings, consolidating specialty services, increasing productivity through digital tools and workforce redesign, and introducing new models of care such as hospital-at-home. Evaluated each scenario for its effect on access to services (including regional and cross-border flow), quality of care, staffing sustainability, and system-wide financial outcomes. Identified strategic options that would enable the country to meet clinical standards, maintain appropriate access for rural populations, and slow the rate of healthcare expenditure growth.

Developed national transformation plan that provided recommendations on which services should remain centralized, which could be regionalized or shifted to community-based care, and what investments were required to deliver projected system improvements. Recommendations are currently being implemented, including a major rebalancing of long-term care away from acute hospital settings.

Impact of our work

- Supported adoption of the national hospital strategy, directly shaping the government's plan for the future role of acute care
- Enabled creation of a national fund to develop new elderly care facilities, shifting lower-acuity, long-term care out of acute hospitals
- Demonstrated how Iceland could achieve an 85% bed-occupancy target by 2026 through shifting approximately 55% of long-term care out of hospitals, resulting in a temporary reduction in beds through 2030 before rising again through 2040 due to demographic pressures
- Reduced outpatient demand and emergency department congestion by shifting primary care delivery, while accommodating a projected 30% increase in operating room demand by increasing utilization rates from 56% to 73%
- Forecasted near-term workforce growth through 2026 to meet structural and research demands, followed by gradual reductions driven by productivity gains and digitization, resulting in slowed cost growth as long-term care transitions out of the hospital setting and operating efficiencies take effect

Example 7:

Missouri, Montana, West Virginia – Rural health transformation analytics and strategies	
Dates of engagement	October 2025
Relevance	
Focused on multi-year strategies to transform rural health systems, improve access to care, ensure long-term financial sustainability, and shift toward innovative care models. Required geospatial access modeling, regionalization scenarios, workforce strategies, and alternative payment model development to support statewide hospital transformation.	
Description	
Supported multiple states in designing their five-year rural health transformation strategies for submission to the CMS Rural Health Transformation Program. Worked with colleagues across the Governor's Office, State Medicaid Program, and other health and human services agencies. Conducted a comprehensive assessment of the States' rural healthcare ecosystems and evaluated hospital financial sustainability, patient access patterns, quality outcomes, and workforce capacity. Used geospatial modeling to analyze travel times, access challenges, and care delivery gaps across regions. Collaborated with state agencies and stakeholders to define clear, measurable "North Star" objectives focused on improving care access, outcomes, and provider viability. Designed fit-for-geography care delivery models combining in-person, hybrid, virtual, and hospital-at-home capabilities. Developed strategies to grow the pipeline, retain local providers, and upskill community-based talent. Assessed opportunities for rationalizing services, creating regional partnerships, and improving operating efficiency through digital tools and shared services. Designed payment models that reflect rural realities, move away from fee-for-service structures, and support financial sustainability. Developed implementation roadmaps, metrics, and governance models to guide execution over the five-year period	
Impact of our work	
- Helped states develop comprehensive and actionable five-year rural health transformation plans aligned with CMS requirements - Shaped strategy across access, workforce, sustainability, and payment reform domains - Supported the identification of outcome-focused goals and the creation of implementation roadmaps tied to each state's rural health priorities - Enabled states to better understand current rural healthcare landscape and define achievable pathways to strengthen care delivery, enhance provider viability, and improve outcomes	

Example 8:

West Health - National primary care capacity transformation model	
Dates of engagement	August 2025 – October 2025
Relevance	
Built a national, state-level, and regional analytic model to evaluate current and projected primary care system capacity, assess population health trends, benchmark performance using evidence-based standards, and simulate the impact of care delivery innovations over 5-, 10-, and 25-year horizons. Quantified system bottlenecks, analyzed the financial sustainability of care delivery approaches, and modeled the impact of transformation levers such as workforce expansions, care model redesigns, and curative therapeutics on cost, access, and capacity.	
Description	
Developed a dynamic, interactive analytical model of the U.S. healthcare system with a focus on primary care capacity to meet evolving patient needs driven by demographic shifts, chronic disease burden, and emerging innovations in care delivery. Quantified national and state-level capacity across major disease areas, including brain health, metabolic disease, and age-related conditions. Projected	

West Health - National primary care capacity transformation model

the system's ability to diagnose, treat, and manage these conditions within primary care settings over the coming decades. Developed a comprehensive market and financial analytic framework, integrating national and state-level data related to workforce supply, patient demand, utilization trends, payer mix, and cost projections. Conducted evidence-based benchmarking across comparable population health models, identifying current gaps relative to established quality, access, and sustainability standards. Developed transformation scenario models to evaluate the impact of policy interventions and delivery system reforms. Assessed how innovations such as expanded use of primary care extenders, virtual care, value-based payment models, and curative therapeutics could alter system capacity, reduce cost trends, and improve access to care. Future phases of support (slated for December 2025 – January 2026) will include development of public-facing platform to inform policy leaders, payers, health systems, and advocacy groups on strategic decisions related to capacity planning and healthcare transformation.

Impact of our work

- Developed the first dynamic, interactive model to project U.S. and state-level primary care system capacity over 5-, 10-, and 25-year horizons
- Quantified bottlenecks in capacity across workforce, care delivery models, and disease-specific service lines, enabling stakeholders to identify where strategic investment is needed
- Modeled the financial and utilization impact of transformation levers such as workforce expansion, extender models, and new therapeutics
- Provided evidence-based benchmarks comparing current primary care capabilities against projected demand under multiple population aging and chronic disease prevalence scenarios
- Produced a scalable analytic framework to evaluate the impact of health system innovation on cost, access, and system readiness

Example 9:

California Department of Health Care Services – Statewide behavioral health analytics and access expansion

Dates of engagement January 2022 – June 2025

Relevance

Focused on designing and implementing statewide, multi-stakeholder healthcare transformation initiatives supported by data, predictive modeling, and performance-based accountability structures. Required end-to-end operating model design, multi-sector system integration, and the development of evidence-based frameworks to support sustainable reimbursement, program scalability, and data transparency. Initiative was conducted under a legislatively directed mandate requiring rapid-cycle implementation, equity-centered outcomes, and measurable impacts on population health and service access.

Description

Supported DHCS to design and implement major components of California's multi-billion-dollar CYBHI, a statewide effort to expand access to behavioral health services for children, youth, and families. Scope involved support across three of CYBHI's twenty initiatives. Created and launched two free, virtual behavioral health applications by setting the program vision, scanning ~450 apps to define system requirements, co-designing with 500+ users, and instituting performance-based contracts that tied vendor payments to engagement. Products (including traditional, digital, telehealth, and community-oriented services) went from vision to launch in under two years. Partnered with the State to design and roll out a first-of-its-kind, multi-payer fee schedule that sustainably reimburses school-linked behavioral health services. Work included defining benefit scope and provider eligibility, coordinating with TPAs for network and claims, and forecasting reimbursement scenarios to guide adoption and budgeting, improving affordability, quality, patient experience, and healthcare access. Helped stand up and scale a \$380M+ grant program for evidence-based and community-defined practices by modernizing RFI and review processes, introducing standardized scoring and batch uploads, and building tools for scenario planning to optimize equitable fund distribution. Coordinated

California Department of Health Care Services – Statewide behavioral health analytics and access expansion

extensive stakeholder engagement across pillars (125+ sessions across 10+ formats), established dashboards and targets for ongoing performance management, and supported organizational governance and capability building to sustain operations post-launch.

Impact of our work

- Launched two statewide BH apps that reached >100,000 users in year 1, with 4.9 and 4.3 App Store ratings
- Designed and initiated the CYBHI Fee Schedule (Jan 2024 launch; broader opt-in planned Jan 2025), enrolling >600 LEAs/IHEs and impacting >3.6M students
- Designed and scaled >\$380M in grants for EBPs/CDEPs, plus ~\$400M in school-based grants tied to the fee schedule
- Cut grant application review/award time by ~50% through workflow redesign and standardized scoring
- Improved equity in grant distributions by 150% for the lowest-SVI quartile counties via scenario-planning tools
- Established performance dashboards, governance, and capability-building programs to sustain operations and promote continuous improvement statewide

Example 10:

Ohio Department of Aging – PACE program, quality, and service expansion

Dates of engagement | April 2024 - September 2024

Relevance

Focused on transforming long-term care delivery through regionalized service models, quality improvement infrastructure, data-driven planning, and system sustainability, improving affordability, quality, and patient experience. Required scenario modeling, redesign of waiver services, alignment to federal requirements, and statewide deployment strategies.

Description

Worked with the Ohio Department of Aging, in partnership with the Ohio Medicaid program, to position Ohio as the premier state for aging in the United States, with a focus on improving quality across nursing homes, PACE, ALFs and other waivers. Developed the “North Star” for PACE, waivers, ALF and nursing homes, emphasizing key priorities: expanding access, improving quality of life, enhancing quality of care, safeguarding resident’s rights, and optimizing system performance.

On PACE and 1915c waivers, conducted a comprehensive current state assessment, mapping out current services and capabilities and benchmarking them to best-in-class practices across the nation, identifying opportunities for improvement for Ohio. For the 1915c waiver, fully mapped services to enable more person-centered care service design and eligibility, and modernization of supporting waiver operations such as certification, contracting and billing. For PACE, designed 20+ interventions with PACE organizations including QoL assessments, integrated BH, HRSN coordination, etc. and planned for the expansion of PACE to additional counties. Across both PACE and waivers, developed a continuous improvement framework, anchored on key outcome measures and brought to life through a dashboard that enables better state decision making and intervention.

Impact of our work

- Created a service design playbook for 1915c waivers to enable person-centered care and modernized service delivery
- Identified operational efficiencies and modernization strategies to improve financial stewardship and administrative performance, improving affordability, quality, patient experience
- Designed a technical assistance program to support PACE organizations in implementing quality improvement interventions
- Built a statewide dashboard to support real-time monitoring of PACE program expansion, quality performance, and strategic outcomes

Example 11:

Ohio Department of Aging – Statewide nursing home quality and capacity optimization	
Dates of engagement	October 2023 – September 2024
Relevance	
Required statewide system redesign, multi-agency coordination, and data-driven frameworks to advance affordability, quality, patient experience and strengthen healthcare access, network, service expansion or optimization, with targeted healthcare capacity and workforce planning to support implementation.	
Description	
Supported the Ohio Department of Aging, in collaboration with the State’s Medicaid Program, to transform the quality, experience, and sustainability of the State’s certified nursing homes. Supported development of a statewide “North Star” vision to define what high-quality, person-centered nursing home care should look like, grounded in resident dignity, clinical excellence, and system accountability, improving affordability, quality, patient experience. Conducted a comprehensive current-state analysis, synthesizing insights from over 100 stakeholders including agency leadership, nursing facility administrators, ombudsmen, frontline staff, and residents. Analysis incorporated over 20 data sources to identify performance variation and system challenges related to healthcare access, network, service expansion or optimization. Facilitated the Governor’s Nursing Home Quality and Accountability Task Force to refine the strategic direction and prioritize interventions. Led an extensive resident engagement effort, hosting eleven in-person forums across the State and providing virtual channels to ensure resident voices guided policy decisions. Helped design and supported the implementation of key transformation initiatives focused on strengthening quality reporting, launching a resident-facing navigator tool, building a technical assistance infrastructure, and enhancing workforce capacity and ombudsman services to protect resident rights and improve outcomes.	
Impact of our work	
- Established the statewide quality standard that is now driving accountability and funding decisions affecting 80,000+ residents - Launched a resident-facing Nursing Home Navigator, improving transparency and consumer choice - Designed and supported rollout of a statewide technical assistance program to improve performance of rural and low-performing facilities for healthcare capacity and/or workforce planning - Supported enhancement of ombudsman capacity to enforce resident rights - Developed a workforce strategy to address staffing shortages and improve care outcomes - Delivered a final report with 23+ recommendations to the Nursing Home Quality and Accountability Task Force	

Example 12:

New York State Medicaid - Social Care Network program and services design and launch	
Dates of engagement	May 2024 - October 2025
Relevance	
Designed and launched a statewide Health-Related Social Needs (HRSN) benefit under the CMS 1115 waiver, which established new delivery, payment, and governance models for integrating clinical and social care. Supported transformation planning, modeled population needs using claims and social determinants of health data, designed payment incentives to sustain new service lines, and established performance management structures to monitor implementation.	
Description	

New York State Medicaid - Social Care Network program and services design and launch

Partnered with the New York State Medicaid agency to support the design and implementation of a transformative new Health-Related Social Needs (HRSN) benefit through the State's 1115 waiver. Defined the statewide operating model for the HRSN benefit, including eligibility criteria, screening protocols, covered services, and benefit structure. Supported State leadership in designing the procurement strategy for Social Care Networks, regional entities responsible for building networks of community-based organizations (CBOs), managing payments, and coordinating delivery of services. Developed the RFP that was used to select these entities and supported decision-making on program design, governance, and compliance with CMS guidelines. Analyzed Medicaid claims data, hospital utilization trends, and regional population needs to inform network design and ensure geographic equity. Designed the payment model, including fee schedules, prospective payment processes, and reconciliation methodologies to support financial sustainability and accountability for outcomes. Developed the governance and performance framework that the State now uses to oversee Social Care Networks and ensure accountability for member access, provider participation, and quality outcomes.

Impact of our work

- Supported the State's successful launch of the HRSN benefit in January 2025
- Delivered full program design including eligibility criteria, covered services, benefit operations manual, payment model, and fee schedules
- Conducted readiness assessments of Social Care Network awardees to evaluate capabilities and launch preparedness
- Enabled successful integration of HRSN technology platforms with the State's Medicaid Data Warehouse
- Established governance and performance management structures to drive data-informed decision-making with state leadership and network entities
- Facilitated coordination between state agencies, hospitals, managed care plans, and community-based organizations to support equitable access and system-wide adoption

Example 13:

Virginia Office of the Secretary of Health and Human Resources – Statewide behavioral health access and service transformation

Dates of engagement November 2022 – April 2023

Relevance

Designed and launched a data-driven behavioral health transformation strategy focused on access and system sustainability, aligned with federal requirements. Evaluated cross-agency data, assessed capacity constraints, modeled cost and workforce implications, and supported the development of a multi-year roadmap to guide investment and policy decisions across crisis services, substance use disorder treatment, and long-term support for individuals with developmental disabilities.

Description

Partnered with Virginia's Office of the Secretary of Health and Human Resources to create a comprehensive transformation strategy that would ensure timely access to behavioral health and developmental disability services, address workforce shortages, and improve financial sustainability across Medicaid-funded programs. Conducted rapid diagnostic of the behavioral health system, integrating data across Medicaid, state-operated facilities, community behavioral health providers, and waiver programs to assess gaps in crisis services, community-based capacity, and funding models. Developed a three-year behavioral health transformation plan informed by national benchmarks and evidence-based care models, focused on six pillars including crisis care, substance use disorder services, community capacity, workforce stabilization, and innovative care delivery approaches. Facilitated engagement across six state agencies and more than seventy leaders to align on strategic priorities, governance, and implementation pathways. Developed near-term implementation plans to reduce the intellectual and developmental disability waiver waitlist, expand value-based payments for behavioral health providers, and strengthen accountability and performance management for managed

Virginia Office of the Secretary of Health and Human Resources – Statewide behavioral health access and service transformation

care organizations. Supported policy design, cost estimation, and implementation planning to secure legislative and budgetary support for system-wide transformation.

Impact of our work

- Developed and launched the Commonwealth’s Right Help, Right Now strategic plan for behavioral health transformation
- Supported approval of a \$230 million investment in fiscal year 2023 to expand behavioral health services and crisis infrastructure
- Created detailed implementation plans for more than 130 initiatives within 60 days to accelerate system change
- Enabled a twofold increase in mobile crisis team capacity and a threefold increase in crisis receiving centers
- Informed the Governor’s fiscal year 2024 budget request of \$500 million for continued behavioral health investment
- Advanced policy and operational actions to reduce the developmental disabilities services waitlist and increase uptake of value-based care models among behavioral health providers

Example 14:

Kansas Department of Aging and Disability Services – Psychiatric care system access and service transformation

Dates of engagement May 2024 - June 2025

Relevance

Conducted a comprehensive environmental analysis and transformation design of the State’s Psychiatric Residential Treatment Facility (PRTF) system through market and financial analysis, evidence-based benchmarking across states, and transformation scenario modeling to improve access, ensure system sustainability, and optimize funding mechanisms. Focused on identifying capacity gaps, analyzing provider economics, and developing plans to reduce waitlists, address specialized care needs, and generate downstream fiscal savings.

Description

Partnered with KDADS to assess the full continuum of psychiatric residential treatment services for youth in Kansas in response to increasing waitlists, rising demand for high-acuity behavioral health services, and challenges in providing timely access to clinically appropriate care. Conducted quantitative analysis of current capacity, utilization trends, and demand forecasts, supplemented by more than fifty stakeholder interviews across PRTF operators, behavioral health providers, child welfare agencies, and Medicaid leadership. Benchmarked Kansas’ system against five peer states to evaluate performance on access, outcomes, and sustainability. Identified key barriers driving insufficient access, including limited provider capacity for youth with specialized needs, workforce shortages, and reimbursement misalignment. Developed a set of prioritized transformation options to reduce the PRTF waitlist by more than ninety percent, enhance clinical appropriateness of placements, and improve long-term sustainability of the system. Each option included detailed financial modeling to estimate implementation costs, funding pathways, and expected downstream savings associated with improved access and reduced utilization of emergency and inpatient care. Recommendations formed the basis for a statewide PRTF strategy focused on expanding access, strengthening the behavioral health continuum, and ensuring financial viability for providers delivering specialized levels of care.

Impact of our work

- Identified transformation pathways projected to reduce the PRTF waitlist by more than ninety percent
- Developed detailed financial estimates for each option, quantifying fiscal impacts and downstream savings from improved access and reduced readmissions

Kansas Department of Aging and Disability Services – Psychiatric care system access and service transformation

- Built a prioritized set of feasible, stakeholder-aligned solutions tailored to youth with specialized needs, including dual I/DD, SUD, complex medical conditions, aggressive behaviors, and eating disorders
- Conducted extensive stakeholder engagement through interviews with more than fifty leaders across Kansas and peer states
- Provided actionable recommendations to strengthen the behavioral health continuum and ensure clinically appropriate, sustainable service delivery for youth in need of PRTF-level care

Example 15:

Oklahoma Department of Mental Health and Substance Abuse Services – Behavioral health transformation analytics and diagnostic

Dates of engagement November 2023 – March 2024

Relevance

Developed a comprehensive analytic foundation for statewide behavioral health system transformation by analyzing specialized population needs, quantifying behavioral health demand at the county level, benchmarking system performance, and identifying areas where access, quality, and financial sustainability required targeted interventions.

Description

Partnered with the Oklahoma Department of Mental Health and Substance Abuse Services to conduct a statewide diagnostic of behavioral health system performance, capacity, and population needs. The State sought to establish a data-backed understanding of behavioral health utilization trends, service gaps, and performance variation across regions to inform future system planning, resource allocation, and accountability efforts. Built a comprehensive fact base using more than twenty million claims over a four-year period, integrating Medicaid, state-funded services, and behavioral health program data to evaluate who was being served, where gaps existed, and what populations had unmet needs. Identified population segments by condition type, severity, and demographic profile to determine where current service delivery models were insufficient to meet behavioral health demand. Identified counties with the lowest access to behavioral healthcare relative to population need through geospatial analysis, helping State leadership prioritize investments and policy actions. Assessed performance variation across different behavioral health conditions, highlighting areas of strong outcomes and areas where critical service gaps remained. Diagnostic established the State's first shared analytic baseline across agencies, enabling informed decision-making and laying the groundwork for long-term transformation of the behavioral health system.

Impact of our work

- Established a shared, data-driven understanding of behavioral health system performance across agencies
- Quantified that the State currently serves an average of 52 percent of individuals with a behavioral health need
- Identified significant variation in access across counties ranging from fewer than 25 percent of needs met to more than 90 percent
- Highlighted high performance in treatment for serious mental illness and major gaps in substance use disorder services
- Enabled State leadership to target investments and interventions toward populations and regions with the highest unmet needs

Example 16:

Indiana Family and Social Services Administration – LTSS Data Surveillance Plan Design and Implementation	
Dates of engagement	January 2022 - July 2022
Relevance	
Partnered with the Indiana Family and Social Services Administration (FSSA) to design and implement a comprehensive data surveillance and analytics framework to support the State's Long-Term Services and Supports (LTSS) reform. Integrated claims, assessments, and authorization data at scale; engaged stakeholders across agencies; built dynamic dashboards; modeled performance across quality, cost, access, and sustainability.	
Description	
Partnered with Indiana FSSA as the State prepared to launch its managed Long-Term Services and Supports program to improve quality, reduce institutional reliance, and support individuals in the least restrictive care setting. Indiana aimed to build a data-driven monitoring system that could measure progress, assess program performance in real time, and enable leadership to make evidence-based decisions throughout implementation. Worked with stakeholders across Medicaid, IT, aging and disability services, and analytics teams to develop a comprehensive surveillance plan consisting of key performance metrics, data integration requirements, analytic methodologies, and reporting structures to support LTSS reform. Facilitated alignment among more than thirty stakeholders representing different data systems and policy domains, ensuring shared definitions, consistent measurement approaches, and implementation readiness. Developed executive-level dashboards featuring high-level performance indicators and operational dashboards including over sixty metrics to monitor day-to-day program outcomes. Dashboards were built using an enterprise data model that integrated over three hundred million claims, twenty-four million prior authorizations, and three million health assessments to support analysis by population segment, geography, and program type. Enabled Indiana to track LTSS outcomes across quality, cost, choice, and managed care effectiveness, and to make decisions based on real-time performance data.	
Impact of our work	
- Designed and implemented a statewide LTSS data surveillance plan to guide Indiana's LTSS reform and transition to managed LTSS - Created a comprehensive LTSS metric library with more than three hundred metrics across eight domains including quality, cost, choice, and managed care performance - Integrated more than three hundred million claims, twenty-four million prior authorizations, and three million health assessments to power dashboards and analytic tools - Developed an executive dashboard with ten strategic performance indicators and an operational dashboard with more than sixty metrics to monitor implementation in real time - Aligned over thirty cross-agency stakeholders on shared definitions, data requirements, and transformation goals to ensure successful implementation	

Example 17:

New Jersey Department of Health – COVID-19 response and public health data modernization	
Dates of engagement	November 2020 – July 2023
Relevance	
Built and operationalized a statewide transformation model grounded in analytics, care system redesign, and cross-agency coordination. Rapidly integrated multi-sector data, established enterprise governance, built predictive analytics capabilities, and transformed public health operations across Medicaid, hospitals, primary care, community services, and digital and telehealth delivery models.	
Description	

New Jersey Department of Health – COVID-19 response and public health data modernization

In response to the COVID-19 pandemic, we partnered with the New Jersey Department of Health to design and implement an enterprise public health operating model that ensured rapid, adaptable, and equitable access to lifesaving services. Work centered on establishing a centralized data and analytics backbone to enable real-time tracking of hospital capacity, testing utilization, vaccination rates, community transmission, and health equity metrics. Built the State's predictive analytics infrastructure using a secure cloud-based data platform and integrated data from federal, state, and local sources to establish a single source of truth for public health and hospital response operations. Collaborated with State leadership to design the future of public health data modernization including standing up a Centralized Data and Analytics Center of Excellence to support long-term decision making, developing governance structures for cross-agency collaboration, and building the technical infrastructure required to support care models across hospital, primary care, behavioral health, long-term care, and community-based organizations. Enabled the State to transition from crisis response to long-term transformation, using analytics to support chronic disease prevention, maternal health, rural access planning, and equity-focused system redesign. Aligned more than fifteen divisions, coordinated across state and local agencies, and supported capability building to ensure sustainability and ownership within the Department of Health.

Impact of our work

- Enabled real-time decision-making through analysis of more than 220 metrics reviewed daily by the Governor's Office and Commissioner of Health
- Supported record-time establishment of a statewide vaccination and testing network spanning 2,000 sites, including mega-sites and targeted community-based locations
- Maintained more than 40 dashboards to support ongoing public health use cases beyond COVID-19, including maternal health, chronic disease, and lead exposure
- Reduced time from data to action by approximately 90 percent through automation and streamlined analytics workflows
- Established the public health analytics operating model that continues to guide modernization efforts across Medicaid, hospitals, emergency preparedness, and population health

Example 18:

Massachusetts Executive Office of Health and Human Services – COVID-19 long-term care response and system stabilization

Dates of engagement November 2020 - December 2022

Relevance

Designed and operationalized a statewide, analytics-enabled vaccine distribution model through integration of geospatial access modeling, demand forecasting, provider capacity analytics, rural access planning, and supply distribution optimization. Used predictive analytics to allocate scarce resources, ensure equitable access across regions, and redesign care delivery pathways that leveraged traditional providers, hospitals, mobile units, and community-based settings.

Description

Partnered with Massachusetts EOHHS to design the statewide COVID-19 vaccination strategy and distribution model, focusing on ensuring rapid, equitable, and data-driven access to vaccines. Built the geospatial and demand forecasting infrastructure that enabled the State to determine where vaccines should be allocated across hospitals, community health centers, mobile clinics, and pop-up sites. Integrated population data, case trends, and social vulnerability indices to identify priority geographies and at-risk populations, including rural areas with limited provider access. Partnered with the National Guard and the Department of Public Health to establish mass vaccination sites, deploy mobile vaccination units, and scale capacity across pharmacies and community-based organizations. Analytics platform enabled real-time tracking of vaccine throughput, second dose adherence, inventory levels, and provider readiness. Infrastructure supported supply-to-demand matching, ensuring no site was oversupplied while protecting against access gaps in underserved regions.

Massachusetts Executive Office of Health and Human Services – COVID-19 long-term care response and system stabilization

Impact of our work

- Maintained an advanced analytics dashboard that enabled daily tracking of infection rates, mortality data, and staffing capacity across more than 300 facilities
- Supported the rapid distribution of vaccines statewide

Example 19:

Nebraska CyncHealth and Department of Health and Human Services – Digital Health Hub and immunization platform development

Dates of engagement August 2021 – August 2023

Relevance

Designed and implemented a statewide digital health platform that enabled secure, real-time access to patient health records and immunization data. Platform served as a foundational tool for hospital transformation by improving access to real-time data across care settings including hospitals, primary care, specialists, payers, and public health agencies. Platform supported preventive care, chronic disease management, and improved patient experience while reducing avoidable utilization.

Description

Partnered with CyncHealth and the Nebraska Department of Health and Human Services to build and launch a statewide digital patient portal and immunization records hub that would improve care coordination, promote interoperability, and provide residents with secure, user-friendly access to their longitudinal health data. Assessed CyncHealth's existing technology, data assets, and governance structures, and developed a comprehensive data strategy and master data management approach. Led the full lifecycle implementation of the platform, starting with concept design and evolving through application development, data migration, system integration, launch, and user adoption. Platform was designed using Fast Healthcare Interoperability Resources (FHIR) standards to ensure compatibility with state and federal systems, laboratories, and payer databases. Built cloud-based data pipelines capable of ingesting twenty terabytes of immunization and clinical history spanning forty years and more than two million residents. Stood up an enterprise data warehouse, implemented data quality processes, and built advanced analytics features to support public health and hospital decision-making. Trained client teams on ongoing operations and governance and established agile delivery models with cross-functional squads to support capability building. Solution improved clinical data access and provided a scalable foundation that could be expanded to support additional health data domains, predictive analytics, and statewide transformation initiatives.

Impact of our work

- Deployed a production-ready, statewide web-based platform enabling more than 1,000 users across hospitals, providers, payers, and public health agencies to securely retrieve immunization and health records in real time
- Integrated twenty terabytes of health data across two million residents using interoperable APIs and cloud infrastructure to support future analytics and transformation use cases
- Developed a flexible, extensible data architecture to support population health, preventative care, and care coordination initiatives
- Improved data quality and reduced unnecessary utilization by enabling providers to access up-to-date immunization and health histories, reducing redundant tests and hospital readmissions
- Established an agile operating model and trained State teams to maintain and evolve the platform, supporting long-term sustainability and alignment with statewide health transformation goals

2.4 Technical Proposal of Workplan

Overall, our delivery of this engagement will be powered by our *Healthcare Capacity Planning and Optimization platform*, described in detail in **Section 2.3.2**. In summary, this platform will allow us to leverage integrated national, proprietary, and Vermont-specific datasets to enable predictive modeling, real-time scenario testing, and tailored solutions to measure quality and to optimize access, affordability, and sustainability across community, regional, and statewide levels. Supported by the secure, HIPAA-compliant Nebula platform and guided by McKinsey's unmatched expertise from hundreds of healthcare transformations in the US and globally, this approach ensures rigorous data governance, practical insights, and the translation of analytics into actionable strategies for Vermont's unique healthcare ecosystem within a targeted 12 weeks of project initiation and receipt of relevant data. The State would continue to have access to the data and outputs through the full 12-month period, with McKinsey being available to answer any questions about the deliverables.

Using this platform allows us to deliver across all 6 deliverables in 12 weeks. Please note that the work in the timelines listed below occurs concurrently to ensure rapid delivery and optimal utilization of resources. Specifically, we will:

- **Deliverable 1 - Data Integration & Analysis Plan** (~4 weeks upfront, beginning upon receipt of relevant data; ~3 weeks during final stage of the project for sustainability plan development): We will build a unified and analytics-ready data foundation using McKinsey's Nebula platform and proprietary Healthcare Capacity Planning and Optimization engine – integrating relevant payer, provider, and hospital datasets into a single source of truth to power statewide modeling and long-term sustainability, focusing on known and available metrics (e.g., STAR ratings)
- **Deliverable 2 - Hospital & Regional Access Modeling** (~4 weeks): We will leverage advanced geospatial, dynamic, AI-backed utilization and capacity analytics to model how Vermonters access care today and how those patterns will evolve, integrating rural realities and future demand drivers to guide targeted health system planning, focused on Tier 1 services (primary care, emergency care, maternal and newborn services for all, regionalized EMS)
- **Deliverable 3 - Market & Financial Analysis** (~3 weeks): We will develop a comprehensive, data-backed view of key dimensions of Vermont hospitals' financial and operational health, using McKinsey's experience supporting hundreds of hospital transformations, turnarounds, and workforce redesigns, and our benchmarking frameworks to pinpoint performance drivers and model sustainable workforce implications under different transformation scenarios
- **Deliverable 4 - Evidence-Based Benchmarking Evaluation** (~2 weeks): We will embed national, state-specific, and service-line specific benchmarks across access, quality, and workforce standards (and how they may evolve), ensuring all analyses and future scenarios are transparent, defensible, and aligned with leading national practices
- **Deliverable 5 - Transformation Scenario Model** (~3-4 weeks): We will use our proprietary, AI-backed Healthcare Capacity Planning and Optimization engine to simulate multiple, dynamic "ideal state" configurations of Vermont's healthcare system that address financial, access, and quality dimensions and incorporate innovative care models such as telemedicine, hospital-at-home, and AI-enabled care coordination over the next 5 years. This will allow Vermont to develop concrete insights to inform future planning based on how the health system needs to evolve to meet patient and community needs in a sustainably
- **Deliverable 6 - Final Regional and Hospital Transformation Analytics Report** (~4 weeks): We will synthesize findings into a comprehensive, visually compelling analytics and

recommendations report, outlining prioritized transformation initiatives and a clear roadmap to achieve measurable, sustainable outcomes

Together, these deliverables will give Vermont a trusted, data-rich foundation and actionable plan to achieve its access goals, measurable improvements in care and affordability statewide, and financial sustainability of its healthcare ecosystem.

Currently, we anticipate performing the entire scope of work as the sole prime contractor and do not anticipate the use of subcontractors. If required, we can onboard subcontractors if there is a determined need by the State.

2.4.1 Deliverable #1: Data Integration & Analysis Plan

We will build a secure, comprehensive, and analytics-ready data foundation that unifies McKinsey's data assets (as needed) with Vermont's healthcare data across payers, providers, and system.

What differentiates our approach:

- **Our Nebula platform and proprietary data model**, which enables us to rapidly ingest, harmonize, and enrich claims, clinical, and third-party datasets to create an integrated, deidentified dataset to power all downstream analyses and scenario modeling
- **Our powerful Healthcare Capacity Planning and Optimization analytics engine**, which creates a single source of truth on current demand, supply, and performance of current providers. This will be the basis for the other Deliverables in this scope (e.g., predictive forecasting future demand and supply, accounting for care model and technology innovations, identifying gaps and opportunities to expand or repurpose capacity)
- **Our data assets**, which cover over 200 million patients across all major US payer segments, supplement Vermont's data as well as provide a basis for models already built by McKinsey, including workforce projections and population demographic changes.

Details of activities:

Data Ingestion and Harmonization (~3-4 weeks):

- We will ingest State-provided All-Payer Claims Database (APCD) claims data, clinical data, hospital reports, and other relevant datasets made available to us via secure file transfer protocol (SFTP) at the outset of the project and load them onto the Nebula platform that securely enables downstream harmonization and transformation
- We will then use our proprietary data model and established data transformation system to convert these datasets into a standardized data structure, accounting for de-duplication, data reconciliation, and data conversion where necessary, as well as PHI/PII encryption to ensure compliance and quality
- We will integrate the transformed dataset with any of our licensed third-party claims and non-claims data sources to create a comprehensive and consistent foundation for analysis, including but not limited to Medicare FFS and MA data via VRDC and Medicaid FFS and MCO data via VRDC (see **Section 2.3** for more detail on datasets)

Creation of an Integrated Analytic Dataset (~1.5 weeks):

- We will create a dedicated and integrated dataset—between the State's data and our multiple third-party data sources, with a focus focused on Tier 1 services (primary care, emergency care, maternal and newborn services for all, and regionalized EMS)
- We will build this dataset at the member and claim-level, with deidentified member information so that it serves as the dynamic backbone for our Healthcare Capacity Planning

and Optimization analytical engine. This will enable provider performance assessments, demand forecasting, and healthcare capacity planning in the other Deliverables

- We will ensure that while the integrated dataset will support analytics at the member or claim level, reporting can be provided at the aggregated levels, such as service line, line of business, geographic units (e.g., county, zip, census tracts, region), and service locations

Alignment on Standards and Benchmarks (~1.5 weeks):

- We will incorporate industry standards, national benchmarks, and regulatory requirements into our modeling, which will enable evaluation of current provider performance and enable simulations of future healthcare requirements based on defined scenarios. Standards and benchmarks include:
 - Quality performance benchmarks such as STAR ratings, Patient Safety Indicators (PSI), Prevention Quality Indicators (PQI), Leapfrog Hospital Safety Grades, and American Hospital Association (AHA) data
 - Existing network adequacy regulatory requirements of the State of Vermont; additionally, we will work with the State to incorporate any future Vermont-specific requirements that are planned for implementation or in consideration
 - Regulatory requirements from peer states and federal agencies, including CMS network adequacy standards for Medicare Advantage plans and ACA exchange plans
- This alignment will enable us in other deliverables to use these standards to assess network adequacy and provider performance in proposed scenarios against established benchmarks

Sustainability Plan Development (~4.5 weeks; ~1.5 weeks upfront, ~3 weeks during final stage of project):

- We will collaborate with the Agency of Human Services (AHS) and the Green Mountain Care Board (GMCB) to develop a sustainability plan that ensures the data models and work completed under this engagement can be carried forward. This includes delivering all analytics, methodologies, and benchmarks using a format aligned with the State's needs; for example, a compilation of static reports, including detailed spreadsheets
- We will ensure continued access and usability beyond the project's completion, for at least one year, including options to host the reports generated during this scope in either the State's technology environment or McKinsey's environment, and working sessions to transition the reports to designated teams of the State, ensuring transition of knowledge and capabilities
- Our leadership team and subject matter experts will also remain available to answer any questions about the integrated dataset after the scope of this work has been completed
- The integrated analytic dataset will also be built in a way that can be refreshed at a future time as updated data sources are made available in the future.

2.4.2 Deliverable #2: Hospital & Regional Access Modeling

Overall, we will apply advanced geospatial and utilization modeling to deliver a comprehensive, forward-looking view of healthcare access and demand across Vermont. We will then analyze how Vermonters access essential services today and how those patterns will evolve as a result of the forward-looking view.

What differentiates our approach:

- **Our understanding of the impacts of future care models, site of care shifts, and technology that is grounded in our global support of innovators across the care continuum**, including health service and technology companies, AI startups and technology players across all industries, and care model innovators in both the public and private

sectors. We have a documented catalogue of innovations and potential impacts that we can draw on as a starting point

- **Our capacity modeling that has pre-built network adequacy criteria but that can also be adapted for alternative future access objectives**, and workforce models built on clinical expertise and tested and deployed in hundreds of situations
- **Our ability to rapidly iterate multiple scenarios through our Healthcare Capacity Planning and Optimization analytics engine**, enabling testing of a wide range of scenarios and modeling sensitivities in hours, not days or weeks

Details of activities:

Modeling of Access to Essential Services and Care Delivery Patterns (~2.5 weeks):

- We will model access to Tier 1 services and care delivery patterns by leveraging national provider directory data (e.g., National Plan and Provider Enumeration System – NPPES from CMS, and national provider directory from Ribbon Health), network adequacy and access standards calibrated to Vermont's objectives, and advanced geospatial analytics capabilities in the Healthcare Capacity Planning and Optimization platform. This will enable us to develop the following standard analytical models:
 - *Seasonal Drive-Time Analyses*: We will calculate drive times for Vermont residents to access care during different seasons, capturing "worst-case" scenarios for access to care. Analyses will be stratified by essential service and county.
 - *Population Access Assessment*: Based on drive-time analysis and other geospatial information, we will evaluate the percentage of the population with access to the nearest care site, by each essential service, within a reasonable drive time and distance
 - *Care Delivery Patterns*: We will use the integrated claims dataset to analyze patients' actual travel times and site-of-choice patterns for accessing care
 - *Network Adequacy Assessment*: We will compare current access performance against national and state network adequacy standards (e.g., CMS network adequacy requirements for Medicare Advantage and ACA plans, Vermont-specific requirements), identifying geographic access gaps by essential service and county
 - *Future Utilization Modeling*: We will incorporate future utilization projections (see description below) to model anticipated changes in drive-times, population access percentages, and care delivery patterns
- We will work with the State to design, review, and align on specific factors, assumptions, and parameters to be used in these analytics, and ensure insights and analytics outputs are tailored to Vermont-specific context and strategic goals. Our tools and analytics are designed to be flexible and able to ingest customized assumptions and parameters
- We will develop these access and care delivery models for essential services in Vermont and its neighboring states (i.e., NY, MA, and NH) to provide a comprehensive view of care access for Vermonters

Modeling of Current Utilization and Predictive Future Utilization Forecast (~3 weeks):

- We will conduct a detailed assessment of current utilization for Tier 1 services delivered in Vermont hospitals, leveraging the dataset built in Deliverable #1. This analysis will include detailed breakdowns by county, each essential service, and service location.
- We will develop a future-utilization demand forecast, using our pre-built claims predictive forecasting capability. This capability uses machine-learning models that generate statistically grounded growth trajectories, and a mathematically disciplined calibration layer to translate expert and policy insight into granular, auditable adjustments, with built-in guardrails to keep projections plausible, stable, interpretable, and causally grounded in county-level micro-details

- We will also incorporate various demographic and non-demographic factors into the future forecast, including:
 - Changes in demographic and health risk composition and population shifts across Vermont
 - Baseline urban versus rural breakdowns to ensure the forecasts reflect the unique characteristics of Vermont's landscape
 - Known or expected changes to the care delivery ecosystem that may result from other initiatives or capabilities (e.g., Rural Health Transformation, virtual care, RPM)
 - Historical Utilization Trends by service line, accounting for aberrations such as post-COVID returns of utilization
 - Major trends in the shifting of services from hospital outpatient facilities to freestanding surgery centers, labs and other care settings
- We will work with the State to design and review Vermont-specific factors and assumptions as we iterate jointly on the forecast. Our Healthcare Capacity Planning and Optimization engine has been built to allow for this flexibility in a short timeframe

2.4.3 Deliverable #3: Market & Financial Analysis

We will develop a comprehensive, data-driven view of Vermont's hospitals, integrating financial, operational, and workforce analytics to identify key performance drivers and improvement opportunities. Specifically, using McKinsey's proprietary frameworks and benchmarking, we will diagnose variations across hospitals and regions on multiple dimensions – including cost efficiency, operational excellence, access, quality, and clinical performance – to support downstream scenario evaluation in Deliverable #5. In parallel, we will model staffing implications to ensure that future transformation scenarios are grounded in realistic, sustainable workforce planning.

What differentiates our approach:

What differentiates our approach is our deep experience and expertise and therefore our frameworks, approaches, and tools:

- **End-to-end hospital transformations and turnaround**, which means we not only know how challenging these can be and what it takes to turn imperatives for sustainability into comprehensive action plans, we also know the nitty-gritty of hospital financial and operational data. We understand the limits of clinical operations optimization and know how real-life hospital leadership teams react to potential changes as transformational or disruptive as capital realignments and network wide service line optimization.
- **Clinical workforce redesign**, which means we know the detailed staffing and management models required to deliver high-quality care, the best practices from industry leaders who manage workforce most effectively and have deep expertise on what future workforce innovations are likely to achieve.

Details of activities:

Analysis of Vermont hospitals (~3 weeks):

- We will build a hospital-level fact base on key dimensions and roll it up by region and statewide. For each hospital, we will track (as available):
 - Cost efficiency: Cost per adjusted discharge, salaries/wages/benefits (SWB) as % of NPSR, supplies %, drug spend %, admin/overheads %
 - Clinical operations: Observed-to-expected LOS, OR utilization, occupancy rate, ED throughput
 - Access: Appointment availability, ED boarding time

- Quality: HCAPS patient experience scores, HAC score, sepsis mortality and timely treatment compliance, vaccination rates, and other quality measures using a variety of sources (e.g., CMS Hospital Compare, Leapfrog)
- Using these measures, we will assess the contribution of essential services to hospital financial sustainability. Additionally, we will evaluate how opportunities across each category (i.e., cost efficiency, clinical operations, access, and quality) could contribute to financial resiliency in the context of potential service shifts at different facilities

Modeling of Staffing implications (~1 week):

- Additionally, we will also model the staffing implications, both as part of this deliverable and as part of the transformation scenarios, to ensure access and sustainability
- We will focus on core clinical workforce (e.g., MDs and RNs) and account for key trends, e.g., risks for future nursing pipeline (future potential waves of retirement) and rising physician shortages (projected 37% increase in physician shortages)
- We will assess workforce needs across different regions of Vermont, developing an integrated perspective on clinical workforce needs by provider type and region across transformation scenarios

2.4.4 Deliverable #4: Evidence-Based Benchmarking Evaluation

Building on the network adequacy standards established in Deliverable #1, we will partner with key State stakeholders to expand and align on a broader suite of evidence-based benchmarks and regulatory standards that will inform and strengthen scenario evaluation in Deliverable #5.

What differentiates our approach:

- **Tools pre-built with network access/adequacy regulations** across Medicare, Medicaid, and ACA plans that are also customizable, for instance to more aggressive or bespoke access objectives, e.g., improved access to maternal health services
- **Expert insights to go beyond simple regulatory compliance**, integrating insights across access, quality, workforce, and service volume from federal, State, and industry sources to ensure that Vermont's future healthcare scenarios are not only data-driven and locally grounded, but also nationally benchmarked and future-ready

Details of activities:

Incorporation of Benchmarks for Essential Service Volume and Travel Time (~2 weeks):

- We will use national standards, academic literature, and evidence-based practices to evaluate if Vermont's essential services and hospitals meet quality and access standards. This includes incorporation of:
 - Existing regulatory requirements of the State of Vermont as well as any future Vermont-specific requirements that are planned for implementation or in consideration
 - Regulatory requirements from peer states and federal agencies, including CMS network adequacy standards for Medicare Advantage plans and ACA exchange plans
- We will include metrics for service line volume and travel time, including:
 - Minimum provider and facility ratios per 1,000 beneficiaries/members
 - Maximum allowable travel distances and times
 - The percentage of the population with access to care within these thresholds
 - Any other access metrics Vermont is currently using and/or plans to implement
- We will configure and incorporate aligned benchmarks for essential service volume and travel time into the Healthcare Capacity Planning and Optimization engine to support downstream scenario simulation and evaluation

Incorporation of Benchmarks for staffing and workforce (~2 weeks):

- We will evaluate workforce shifts implied across our different scenarios, to evaluate if sufficient staffing, access, and quality standards are met across priority services. Analyses will incorporate projected clinical workforce needs by facility across provider types (i.e., MDs and RNs)
- All sources, definitions, and methodologies for the benchmarks described above will be thoroughly documented in our reports and analytics to ensure transparency and accessibility for future reference.

2.4.5 Deliverable #5: Transformation Scenario Model

Overall, we will use our Healthcare Capacity Planning and Optimization engine to run advanced optimization and geospatial analytics that solve for cost efficiency, quality and access measures, to generate quantified, evidence-based “ideal state” scenarios to meet future utilization needs. By combining state-specific data, advanced analytics, and on-the-ground transformation experience, our approach delivers a uniquely actionable roadmap for Vermont – not just identifying challenges, but charting a measurable, evidence-based path to achieving equitable, sustainable care for all Vermonters.

What differentiates our approach:

- **The pre-built Healthcare Capacity Planning and Optimization engine that, once configured, can dynamically and rapidly create scenarios in hours, not days or weeks**, and that has been tested across multiple US state and global contexts, both in the public and private sectors
- **The adaptability of our tools** that allows for customization to Vermont’s context, including its urban-rural distribution, its demographic and workforce trends, the local ways that care model and technology shifts will impact the ecosystem, and the State’s specific access, quality, affordability, and hospital sustainability objectives—and allows for fast adjustment of data inputs and assumptions, so multiple scenarios can be explored rapidly
- **Deep and broad expertise of our leaders and SMEs who have deployed these approaches** in a wide range of situations, including with governments, providers, integrated delivery networks, and payers worldwide. They will bring best practices in capacity and innovative care model innovation and possess the pattern recognition of when and how to customize approaches for new contexts, as well as the practical timing of impact of these solutions, so modeling assumptions are grounded in operational realities

Details of activities:

Assessment of impact of innovative delivery models (~3 weeks):

- We will evaluate how new care models, including hospital-at-home, telemedicine, and enhanced triage and transfer systems, can improve access, quality measures, and affordability across Vermont’s healthcare system. This will leverage our expertise on care model innovations nationally, globally, and in other markets. The assessment will consider the full care continuum, from prevention to chronic condition management, and assess how digital and virtual tools can complement in-person care and be used to promote high-quality, low-cost access to care. This includes:
 - Hospital-at-Home and Telemedicine: Estimates of which inpatient and outpatient services can safely shift to home or virtual settings. Estimates of impacts on cost, staffing, and patient outcomes, and assess technology and reimbursement readiness
 - Site of care optimization: Estimates of the volume of care that currently occurs in acute settings that could be shifted to ambulatory sites to promote access and lower total cost of care

- EMS/EMT Triage and Transfer Enhancements: Estimates of how better field triage, tele-triage, and coordinated transfer systems can reduce unnecessary emergency use, shorten transport times, and improve bed utilization
- Digital and Virtual Care Tools: Estimates of how remote monitoring, digital pharmacy, and clinical decision support to fill workforce gaps, expand rural access, and improve care coordination
- Demographically-segmented care models: Estimates of impact from demographic specific care models (e.g., diabetes management, 65+, women's health) to more effectively and efficiently address patient needs for specific cohorts and improve patient outcomes
- AI and Interoperability: Estimates of how AI-enabled analytics and stronger data-sharing infrastructure can improve scheduling, prediction, and care coordination across providers
- This assessment will feed into our scenario development process and refine the assessment of areas with excess or insufficient capacity. The underlying drivers will be categorized as factors primarily affecting supply or demand. These factors will be incorporated into the modeling to enable the development of scenarios reflecting different assumptions about the future adoption of innovative delivery models

Development of “Ideal State” Scenarios (~3 weeks):

- We will use the Healthcare Capacity Planning and Optimization engine to develop a set of “ideal state” scenarios that balance access, quality measures, affordability, and financial sustainability with different tradeoffs and prioritizations. These will be co-developed with the State. Example scenarios could include, subject to refinement:
 - Scenario that maximizes the use of existing healthcare locations and minimizes new capital investments
 - Scenario that optimizes for shorter travel distance and time for essential services and prioritizes development of new locations in underserved areas
 - Scenario that optimizes for higher performance on quality measures (based on available metrics) and prioritizes development of regional centers of excellence
- The Healthcare Capacity Planning and Optimization engine develops “ideal state” scenarios using the following approach:
 - As stated in previous deliverables, we will incorporate Vermont-specific claims and provider data, including cross-border patient flows and financial assessment of hospitals, to ensure alignment with the State's unique context.
 - Then, we will identify where supply and demand are misaligned, and test different approaches to areas with excess supply and areas with insufficient supply
 - For areas with excess supply
 - a. Apply machine-learning algorithms to identify high-performing providers that can deliver care more efficiently and affordably
 - b. Use built in network adequacy assessments and aligned network adequacy standards (from Deliverable #1 and #4), identify providers essential for maintaining access in underserved regions
 - c. Create optimized footprints that meet network adequacy requirements and optimizes for provider efficiency and quality measures, using linear regression optimization algorithm
 - d. For sites of care that may have insufficient activity across the essential services to maintain quality standards, access and financial sustainability goals, evaluate options to better align services with community needs while enhancing quality measures and sustainability

- For areas with insufficient supply
 - e. Apply advanced geospatial optimization algorithms to identify nearby facilities that may be able to expand services to meet local demand
 - f. Integrating with provider staffing and workforce capacity assessments and benchmarks (from Deliverable #3 and #4), assess existing nearby facilities to identify excess supply that may be transitioned to meet local demand
 - g. For essential services and geographic areas that may require a new site or expansion of capacity at existing sites, model optimal locations for new or expanded services to fill access gaps with minimal new investment

Evaluation of Hospital and Service-Line Proposals (~3 weeks):

- In partnership with AHS and GMCB, we will also proactively highlight alignment, gaps, and tradeoffs for each of the scenarios, including:
 - **Financial sustainability:** Using outputs from Deliverable #3, evaluate potential financial impact to hospitals if priority services are shifted
 - **Quality:** Use academic literature and peer comparisons to ensure that locations for essential services maintain sufficient volume to support quality standards
 - **Access:** Review how proposed changes affect regional access, including appointment availability and drive times
 - **Hospital and system wide costs:** Evaluate financial impact associated with transition of services and projected workforce needs
- This process will yield a clear, data-backed view of where each scenario aligns with Vermont's transformation objectives and where adjustments may be needed

Evaluation of scenarios and development of recommendations (~2.5 weeks):

- We will model how 'ideal state' scenarios and proposed essential service changes impact financial sustainability, quality measures, access, and system-wide costs, providing a comprehensive, data-driven assessment to support informed decision-making. For each scenario, using assessments and benchmarks from Deliverables #2, #3, and #4, and incorporating impact of innovative care delivery models, we will evaluate the impact of each essential service changes and care delivery innovations across key dimensions, to inform State decision-making:
 - Financial sustainability, including capital investments, transition costs, maintenance cost, etc.
 - Quality of care, comparing against national standards
 - Access to care, comparing against regulatory requirements
 - Affordability for Vermonters
- Our analysis will explicitly quantify tradeoffs between these priorities, enabling the State to balance access, quality, financial health, and affordability. Example tradeoffs are:
 - Tradeoff between reducing capital investments for potential new locations and shortening driving time to access care
 - Tradeoff between regional centers of excellence that offer higher quality of care and shorter driving time to access care in rural areas
- For each scenario, we will provide detailed insights into the benefits, risks, and synergies
- Based on this evaluation, we can quickly, dynamically iterate on scenarios with State input. Then, we will work with key stakeholders to provide actionable recommendations aligned with the State's strategic priorities, supported by a decision-making framework in alignment with the State's vision for healthcare transformation
- Final output will include detailed scenario reports, tradeoff analyses, and a framework to guide the selection of the most impactful and feasible scenarios.

2.4.6 Deliverable #6: Final Regional and Hospital Transformation Analytics Report

Building on insights and analyses from Deliverables #1–#5, we will partner closely with State stakeholders to ensure full alignment and deliver a comprehensive, transparent, and actionable suite of reports and implementation plans. Our deliverables will not only synthesize the quantitative findings, methodologies, and evidence from prior phases, but also translate analytics into clear, prioritized initiatives that guide Vermont from strategy to execution.

What differentiates our approach:

- **Our track record of supporting States on ambitious transformation programs** includes support of not only reports suitable for public transparency and policy / board presentations, but also strategic stakeholder management and communications, organizational change management, and executive coaching.

Details of activities:

Creation of analytic reports and visualization (~3 weeks):

- We will deliver a comprehensive set of detailed analytic reports and visualizations that consolidate findings from all prior analyses in a static format, ensuring continued accessibility for the State. These reports will:
 - Provide a clear and comprehensive summary of key analytics, including hospital and regional access modeling, market and financial analysis, and transformation scenarios. For each area of analysis, detailed outputs and relevant visualizations will be included to enhance clarity and usability
 - Include thorough documentation of methodologies, data sources, and benchmarks to ensure transparency and credibility
- The data will be presented in a static format that is accessible and tailored for public transparency, policy discussions, and board presentations
- Our leadership team and subject matter experts will also remain available for detailed discussion and examination of analyses and scenarios even after the scope of this work has been completed
- The Healthcare Capacity Planning and Optimization engine can store the inputs, assumptions, and scenarios, as a basis for potential future iteration and adjustment. Thus, if of interest to the State, further runs would be possible at a future time

Recommendations of initiatives (~4 weeks):

- We will deliver a comprehensive set of recommendations and transformation initiatives designed to support the State in achieving its strategic healthcare goals
- These recommendations will be phased and prioritized based on their potential impact, feasibility, and alignment with Vermont's stated directional goals, categorized into short-, medium-, and long-term initiatives to ensure a structured and manageable implementation
- Each recommendation will be supported by robust evidence from the analyses conducted, with a clear articulation of tradeoffs such as balancing affordability, access, quality measures, and financial sustainability to enable informed decision-making and communication

2.5 Capacity to Conduct Engagement

2.5.1 Our approach to staffing this engagement

We have assembled a dedicated, robust team with the depth, experience, and flexibility required to deliver all tasks and deliverables in the desired timeline. McKinsey's staffing model ensures continuity, speed, and specialized expertise across data analytics, healthcare operations, and transformation planning. **Our team-based client service approach maximizes partnership with the State of Vermont and includes a dedicated accountable leadership team, a full-time core working team, a team of highly relevant core experts, access to our full network of 25,000+ expert consultants, and additional internal functional support staff as needed.** Our integrated structure enables us to surge additional resources rapidly should Vermont request expanded analytical scope or accelerated delivery.

We will operate as one integrated team, combining McKinsey and client expertise to address both the strategic challenge and practical implementation needs of this work. Our on-the-ground team will be embedded alongside your organization while maintaining direct access to McKinsey's global network of experts and a wealth of real-world learnings from similar efforts. From the outset, we will focus on capability building, ensuring that your team is able to drive lasting impact beyond the scope of this engagement.

Our proposed team represents **at least 20+ dedicated personnel** across the full team, not including our broader expert 25,000+ expert network that is available and reach-back support from our internal functional support teams. The exact number of FTEs depends on the specific phase of the project and can vary by workstream, between 7-10 FTE across phases, inclusive of the team engagement manager, analysts, and data and analytics practitioners but not including our leadership team, core subject matter experts, or admin support.

McKinsey's proposed organizational structure is outlined in **Exhibit 6**, and our team-based client service approach is further described in the following section. Details on our proposed team's experience and qualifications are included in **Section 2.3.3** and full resumes are included in **Appendix A: Resumes**.

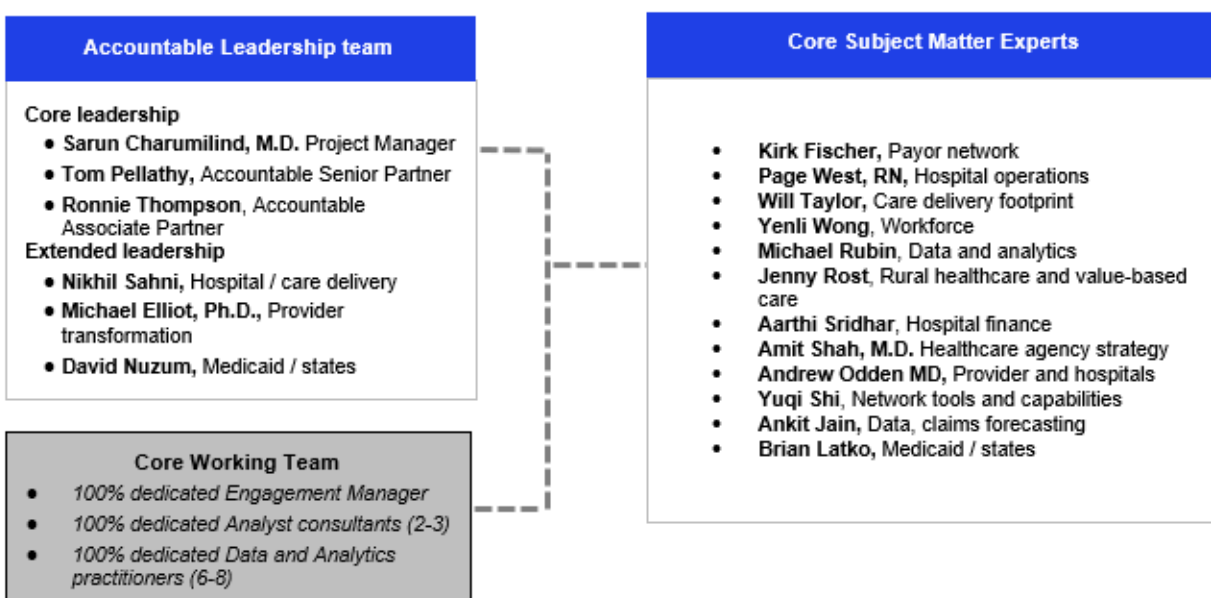


Exhibit 6: McKinsey's organizational structure

Accountable Leadership Team - Senior oversight and partnership with AHS

Our accountable leadership team, comprised of senior partners and directors who have led multiple state healthcare transformation programs, will provide overall engagement direction, quality assurance, and senior-level partnership with the Vermont Agency of Human Services (AHS) and the Green Mountain Care Board.

We have assembled a hand-selected team of experts whose collective experience directly aligns with Vermont's goals for a data-driven hospital and regional transformation. This team brings together a diverse range of perspectives, ensuring a deep understanding of every dimension of this work. Each member of our proposed leadership team was chosen for their proven ability to deliver cost-effective, compliant, and sustainable solutions in both rural and community-based settings, reflecting the State's priorities for access, quality, affordability, and long-term system sustainability. These leaders will guide analytic priorities, ensure alignment with Vermont's legislative and CMS initiatives, and provide direct insight on hospital transformation strategy, health system finance, and predictive modeling. The leadership team will meet regularly with AHS to review progress, validate findings, and resolve key decisions.

Core Leadership: Sarun Charumilind, Tom Pellathy, and Ronnie Thompson will serve as the core leadership for this effort. **Sarun will serve as Project Manager, drawing on nearly 20 years of relevant clinical, managerial, and analytic experience; Sarun will have day-to-day responsibility for managing workflow of the McKinsey team.** Tom will be the Accountable Senior Partner, bringing more than 20 years of relevant experience servicing public sector clients in the U.S. and three other continents, as well as serving healthcare providers and private payers in the design and operations of delivery systems, both as a consultant and an operating executive. Ronnie will be the Accountable Associate Partner and brings deep years of experience partnering with health systems on growth strategy and transformation, with an emphasis on site of care optimization and ambulatory strategy.

Extended leadership: Nikhil Sahni, Michael Elliott, and David Nuzum will provide additional input and oversight, based on the broader leadership responsibilities within McKinsey. Nikhil and Michael lead McKinsey's client service in Vermont in healthcare. Nikhil is the former Policy Director for the Massachusetts Health Policy Commission and currently the leader of McKinsey's Center for Healthcare Innovation. Michael leads McKinsey's Hospital Transformation practice in the US David leads McKinsey's broader client service for the State of Vermont as well as McKinsey's State & Local Government practice nationally, including client service to nearly 40 Medicaid and broader Health and Human Services agencies.

*Details on our proposed team's experience and qualifications are included in **Section 2.3.3** and full resumes are included in **Appendix A: Resumes**.*

Core Working Team - 100% dedicated, responsible for day-to-day delivery

A full-time core team will be responsible for the day-to-day execution of all project tasks, from data integration and modeling to report generation and stakeholder support. The team will be led by a dedicated Engagement Manager and include 1–2 additional full-time consultants, data scientists, and analysts throughout the engagement. These professionals will work closely with State staff to co-develop analytical products and ensure deliverables, including the Hospital and Regional Access Models, Market and Financial Analyses, and Final Transformation Analytics Report, are delivered on time and to specification. Upon award, the team will be immediately deployable, ensuring that work begins promptly to meet the State's November 5 start date.

Core Subject Matter Experts - Specialized, on-demand analytic and policy expertise

McKinsey operates as one integrated firm – a single partnership united by a strong set of values, focused on client impact. Using this “One Firm” model, we can tap into our network of over 25,000 consultants and external Senior Advisors. For this engagement, we have assembled a core set of our most relevant experts. These experts bring deep experience in:

- State Medicaid programs and health and human services (e.g., Brian Latko)
- Healthcare agency strategy (e.g., Amit Shah, M.D.)
- Hospital finances and operations (e.g., Page West, R.N., Aarthi Sridhar)
- Clinical operations (e.g., Andrew Odden, M.D.)
- Care delivery, including alternative sites of care (e.g., Ronnie Thompson)
- Payor network (e.g., Kirk Fischer)
- Care delivery footprint (e.g., Will Taylor)
- Rural healthcare and regional service optimization (e.g., Jenny Rost)
- Workforce (e.g., Yenli Wong)
- Predictive modeling, data integration, and visualization for healthcare transformation (e.g., Michael Rubin, Yuqi Shi, Ankit Jain)

Experts will participate in targeted problem-solving sessions and review deliverables to ensure that Vermont benefits from best practices across national and state health reform efforts. We can pull in additional experts to the engagement to provide specific expertise on various topics and/or to support specific work streams as needed at any time. We will dedicate framework leads for our Healthcare Capacity Planning and Optimization approach to ensure model governance, scenario reproducibility, and transparent coordination across all analytic components.

*Details on our proposed core expert’s experience and qualifications are included in **Section 2.3.3** and full resumes are included in **Appendix A: Resumes**.*

Support Staff - Administrative and technical delivery backbone

Throughout the life of the contract, we can draw upon our internal administrative, design, research, logistical, and communications support staff, including research analysts; administrative assistants; and media, business presentation, and visual graphics designers to support the project. This equates to several additional FTEs supporting the team around the clock.

Collaboration model

We will work in close partnership with the State of Vermont to ensure alignment, transparency, and momentum throughout the engagement. Our collaboration model combines structured governance with flexible working sessions to enable both strategic oversight and rapid problem-solving. A biweekly Steering Committee, composed of State and agency leaders designated by you, will guide direction, validate key decisions, and monitor progress against milestones. In parallel, we will facilitate weekly working sessions with core project teams to test hypotheses, review early insights, and address potential bottlenecks. We will also convene ad hoc working groups with hospital leaders, regional stakeholders, and other partners as needed to inform design choices and ensure solutions are practical and implementable.

Throughout the engagement, we will emphasize capability building; transferring our tools, methodologies, and analytic frameworks to the State so that Vermont can continue to generate insights, update models, and sustain transformation independently long after the project concludes.

2.5.2 Timeline to accomplish the scope of work

This page outlines a proposed project timeline structured around the six key deliverables over 12 weeks. As detailed below, the work will be sequenced based on key dependencies. Note that the dates shown here are highly preliminary and based on RFP suggested start date of Nov 5 and can be adjusted across deliverables as needed in collaboration with the State of Vermont. Note that the deliverables and sub activities match one-to-one with the key activities outlined in **Section 2.4**.

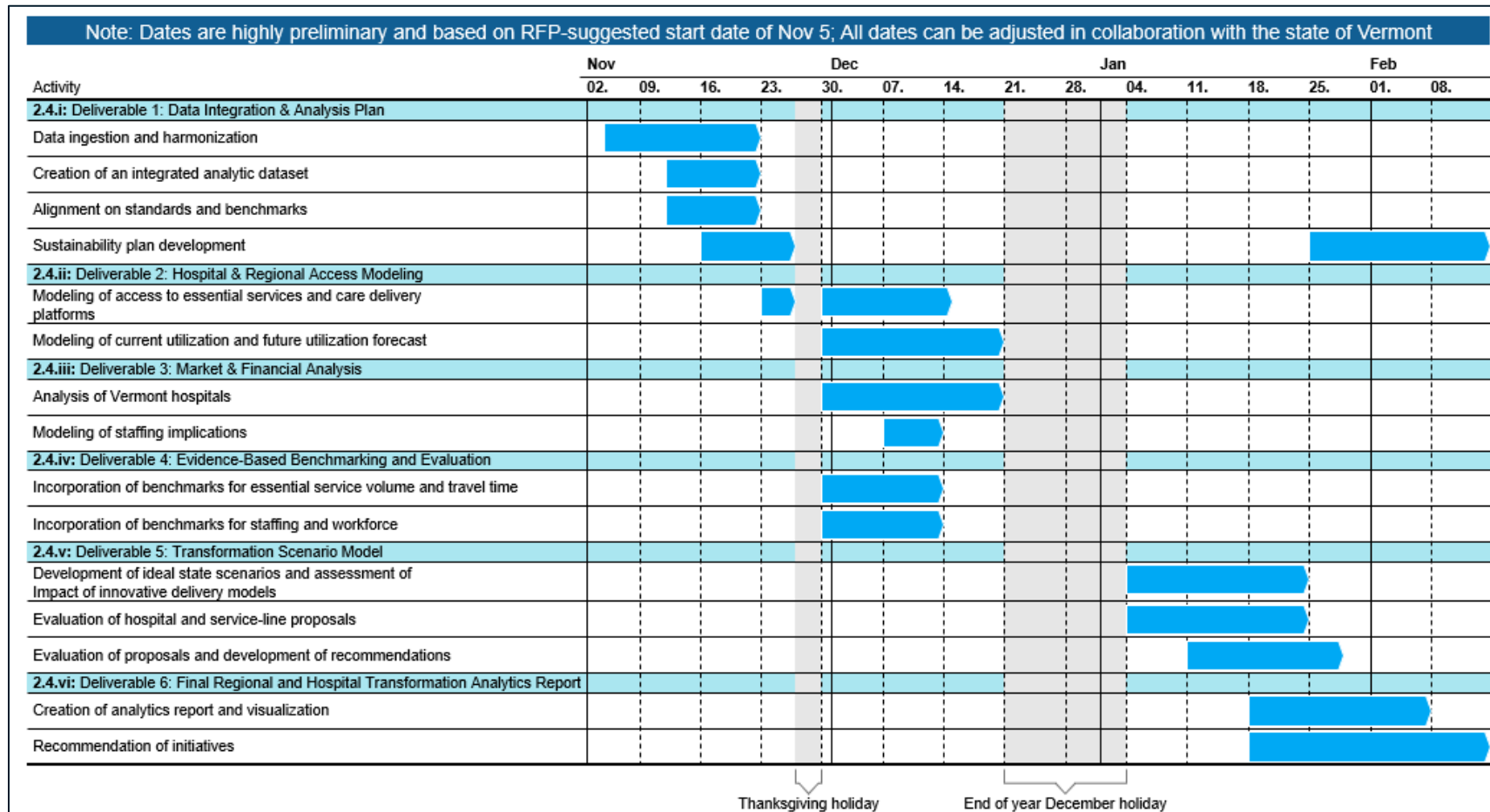


Exhibit 7: Proposed timeline to accomplish the scope of work

3.0 References (4.8)

Below we have included client references for four (4) similar engagements in the last 12 months.

#	Client Reference	Relevance to this work
1	State of New York Darcie Hurteau, Director of Alternative Payment Methodologies (518) 474-8646; darcie.hurteau@health.ny.gov One Commerce Plaza, 99 Washington Avenue, Albany, NY 12210	Regional hospital transformation, access, and value-based care planning
2	State of Ohio Meredith Philabaum, Chief of Staff, Ohio Department of Aging (800) 266-4346; mphilabaum@age.ohio.gov 30 E Broad St 22nd Floor, Columbus, OH 43215	Statewide nursing home quality and capacity optimization
3	Hawaii Medical Service Association Jay Fujimoto, Vice President Provider Network Relations (808) 927-5870; jay_fujimoto@hmsa.com 818 Ke'eaumoku St, Honolulu, HI 96814	Regional provider and network transformation analytics and strategy
4	Blue Cross Blue Shield of Michigan Todd Van Tol, Executive Vice President, Health Care Value (313) 448-0004; TVanTol@bcbsm.com 500 Renaissance Center, Detroit, MI 48243	Statewide provider network analytics, value-base care, and planning

4.0 Reporting Requirements (4.9)

Below we have provided a link to sample reporting documentation from a client engagement example with NHS. We supported NHS in the development of the report. It is relevant to this Vermont effort because it focuses on redesigning the care delivery system in Dorset to promote equitable access to care, address workforce challenges, and ensure financial sustainability in light of changing population dynamics.

Link:

<https://www.ors.org.uk/web/upload/surveys/869972/files/Dorset's%20Clinical%20Services%20Review%20-%20Consultation%20document.pdf>

5.0 Price Schedule (4.10)

PRICE SCHEDULE

A. Fixed Price Deliverables:

Deliverable description	Fixed price
Data Integration & Analysis Plan	\$400,000
Hospital & Regional Access Modeling	\$500,000
Marketing & Financial Analysis	\$400,000
Evidence-based Benchmarking Evaluation	\$200,000
Transformation Scenario Modeling	\$700,000
Final Regional and Hospital Transformation Analytics Report	\$200,000
Total Project Cost (Not to Exceed)	\$2,400,000

Name of Bidder: McKinsey & Company, Inc. Washington D.C. (McKinsey)



Signature of Bidder: David Nuzum, Senior Partner

Date: 10/28/2025

5.1 Assumptions and Disclosures

McKinsey's approach and corresponding price proposal are based on the following assumptions and disclosures.

1) Assumptions:

- In accordance with Attachment C.31, Federal Funding, McKinsey asserts its status as a contractor under 2 CFR 200, not a subrecipient.

2) Proposed Changes to Attachments C, D, and F:

Per Q&A answer 37, McKinsey respectfully proposes the following changes to Attachment C. We look forward to the potential opportunity to discuss and negotiate additional terms further and appreciate the State's consideration.

- **Attachment C.13. Records Available for Audit:** The Party shall maintain all records **directly** pertaining to ~~performance~~ **claims for payment** under this Agreement. "Records" means ~~any written or recorded~~ information, regardless of physical form or characteristics, which is produced ~~or acquired~~ by the Party in the **direct** performance of this Agreement. Records produced or acquired in a machine-readable electronic format shall be maintained in that format. The records described shall be made available at reasonable times during the period of this Agreement and for three years thereafter or for any period required by law for inspection by any authorized representatives of the State or Federal Government. If any litigation, claim, or audit is started before the expiration of the three-year period, the records shall be retained until all litigation, claims, or audit findings involving the records have been resolved.

3) Conflicts Disclosure:

McKinsey & Company, Inc. Washington D.C. ("McKinsey USG") primarily serves public sector clients, with some social sector and commercial client work. To the best of our knowledge, McKinsey USG has not identified a conflict of interest ("COI") as defined in Section 3.3.1 of Request for Proposal ("RFP") for Hospital Transformation Analytical Support for the Vermont Agency of Human Services based on our proposed scope of work, and further confirms that McKinsey USG and its employees staffed to this engagement will comply with its respective obligations as defined in Section 3.3.2 of the RFP. Further, in order to bring the best available expertise to this engagement, McKinsey's Client Service team ("CST") will be comprised of members with the industry experience and expertise the government is seeking. However, our Client Conflict and Confidentiality Policy requires all Firm members to disclose any potential COIs, including any situation in which an individual might have, or be perceived to have, divided loyalties, such as an interest that could reasonably be perceived to undermine their faithful and unbiased performance of a contract with the State that may result from this solicitation. In providing services to clients, including building each CST, we deploy staffing procedures designed to avoid, potential COIs. For additional transparency, we note that McKinsey & Company, Inc. United States ("McKinsey US"), an affiliate of McKinsey USG, serves or has served private sector clients that may provide or have provided products and/or services the State of Vermont and/or The State of Vermont Agency of Human Services on a wide range of topics. Additionally, we note that McKinsey USG and McKinsey US currently serve, or are anticipating serving, the private sector, other states and their subdivisions, and other federal

departments with respect to the Rural Health Transformation Fund. These industries include medical electronics, consumer packaged goods (including food and beverages and consumer healthcare companies), retail pharmaceuticals, healthcare systems & services (including for-profit & not-for-profit hospitals and health systems, payers, services & technology vendors, distributors, and pharmacies), pharmaceutical & medical products, medical technology, foundations, non-profits, bilateral/multilateral organizations, donors, technology (including software), private equity, and institutional investors. McKinsey US's consulting services include analysis, advice, and implementation support across many areas of business, including marketing and commercial growth (including Go-to-Market strategy and execution, sales optimization, pricing & margin management, and customer experience), pricing, operations (including digitization, analytics, automation, process improvements & efficiency, procurement, and capital projects), strategy, value creation (including assessment of capabilities and impact on customers, employees, finance and operations, and social & environmental factors), corporate finance, people and organization (including talent, operating model, and organization design, change and culture management, leadership, capability building, and merger management), management of risk (e.g., portfolio optimization, supply chain), building of resilience against non-navigable events (including crisis response), technology, transformation (including growth-oriented transformations, business building, and helping companies significantly change their enterprise value and performance trajectory by reducing costs (e.g., procurement, labor, fixed costs, digitization/automation) and/or driving growth (e.g., acquisitions, divestitures, new investments, business building, and equity story), mergers & acquisitions topics (including merger strategy, due diligence, integration planning, divestiture planning, IPOs, JVs & alliances), and sustainability.

In order to address any actual or potential conflicts of interest, Within the past 30 days, the University of Vermont Health Network Inc. ("UVMHN") has issued multiple solicitations for work to be presented to the Liaison Group, the Working Group, and the Green Mountain Care Board. McKinsey & Company, Inc. United States ("McKinsey US") has submitted, or is in the process of submitting, proposals in response to these solicitations. McKinsey & Company, Inc. Washington D.C. ("McKinsey USG") does not believe that our it's engagement with the Vermont Agency of Human Services ("AHS") on the work outlined in this proposal creates an Organizational Conflict of Interest ("OCI") with our affiliate's its work for UVMHN as reflected in these solicitations. This conclusion is based on the fact that McKinsey USG's role for AHS would be limited to providing objective data analysis to support AHS and Green Mountain Care Board ("GMCB") decisions, without influencing public procurement processes or making recommendations regarding allocation decisions.

Additionally, to address and mitigate any potential or perceived Personal Conflict of Interest ("PCOI"), McKinsey proposes the establishment of a Core Leadership Team, comprising Tom Pellathy, Sarun Charumilind, and Ronnie Thompson, which will exclusively support AHS and specifically will not support UVMHN on any work in response to the pending solicitations. This Core Leadership Team will retain oversight and discretion over all analyses and deliverables provided to AHS. Additionally, McKinsey USG has included two individuals in this proposal, Nikhil Sahni and Michael Elliott, who are also intended to support UVMHN. McKinsey believes that the expertise of Nikhil and Michael would provide significant value to AHS. To mitigate any potential PCOI, McKinsey USG will clearly delineate their roles to AHS and GMCB leadership and will adhere to AHS's guidance determination regarding the information to which Nikhil and Michael may or may not have access.

Moreover, Nikhil and Michael will be recused from any discussions in the event that where AHS requires confidential input that is entirely independent of any individual supporting UVMHN, Nikhil and Michael will be recused from such discussions. These discussions will instead be

supported exclusively by Tom, Sarun, Ronnie, and other McKinsey USG team members dedicated solely to AHS. To further safeguard the confidentiality of AHS information, the McKinsey USG team leading this engagement, under the direction of Tom, Sarun, and Ronnie, will seek documented guidance from AHS regarding the types of data and working documents that may or may not be shared with Nikhil and Michael, if any. McKinsey will implement appropriate measures to segregate such data and working documents accordingly. Under no circumstances will information related to this engagement be shared with UVMHN or any other external stakeholders without the express authorization of AHS. As a general protocol, confidential information and working documents related to this project will only be shared with UVMHN or other external stakeholders by AHS or GMCB, and not directly by McKinsey USG.

Should AHS determine that the involvement of Nikhil and Michael in this engagement creates a PCOI that cannot be adequately mitigated through the measures outlined above, then McKinsey USG would reconsider the roles of Michael and Nikhil in each of these efforts prior to the initiation of either project(s). Likewise, if AHS requires additional information regarding McKinsey USG's conflicts of interest policies, such materials can be provided upon request. McKinsey USG has structured the Core Leadership Team, subject matter experts, and the broader working team to ensure that the requisite expertise and capacity are in place to fully meet AHS's needs, regardless of any such adjustments.

McKinsey USG does not hold ownership interests in their clients, but we note that other affiliates may hold ownership interests in McKinsey US clients across various industries. Please let us know if you have any questions.

6.0 Certificate of Compliance (4.11)

McKinsey's signed certificate of compliance is included in Appendix B.

Appendix A: Resumes

Sarun Charumilind

Project Manager
Medicaid / states / network

Key expertise and experience

- Over 18 years of experience working in healthcare and over 15 years of experience
- Co-leads McKinsey's Healthcare Affordability & Outcomes team of more than 50 experts, analytics, and data science practitioners and leads the Network and Contracting Excellence service line
- Served six State Medicaid agencies and multiple Managed Medicaid and Medicare Advantage payers on affordability, access, and quality improvement
- Extensively serves public and private sector organizations on network, provider, and payment model strategy, design, and implementation
- Leads our Healthcare Capacity Planning and Optimization engine in the U.S., including its ongoing development and its deployment and customization to new client contexts



Sarun supports a broad range of organizations, including private and public payors, providers, investors, technology innovators, government agencies, and foundations. He tackles healthcare and public-health challenges through clinically driven, data-backed strategy and analytics, and through his leadership of multidisciplinary teams of generalist consultants, advanced analytics practitioners, and technologists. Examples of recent impact in provider and network management, and regional health transformation, include:

- **Network and contracting transformation**, at multiple regional and national payers, with focus on FFS and VBC contracting, broad and high-performance network design, across hospital, professional, and ancillary providers
- **Hospital ecosystem transformation to improve access, health equity, and efficiency**, for a State Medicaid program aspiring to transition safety net hospitals to sustainable global budgets, enable systematic improvements in population health, and address patients' social care needs
- **Integrated delivery system innovation**, to optimize how the provider arm and insurance arm collaborate to ensure high-quality, affordable, accessible continuity of care for patients
- **AI-enabled network design and capability-building**, to enable systematic identification of high-value providers and geospatially optimized network configuration to ensure access for all members
- **Patient and primary care provider matching algorithm and process redesign**, to connect patients with the highest-value, most accessible PCPs, also supported by an agile engagement campaign to activate patients to visit their PCPs and PCPs to reach out to new patients

In addition, during the pandemic, Sarun supported hundreds of public, social, and private sector organizations globally to integrate epidemiology and public health into organizational strategy and business planning.

Education

Institution	Degree	Year
Massachusetts General Hospital	Residency in Internal Medicine, Primary Care	2010
Harvard Medical School	M.D.	2007
London School of Hygiene and Tropical Medicine	MSc in Public Health in Developing Countries	2006
Harvard University	AB in History of Science	2000

Work history

Employer	Role	Years
McKinsey & Company	Partner	2010–Present
Massachusetts General Hospital	Attending Physician	2010–2016

Thomas Pellathy

Accountable Senior Partner Network design / Value-based care

Key expertise and experience

- 20+ years of experience working in healthcare
- Leads McKinsey's center of excellence in healthcare value, which works with healthcare organizations to improve quality and health outcomes while improving financial sustainability
- Extensive work across the US across both state-level and private sector organizations on topics of network design and management, value-based payment design, population health and models of care, digital and analytics, and operational transformation
- Globally has worked with leading publicly-funded health systems on sector-wide transformation in 15+ countries around the world, focused on clinical and population health improvement, digital and analytics transformation, and cost sustainability
- Former senior executive at Highmark Health (non-profit Blue Cross Blue Shield with an integrated health coverage and care delivery model across PA, NY, DE, and WV) with direct operational accountability for product design, quality, care and utilization management, network design and contracting, value-based reimbursement and payment policy, and digital and analytics across Highmark's \$29B diversified enterprise



Thomas Pellathy is a Senior Partner and a core leader of McKinsey's healthcare practice based in Philadelphia. Tom has more than 20 years of experience working in healthcare, as an advisor to leading healthcare organizations across both the public and private sectors and as a senior executive for one of the largest non-profit integrated health coverage and care delivery organizations. His work focuses on improving quality and health outcomes for individuals, families, and communities, while improving access, experience, and affordability of care.

Within McKinsey, Tom leads our dedicated center of excellence focused on improving healthcare value (improving health outcomes and affordability). In this role, Tom works with leading payors and providers across both the public sector (such as state Medicaid agencies) and private sector (with a focus on non-profit health insurers and providers) to design and implement concrete programs around network design, population health and new models of care, value-based reimbursement, care pathway management, and digital and analytics. This includes supporting clients by delivering robust data and analytic solutions and building capabilities to enable them to drive ongoing execution with their own teams.

In addition, Tom is also a core leader of our Rural Healthcare team, which is partnering across states on their approach to rural healthcare transformation.

Previously, while based in London, Tom was the leader of our work globally with publicly-funded health systems. In that role, he led our work with the NHS in the UK and with Ministries of Health across multiple regions around the world. He has worked extensively in 15+ countries on topics of clinical and population health improvement, digital and analytics transformation, and health sector cost sustainability.

Prior to re-joining McKinsey in 2020, Tom served for more than six years as a senior executive at Highmark Health, a \$29 billion diversified healthcare company with an integrated health coverage and care delivery model, serving members across PA, WV, DE, and NY. There he

had operational accountability for product design, quality, care and utilization management, network design and contracting, value-based reimbursement and payment policy, and digital and analytics – and led some of Highmark’s most important strategic initiatives, including the development of Highmark’s integrated payor-provider model with Allegheny Health Network, provider partnerships to create new models of collaboration with key health systems, and a redesign of Highmark’s core platform and analytics capabilities.

Select relevant experience

- **Blue Cross Blue Shield plan – Network design:** Developed comprehensive data-driven network strategy to improve quality and total cost of care. This included new forms of collaboration across the health plan and providers, including win-win collaborations with leading health systems and the design and rollout of a new model of engaging with and supporting primary care and specialists around value-based care.
- **National Health Service (NHS) in the UK – National demand and capacity planning as part of the New Hospital Program:** As part of a £20 billion national program to improve hospital infrastructure across England, developed a robust approach to assess forecasted patient demand (adjusting for future shifts in demographics and risk) against needed hospital and non-hospital capacity (accounting for future shifts in models of care, site of care shifts, changes in clinical productivity, new digital solutions, etc.). Led to concrete re-prioritization of investments to better optimize needs of community based on a network-level perspective of need provider assets and development of a repeatable capability to manage capital allocation (including development of a ‘model hospital’ of the future).
- **Leading the design and implementation of new clinical care models:** Implemented new clinical care models and redesigned pathways across primary care, behavioral health, orthopedics, diabetes management, and cancer, resulting in measurable improvements in care coordination and patient outcomes. Applied advanced predictive algorithms to clinical workflows—helping target highest risk patients and achieving >20% per-episode cost reductions while maintaining quality performance metrics.
- **Multiple publicly-funded health systems – Assessing and improving digital maturity:** Developed and deployed a robust digital maturity assessment spanning from underlying technical infrastructure to usage of solutions to drive outcomes, leading to new investments to support improvements in interoperability, analytics, and change management. Directly support multiple health systems in the build and deployment of digital and data platforms, clinical and population health analytics, and digital operating model redesigns, enabling system-wide improvements in efficiency and clinical performance.
- **Highmark Health – Multiple high-impact efforts while Executive Vice President of Product & Healthcare Services:**
 - **Clinical transformation:** Implemented portfolio of differentiated clinical model of care and population health capabilities, leveraging integrated capabilities of providers and the health plan and underpinned by advanced digital and analytic solution.
 - **Network partnerships:** Developed and negotiated advanced win-win partnerships between Highmark and leading provider systems, ranging from joint ventures to global capitated models with shared clinical, quality, and care model solutions and streamlined end-to-end operational processes.
 - **Value-based reimbursement:** Designed and launched advanced value-based reimbursement models focused on quality and total cost of care across primary care, priority specialty episodes, and health systems across Medicaid, Medicare

Advantage, and Commercial segments, including advanced risk-based models. Developed needed provider-enablement capabilities (reporting, analytics, digital, care model, etc.) to enable providers to succeed under these outcome-based models.

- **Affordability.** Created an integrated enterprise process for improving healthcare affordability while ensuring health outcomes, experience, and access, resulting in a 3-4% year-over-year improvement to total cost of care over a multi-year period and new product designs with ~10% lower premiums for customers.
- **Analytics and digital:** Developed leading analytic capabilities, including new flexible data infrastructure and leading AI solutions. Led \$200m re-design of Highmark's technology roadmap to enable future-state capabilities for product design, reimbursement, network and clinical.

Education

Institution	Degree	Year
University of Oxford	MSc, Forced Migration and DPhil studies in Economic History. Rhodes Scholar	2001–2004
University of Delaware	Master's Degree, Linguistics (Cognitive Science)	1996-2000
University of Delaware	Bachelor's Degree, Philosophy and Mathematics	1996-2000

Work history

Employer	Role	Years
McKinsey & Company	Senior Partner	2022–present
McKinsey & Company	Partner	2020–2022
Highmark Health	Executive Vice President, Product and Healthcare Services	2017–2020
Highmark Health	Senior Vice President, Product, Network & Reimbursement	2014–2017
McKinsey & Company	Partner	2005–2014

Ronnie Thompson

Accountable Associate Partner Alternative sites of care

Key expertise and experience

- Advisor to not-for-profit healthcare systems on growth strategy, transformation, and financial sustainability
- Specialist in ambulatory care delivery models and service line strategy
- Co-leader of McKinsey's Ambulatory Diversification Service Line within the Revenue Excellence Domain
- Deep experience driving site of care optimization, patient access, and financial resiliency strategies for health systems
- Focused on helping clients build integrated, sustainable care delivery ecosystems across the care continuum



Ronnie Thompson is an Associate Partner with deep experience partnering with not-for-profit and regional health systems on growth strategy and transformation, with an emphasis on site of care optimization and ambulatory strategy. She co-leads McKinsey's Ambulatory Diversification Service Line and has extensive experience shaping data-driven ambulatory strategies that increase patient access and lower total cost of care. She has led enterprise-wide growth transformations, service-line strategies, and partnership development initiatives, delivering measurable impact through improved patient access, enhanced provider alignment, and operational efficiency. Her work helps healthcare leaders reimagine where and how care is delivered – driving performance improvement, financial sustainability, and improved patient access.

Relevant experience

- **Health System – Enterprise Growth Transformation:** Led a multiyear transformation spanning physician alignment, patient access, provider productivity, and service-line strategy. Delivered 14 percent growth in acute surgical volumes and \$100 million in run-rate impact through improved access, recruitment, and continuity-of-care initiatives.
- **Regional Health System – Ambulatory and Service-Line Strategy:** Partnered with system leadership to identify approximately \$200 million in EBITDA value through enhanced care continuity, strategic service-line expansion, site of care optimization, and ambulatory growth initiatives.
- **Primary Care Group – Strategic and Operational Plan:** Developed a strategic plan focused on provider satisfaction, patient experience and access, and operational efficiency. Reduced investment per physician by ~20 percent year-over-year, outperforming 75th percentile benchmarks.
- **Health System Clinic and Ambulatory Strategy:** Led the development of an integrated clinic and ambulatory footprint strategy for a \$15 billion regional health system, focused on increasing patient access, enhancing care continuity, and lower total cost of care through operational improvements and investment in new facilities (e.g., urgent care).
- **Ambulatory Surgery Centers – Growth and Site-of-Care Strategy:** Designed and implemented an ASC growth strategy resulting in the construction of five new centers and shifting over 5,000 clinically appropriate cases to outpatient settings to free acute capacity.

Education

Institution	Degree	Year
University of Pennsylvania	M.B.A., Finance	2020
Stanford University	B.S., International Relations	2016

Work history

Employer	Role	Years
McKinsey & Company	Associate Partner	2020-present
Goldman Sachs	Financial Analyst	2016-2018

Nikhil Sahni

Extended Leadership Expert – Hospital / care delivery

Key expertise and experience

- Partner in McKinsey's Boston office with deep experience advising providers, payers, distributors, and manufacturers across the healthcare value chain.
- Leads McKinsey's Center for US Healthcare Improvement, where he drives efforts to improve productivity, reduce inefficiencies, and enhance patient outcomes.
- Previously Policy Director for Cost Trends in Massachusetts' Health Policy Commission
- Recognized thought leader and co-author of key publications including "The Productivity Imperative in US Healthcare Delivery," "Administrative Simplification: How to Save a Quarter-Trillion Dollars in US Healthcare," and "Artificial Intelligence in US Healthcare Delivery."
- Fellow with Professor David Cutler at the Harvard University Department of Economics, contributing to national healthcare productivity research.



Nikhil Sahni is a Partner in McKinsey's Boston office. He serves healthcare organizations—including providers, payers, distributors, and manufacturers—on topics spanning corporate strategy, growth, and operational efficiency. Nikhil leads McKinsey's Center for US Healthcare Improvement, where he partners with clients to reduce inefficiency, enhance affordability, and improve outcomes system-wide.

Previously, Nikhil was the Policy Director for Cost Trends in Massachusetts' Health Policy Commission created under Chapter 224. Nikhil's research and client work are widely recognized for advancing U.S. healthcare productivity. His thought leadership has helped shape national dialogue on administrative simplification, digital health innovation, and artificial intelligence applications in care delivery. He also serves on the Board of Health Care Without Harm, a global nonprofit advancing health and environmental sustainability, and on the Corporation of the Belmont Hill School.

Relevant experience

- **Large Healthcare Company – Growth Integration** - Helped identify, acquire, and integrate new business lines, enabling strategic expansion and operational synergies across the organization.
- **Specialty Hospital – 5-year Strategy** - Guided a joint Executive and Board-led process to develop 5-year strategic plan for a specialty hospital, including research and education missions
- **Healthcare Innovator – Market Entry Strategy** - Collaborated with a new business entering a specific disease area to define its strategic vision, economic model, and market positioning for long-term growth.
- **Healthcare Enterprise – Strategic Pricing Transformation** - Supported a leading healthcare company in designing and implementing new pricing strategies to align with evolving market dynamics and improve profitability.

Education

Institution	Degree	Year
Harvard University	MBA and Master's in Economics	—
University of Pennsylvania	B.S. in Finance; B.A.Sc. in Biomedical Engineering; B.A. in Indian Studies	—

Work history

Employer	Role	Years
McKinsey & Company	Partner	Feb 2016 – Present
Harvard University, Department of Economics	Fellow	Sep 2014 – Present
Kyruus	Senior Director of Strategy, Planning, and Operations	Apr 2014 – Feb 2016
Health Policy Commission, Commonwealth of Massachusetts	Policy Director for Cost Trends and Special Projects	Jan 2013 – Apr 2014
Harvard Business School	Fellow at the Forum for Growth and Innovation	Jun 2012 – Dec 2012
McKinsey & Company, Center for U.S. Health System Reform	Summer Associate Fellow	Jun 2011 – Aug 2011
National Economic Council, The White House	Summer Intern	Jun 2010 – Aug 2010
Boston Red Sox	Special Assistant to President/CEO and Chairman	Apr 2008 – Aug 2009
McKinsey & Company	Business Analyst	Aug 2006 – Mar 2008

Michael Elliott, Ph.D.

Extended Leadership Expert – Provider transformation

Key expertise and experience

- Senior Partner and leader of McKinsey's Healthcare Provider Transformation COE
- Redesigns organizations to enable them to better service their communities through high quality, cost effective, sustainable care
- Specialized in serving healthcare institutions to improve operations and transform care models
- Extensive experience across types of institutions - both not-for-profit and for profit, academic and community, acute and pre/post acute



Michael Elliott is a Senior Partner and leader of the firm's Healthcare Provider Transformation COE. He helps healthcare institutions make transformational improvements using a holistic view of the factors that drive performance. Michael works across the provider landscape in both profit and not for profit, academic and community, adult and pediatric settings to help leaders design and execute comprehensive transformation programs in both clinical and non-clinical areas that increase patient access to high-quality care.

Relevant experience

- **State – Rural Health Transformation:** Leading the application, development and planning for a state's transformation of its rural healthcare delivery network.
- **Health System – Transformations:** Led multiple turnarounds of hospital systems including academic and access facility assets. Applied a range of growth and cost-reduction strategies across physician alignment, labor, supplies, clinical operations, revenue cycle management, and managed-care pricing.
- **Health System – Technology enabled operations:** Led the development of technology roadmaps to use the latest AI tools to improve access and efficiency within a health system.
- **Provider – Data & Analytics:** Led the development of a data & analytics hub for a national provider to better manage capacity and improve efficiency.
- **Health System – Footprint & ambulatory strategy:** Developed the hospital and ambulatory footprint strategy for a multi-state health system to reflect evolving site of care patterns and consumer preferences.
- **Health System – Long range financial planning:** Developed the long-range financial plans for a national health system reflecting demographic, payer, and site of care shifts. Enabled long term capital planning.
- **Provider System – Growth and network strategy:** Developed service-line growth strategies and network optimizations.

Education

Institution	Degree	Year
MIT	Ph.D., Nuclear Engineering	2010
MIT	M.S., Nuclear Engineering	2006
MIT	B.S., Nuclear Engineering	2006

Work history

Employer	Role	Years
McKinsey & Company	Partner	2013-present
Mars & Co.	Senior Consultant	2010-2013
Schlumberger	Intern	2008

David Nuzum

Extended Leadership Expert – Medicaid / states

Key expertise and experience

- Co-founded McKinsey's State and Local Government HHS Practice
- 20 years of experience with public health, mental health, MCOs, Medicaid, safety net providers, and CBOs
- 25 years of experience with strategic planning, innovation, program development, and program management across 50+ clients
- Strategic planning, innovation, program development, and program management with HHS agencies in 15+ states



David Nuzum is a Senior Partner and leader of McKinsey's US State Government Practice and founder of State HHS Practice. David leads McKinsey's Healthcare Innovation Practice and has over 25 years of experience in health and human services, including 12 years providing strategic and implementation advisory services to 17 state governments in health and human services, including MA, CT, ME, NY, NJ, PA, DE, MO, AR, IN, OH, MI, IL, IA, GA, NC and VA. David's support to Medicaid, public health organizations, public and private payers, and healthcare providers covers strategy formation, business design, and implementation of new models for value-based payment, care delivery, care management, and other approaches to improving the quality and affordability of care. Prior to joining McKinsey in 2003, David served as Vice President at Galen Interactive Inc. and Associate at William M. Mercer Inc. David received his M.B.A. in Healthcare Management from The Wharton School of the University of Pennsylvania, and his B.A. from Harvard University.

Selected relevant experience

- **New York State Medicaid – Innovation.** Supported program design and development of technical requirements for the State's Social Care Network program as part of its amended Medicaid 1115 waiver. Supported the detailed design of New York State's AHEAD model for global hospital budgets. Supported stakeholder engagement across agency leaders on key program design choices including provider eligibility, service delivery and reimbursement, value-based care incentives, data analytics, and reporting requirements to support delivery of services for Health-Related Social Needs for over 2 million New Yorkers upon implementation.
- **Missouri and West Virginia – Rural Health Transformation.** Supported the development of 5-year strategies and federal grant application for the Rural Health Transformation Program, including changes in service lines, hospital productivity improvement, telehealth, mobile clinics, school-based care, alternative payment models, workforce development, and other innovations.
- **Pennsylvania Department of Health – Global Budget Model for Rural Hospitals.** Supported the development of a strategic plan and detailed design of an alternative payment model implemented by Medicaid, Medicare, and multiple private payers for 18 rural hospitals.
- **Massachusetts, New York State, Ohio, Tennessee (Medicaid) – Innovation.** Supported multiple state Medicaid programs in the development of new policies, strategies, and

business designs to enable the transition to alternative payment models, including support to development of State Plan Amendments and 1115 waivers.

- **Georgia Department of Community Health (Medicaid) – Managed care re-procurement.** Co-leading McKinsey's client service team in support of the Department of Community Health to develop the strategy, program design, and procurement for the State's next-generation managed care program. Overseeing claims-based analysis to compare patterns of cost and utilization among existing MCOs, between MCOs and FFS, and across different regions of Georgia. Supporting engagement across six health and human services agencies to gather input and jointly develop the priorities, RFP, and contract for the re-procurement.
- **Missouri Department of Social Services – Medicaid Transformation and Opportunity Assessment.** Assessed Medicaid programs, effectiveness, and operations to identify opportunities to improve program outcomes, quality of care, sustainability, and reduce costs for the State and recipients. This assessment included core components of the State Medicaid program such as medical and pharmacy spend, behavioral health, LTSS, third-party liability, program integrity, IT strategy, and operations.
- **New Jersey Medicaid – Public Health Emergency Unwinding.** Oversaw a team of analysts as well as technology and operations consultants in support of the State Medicaid agency, to guide eligibility redeterminations of substantially all Medicaid members, in the wake of the unwinding of the Public Health Emergency. Segmented and prioritized membership for redeterminations. Designed and implemented strategies for communicating with members regarding eligibility status and redetermination processes. Identified and helped to resolve bottlenecks in systems and processes. Analyzed variation in processes and patterns of redeterminations based on demographic factors, in order to allow for adjustments to be made to ensure equitable treatment.
- **Massachusetts, New Jersey, New York State – COVID-19 response.** Oversaw teams working to mitigate the impact of COVID-19, including analyzing trends in case rates, increasing testing capacity, enabling contact tracing, as well as vaccine rollout.
- **Massachusetts EOHHS (MassHealth) – Innovation and Program Development.** Co-led McKinsey team in defining ACO strategy, as well as ACO reporting strategy, including measures (cost, quality, and utilization) and targets. Co-led the development and implementation of production-grade analytics in support of ACO program.
- **Massachusetts EOHHS (MassHealth) – Program Management.** Conducted baseline assessment to identify and implement changes in policies and processes to support eligibility verification, prior authorizations, payment methods and fee schedules, pre-payment analytics, and post-payment analytics for payment integrity and recoveries.
- **Indiana Family and Social Services Administration (HHS) – MLTSS reform, monitoring and reporting.** Supported agency to design and implement a data surveillance plan (metrics identification, data flow mapping, dashboard design and build, analytics) for a State Medicaid agency's transition from fee-for-service to managed care for long-term care services and supports.
- **Virginia Medicaid and Human Services – Program management.** Assessed technical and operational readiness and risk for Medicaid expansion. Separately, evaluated consolidation options for Medicaid and Human Services call center operations to optimize cost and consumer experience.

Education

Institution	Degree	Year
The Wharton School of the University of Pennsylvania	M.B.A., Healthcare Management	2003
Harvard University	B.A., Biological Sciences	1996

Work history

Employer	Role	Years
McKinsey & Company	Senior Partner	2003-present
Galen Interactive Inc.	Vice President	1999-2001
William M. Mercer, Inc.	Associate	1996-1999

Kirk Fischer

Senior Advisor – Payor networks

Key expertise and experience

- Over 30 years of experience in making healthcare available to individuals, businesses, and government entities with emphasis on strategy, execution, network operations, design and negotiations
- Serves as a Senior Advisor to McKinsey, with focus on provider and network strategy, design, data and analytics, and implementation, to improve affordability, quality, and access to healthcare



Kirk has served as a network executive at multiple health insurers, including Scripps Health, Kaiser Permanente, Aetna, Cigna, Multiplan, Florida Blue and Centene. Examples of impact in provider and network management include:

- Execution: lead team that integrated health plan networks that delivered over \$150M annual savings in less than 2 years
- Strategy: built out a national value-based program across a Medicaid, Medicare, and Marketplace product offering that resulted in over 30% of all membership in downside risk relationships (approximately 7 million lives) across all markets
- Network operations: established and built two different national network organizations which resulted in consistency, efficiency, standardization, and playbook for use by local markets to not only grow the business but resulted in lower operating costs
- Design: built new provider networks for the Marketplace product (ACA) which generated over \$5B in revenue, covered 1M lives, and achieved status of the largest single state carrier
- Negotiations: negotiated a multiyear hospital agreement with a large healthcare system resulting in \$18M in savings in the first year

Education

Institution	Degree	Year
University of San Diego	M.B.A.	1985
University of San Diego	B.S. in Economics	1981

Select Work history

Employer	Role	Years
McKinsey & Company	Senior Advisor	2024-Present
Centene Corporation	SVP, Network Development and Contracting	2018-2024
Florida Blue	VP, Delivery System VP, Provider Network Operations	2012-2018
MultiPlan	SVP, Network Development	2009-2012
CIGNA Healthcare	Senior Operations Director VP, Network Operations and Analysis	2003-2008

Page West, RN

Senior Advisor – Hospitals operations

Key Qualifications

- 30+ years of relevant clinical and healthcare provider-focused experience
- Former Chief Nurse Executive of Dignity Health
- Extensive experience supporting McKinsey's clients on workforce optimization, operations, and implementation engagements



Page is a Senior Advisor to McKinsey's healthcare systems. She is an experienced healthcare administrator with 30+ years of profession and clinical expertise and the former Senior Vice President/System Chief Nurse Executive of Dignity Health. In that role, Ms. West oversaw nursing at 39 hospitals. She is an expert in workforce optimization, operations, and implementation.

Education:

Institution	Degree
University of San Francisco	MPA / MPH
Indiana University Bloomington	Bachelor's degree, Registered Nursing

Work History:

Employer	Role	Years
McKinsey & Company	Senior Advisor	2015-Present
Dignity Health	Chief Nursing Executive	2010-Present
Mercy General Hospital	Chief Nursing Executive	1987-2010

Will Taylor

Expert – Care delivery footprint

Key expertise and experience

- Expert Associate Partner with over 10 years of experience advising health systems, payers, and governments across the UK, Europe, North America, and the Middle East
- Deep expertise in hospital and health system transformation, capital planning, and care model redesign to improve affordability, access, and sustainability
- Experienced in regional and national healthcare network transformation, including multi-year capacity, infrastructure, and clinical productivity programs
- Leads advanced data and analytics-driven health planning for governments and payers, integrating demographic, utilization, and financial modeling to inform reform design
- Supported state and national health authorities on healthcare reform strategy, cost optimization, and provider financial resilience



Will Taylor is an Expert Associate Partner. He works globally with payors, providers, regulators, investors, and governments to transform healthcare systems through strategic planning, operational excellence, and data-driven innovation. Will specializes in health system transformation, clinical service strategy, and capital and network optimization – helping clients enhance affordability, capacity, and quality while ensuring long-term provider sustainability.

Since joining McKinsey in 2015, Will has led major transformation efforts for state authorities, national health systems, and regional providers. His recent work spans hospital and care model redesign, healthcare capacity planning, and digital and data strategy for both public and private sector clients. He also served as Deputy Director for the UK Government COVID-19 Taskforce, reporting to the UK Prime Minister on national healthcare and capacity response planning.

Relevant experience

- **State Authority – Health System Capital Optimization:** Led a capital deployment program for a large public health authority, managing a €25B hospital refurbishment and infrastructure budget, improving capacity utilization and regional access.
- **UK Regional Health Systems – Transformation and Care Model Redesign:** Directed eight system transformation programs redesigning clinical models and facility footprints for health economies serving 1–9 million residents, balancing cost saving, utilization, and patient access.
- **European Health Ministry – Health System Capacity Planning:** Designed a national capacity and hospital footprint model to align service provision with future population, demographic, and utilization trends, ensuring equitable distribution of clinical resources and patient access.
- **Middle East Health Agency – Capital Allocation Strategy:** Advised on a national portfolio strategy for hospital modernization and regional provider consolidation, integrating financial sustainability metrics and performance analytics to drive data-driven recommendation.

- **National Health System – Clinical Productivity and Medical Cost Reform:** Led a comprehensive cost review and productivity analysis covering 60M lives, identifying €2B+ in efficiency and care quality improvement opportunities.
- **Major European Provider – Claims and Cost Transformation:** Supported a multi-year transformation of claims management for a leading private insurer, optimizing administrative processes and applying analytics to drive medical cost reduction.
- **Public Health Systems – Data and AI Strategy:** Directed development of six data and AI strategies for public health systems and private investment funds, building advanced analytics capabilities for systemwide planning and transformation.
- **Health Technology Scale-Up – Global Strategy Leadership:** As Global Strategy Lead for a medtech company, launched and led US market, led commercialization efforts, and performed interim Chief Commercial Officer role.
- **UK Government – COVID-19 Taskforce:** Served as Deputy Director leading the healthcare response team within the Cabinet Office, coordinating provider capacity, ICU modeling, and national contingency planning across Whitehall bodies and NHS, briefing the Prime Minister weekly.

Education

Institution	Degree	Year
University of Cambridge	BA, Mathematics	2010

Work history

Employer	Role	Years
McKinsey & Company	Expert Associate Partner, Healthcare	Jun 2021 – Present
McKinsey & Company	Engagement Manager / Practice Expert, Healthcare Systems & Services	Jan 2015 – Dec 2019
UK Government Cabinet Office	Deputy Director, COVID-19 Taskforce	Oct 2020 – Apr 2021
Sight Diagnostics	Global Strategy Lead	Jan 2020 – Oct 2020
Concentra Consulting	Senior Consultant	Oct 2010 – Dec 2014

Yenli Wong

Expert - Workforce

Key expertise and experience

- Partner serving healthcare providers and academic medical centers on workforce strategy and planning, productivity, and organizational transformation
- Deep expertise in talent and leadership, spans and layers, and service-operations redesign for hospitals and ambulatory systems
- Leads transformation programs focused on workforce excellence, frontline engagement, and cost efficiency
- Brings extensive experience supporting senior executives in academic and private health systems on people strategy and performance improvement
- Core leader in McKinsey's SHaPE Healthcare and Organization practices, with a focus on workforce redesign and productivity enablement



Yenli Wong is a Partner in McKinsey's Southern California office. She works with academic medical centers, hospitals, and healthcare provider organizations to strengthen workforce performance and operational productivity. Yenli helps executives design and implement organizational transformations that improve efficiency, culture, and frontline experience. Her work combines deep expertise in healthcare operations and people strategy with a focus on sustainable change and leadership development.

Relevant experience

- **Healthcare Provider – Digital Enablement and Workforce Analytics:** Designed a data-driven workforce analytics platform to track utilization, staffing ratios, and productivity benchmarks; enabled evidence-based decision-making for workforce.
- **Academic Medical Center – Workforce and Organizational Redesign:** Led an end-to-end workforce transformation, redesigning spans and layers, role structures, and leadership models to drive performance improvement and cost efficiency.
- **Private Hospital System – Talent and Culture Transformation:** Partnered with executives to implement a comprehensive talent strategy addressing recruitment, retention, and engagement challenges; improved frontline productivity and reduced turnover to drive workforce efficiencies.
- **Health System – Service-Operations Excellence Program:** Supported the design and launch of a service-operations improvement program across hospital departments, aligning staffing models, automation opportunities, and performance metrics.
- **Ambulatory Care Network – Organizational Effectiveness Initiative:** Guided the development of a new operating model and leadership framework to support multi-site growth and optimize clinical and administrative coordination.

Education

Institution	Degree	Year
Pomona College	B.A., English	2015

Work history

Employer	Role	Years
McKinsey & Company	Partner	2015-present

Michael Rubin

Expert – Data and analytics

Key expertise and experience

- Senior leader in McKinsey's SHaPE Tech practice, focused on healthcare innovation, digital assets, and analytics solutions
- Expert in healthcare data, financing, and policy, with extensive experience supporting public health agencies and payers
- Deep knowledge of Medicare and Medicaid systems, payment reform, and episode-based care models
- Proven track record in asset leadership, solution delivery, and technology modernization for healthcare organizations
- Brings over 15 years of experience across government, provider, and payer settings, spanning product design, digital strategy, and performance improvement



Michael Rubin is a Senior Asset Leader within McKinsey's SHaPE (Social, Healthcare, & Public Entities) Tech practice, based in New York. He helps public- and private-sector healthcare organizations design, build, and scale digital and data-driven solutions that improve performance, quality, and affordability. Michael has led large-scale innovation and transformation programs with national health agencies, public payers, and technology providers, integrating healthcare policy expertise with deep technical and operational experience.

Relevant experience

- **National Department of Health – Innovation and R&D Strategy:** Led portfolio strategy and innovation design for a federal health agency to accelerate research translation and data infrastructure modernization.
- **Public Healthcare Payers – Policy and Strategy Transformation:** Supported large-scale transformation programs for state and national payers, advancing digital adoption, analytics enablement, and value-based care design.
- **Managed Care Organization – Digital Strategy:** Directed enterprise-level digital strategy for a managed care organization, defining technology architecture, analytics priorities, and digital-member engagement models.
- **Technology and Health Services Firm - Business Building and Scaling:** Partnered with leadership to build and scale new healthcare technology assets, integrating data analytics, AI solutions, and product delivery models to expand market reach.
- **Healthcare System – Episodes of Care and Bundled Payments Design:** Developed and operationalized new episode-based payment models and performance dashboards to improve care quality and financial accountability and sustainability.

Education

Institution	Degree	Year
New York University	M.P.A., Health Policy	2011
Columbia University	B.A., Psychology	2004

Work history

Employer	Role	Years
McKinsey & Company	Senior Asset Leader	2024-present
	Director of Solution Delivery	2022-2023
	Solution Leader and Associate Partner	2018-2022
	Solution and Delivery Roles	2014-2017
U.S. Department of health and Human Services, Office of Inspector General	Program Analyst	2010-2014
NYU Pain Management Center	Project Coordinator	2009–2010
Center for Medicare Advocacy	Analyst and Project Coordinator	2006–2008
New York State Psychiatric Institute	Research Assistant	2004–2006

Jennifer Rost Expert – Rural Healthcare, value-based care

Key expertise and experience

- Partner with 14+ years experience driving care innovation and healthcare value with public and private sector clients
- Led McKinsey support for State of Pennsylvania Rural Health Model
- Extensive expertise working with state governments, health systems, integrated payer/providers, Medicaid MCOs, and virtual health providers
- Co-leads McKinsey's value-based care & delivery innovation service line



Jennifer Rost is a Partner in McKinsey's Washington, D.C. office. She is a leader in the Firm's healthcare practice and focuses on serving public and private sector healthcare clients to develop and scale sustainable models of care delivery and payment innovation.

She has led work in multiple states to design and implement statewide, multi-stakeholder payment reform and delivery system transformation initiatives, including Pennsylvania's Rural Health Model. Through her work, Jennifer has developed a deep understanding of the regulatory and policy impacts of healthcare and social service reform. She has experience working with rural financing systems as well as federally qualified health center financing systems.

Jennifer also serves regional health systems and Medicaid MCOs on strategies, operational transformations, and technology-enablement to improve access to care, including in their rural populations (e.g., through expansion of virtual health and community supports).

Relevant experience

- **Pennsylvania – Rural Health Initiative:** Led a team supporting the Commonwealth to design its Rural Health Model, a multi-payer global budget model for rural hospitals. Supported the Commonwealth from the initial conceptual design through CMS model approval, and from high-level design to detailed technical specifications and pre-implementation support.
- **Arkansas and Ohio – Statewide Payment Reform Initiatives:** Led teams responsible for coordinating activities of a statewide payment reform initiative. Managed overall strategy and governance, stakeholder coordination, legislative processes. Supported transition of initiative focus from design to implementation, including establishment of operational structure to track, manage, and refine new payment models and development of early performance metrics.
- **Regional integrated health system – Transformation:** Led strategy development and transformation implementation with multiple regional integrated health systems, including their rural care delivery footprint and statewide Medicaid health plans. These programs focused on improving care access, affordability, quality, and health outcomes, while driving sustainable health system operational and financial performance, via initiatives within each business unit and by enhancing collaboration and resource deployment across the care delivery and health plan entities.
- **Health plans – Care innovation:** Led multiple strategy and capability-building programs with national and regional health plans to drive care delivery innovation and transformation (e.g., specialty care models, social needs integration, virtual and omni-channel care models, provider partnerships).

- **Healthcare innovators:** Led work with healthcare disruptors – including, virtual health providers, value-based care primary care providers, data and analytics services companies – including data-driven strategy development, new business model growth and scaling.

Education

Institution	Degree	Year
Yale University	M.B.A.	2011
Yale University	B.S., Biomedical Engineering	2006

Work History

Employer	Role	Years
McKinsey & Company	Partner	2011-present; 2006-2009

Aarthi Sridhar

Expert – Hospital finance

Key expertise and experience

- Advisor to public and private healthcare providers on financial, operational transformations and strategic planning
- Specialist in driving holistic cost and growth improvement for long-term financial sustainability
- Thoughtful counselor on driving change during transformation journeys especially around accountability and performance management
- Deep experience across public and private hospitals, integrated delivery networks and academic medical centers
- Inclusion Advisor within the Firm, advancing diversity and leadership initiatives



Aarthi Sridhar is an Associate Partner in McKinsey's Bay Area office, specializing in health system transformations, with a focus on financial sustainability and growth. Drawing on her technical foundation in engineering and analytics, she brings a rigorous, systems-based approach to solving complex healthcare challenges. She has led large-scale transformation programs that deliver measurable impact – spanning cost optimization, growth acceleration and institutionalizing financial and operating infrastructure to improve rigor and sustainability. She believes deeply in partnership and capability building, working alongside leadership teams to strengthen internal processes, develop lasting capabilities, and embed a culture of continuous improvement.

Relevant experience

- **Private Integrated Delivery Network – Holistic Transformation:** Leading a multi-year transformation program across provider and payor businesses, with focus on optimizing coordinated care delivery and improving access. Institutionalizing financial rigor through mini value identification “sprints” and improved forecasting / budgeting practices. Launched regional growth and physician group efforts to increase patient access and network integrity.
- **Private Hospital System – Holistic Transformation:** Led a multi-year transformation program to improving care-delivery efficiency and cost performance cross system. Deep efforts to revamp finance function include monthly operating reviews, financial dashboards, budgeting / capital process. Focused capability building to set up internal transformation office to lead efforts and sustain future continuous improvement model.
- **Private Hospital System – Corporate and Business-Function Redesign:** Supported executive teams in re-architecting administrative and corporate functions to drive operational excellence and cost optimization across business units.
- **Academic Medical Center – Cost and Growth turnaround:** Rapidly identified opportunities to improve financial and operational performance (e.g., clinical operations, productivity, workforce optimization, patient engagement, provider efficiency) for several under-performing acute assets.
- **Children's Hospital – Growth Strategy:** Developed a comprehensive market-growth strategy to expand pediatric specialty services (e.g., new programs, ambulatory footprint, access and network) and optimize sources and uses of cash to fund long-term growth

- **Public Healthcare Provider – Service-Operations Transformation:** Led a multi-year transformation program focused on optimizing external spend, e.g., supply chain, purchased services and develop a future corporate office and ambulatory footprint to optimize real estate cost and support financial sustainability

Education

Institution	Degree	Year
Duke University	Ph.D., Structural Engineering	2019
Harvey Mudd College	B.S.E., General Engineering	2015

Work history

Employer	Role	Years
McKinsey & Company	Associate Partner	2019-present
Agency for Science, Technology and Research, Singapore	Research Intern	2012

Amit Shah, MD

Expert - Healthcare agency strategy

Key qualifications

- Leader in multiple efforts with state agencies (e.g., Department of Mental Health, Health and Human Services) on healthcare strategy
- Public and private sector client service across the value chain, including successfully supporting stakeholder engagement across state agencies and prep for regular report out to leadership
- Leader in McKinsey NextGen Care Management domain and Healthcare analytics, including applications for vulnerable populations (e.g., behavioral health, LTSS)
- Analytics and Technology Translator, driving sustainable value by convening and building alignment across business, analytics, and technology stakeholders



Amit Shah is a partner in McKinsey's New York office and a leader in McKinsey's North American Healthcare Systems and Services Practice, serving national and regional payors (both public and private). He co-leads the Firm's client efforts and knowledge development on the topics of Population Health Management, payment reform, and Next Generation Care Management/Care Delivery. He has also helped build, grow, and now co-leads McKinsey Healthcare Analytics.

Relevant experience

- **State Department of Mental Health – Technology and Analytics Roadmap:** Supported Mental Health Agency in defining technology, analytics, and data management roadmap to enable business process redesign with end goal to drive value across relevant stakeholders (e.g., individual receiving care, state employees, providers).
- **Supporting Entity with Public Sector Mission & State Medicaid Agency – Designing and Implementing Data, Analytics, and Reporting:** Collaborated with Medicaid leaders and an organization with inspiring public service mission to design and implement big data analytics and reporting to help physicians improve performance, quality, and efficiency.
- **State Medicaid Agency & Supporting Organization – Analytics/Technology Build-Operate-Transfer:** Collaborated with Medicaid Agency and organization deeply ingrained in operations to build analytics/technology to enable payment reform efforts. We subsequently operated the solution to accelerate implementation and then transferred these capabilities to the Agency and supporting organization.
- **Private Insurer – Optimizing Medicaid Healthcare Utilization Management:** Convened joint client and McKinsey team to identify, implement, and capture \$100M in run rate savings over 3-years based on analysis of medical claims for cost and utilization trends.
- **Private Insurer – Medicaid Medical Claims Auditing and Payment Integrity:** Led effort to assess payment accuracy and identify pre- and post-pay levers to reduce overpayment and/or inappropriate payment.
- **Private Medicaid Insurer – Care Management Redesign:** Led joint client-McKinsey team to redesign Care Management approach, including care management systems, analytics, digitization of care management processes, and training.
- **Multiple States – State Plan Amendments and Waiver Applications:** Supported multiple State Medicaid programs in the development of new policies, strategies, and business designs to enable the transition to alternative payment models, including support to development of successful State Plan Amendments and 1115 waivers.

Education

Institution	Degree	Year
Perelman School of Medicine at the University of Pennsylvania	M.D.	2008
The Wharton School of the University of Pennsylvania	M.B.A, Healthcare Management	2008
Harvard University	A.B., Economics	2003

Work history

Employer	Role	Years
McKinsey & Company	Partner	2008-present

Andrew Odden, MD

Expert – Providers and hospitals

Key expertise and experience

- Associate Partner based out of McKinsey's St. Louis Office
- Expertise in health system, hospital, and Medical Group operations
- Nearly twenty years of healthcare experience, including a decade as a physician leader at Academic Medical Centers
- Experience with AMCs, non-profit health systems, and specialty care
- Prior to McKinsey, served as Vice Chair of Medicine at a major AMC and practiced as an internal medicine hospitalist



Relevant experience

- **National health system – Clinical operations:** Redesigned the preoperative process for a large national health system across scheduling, pre-admission testing, and financial clearance.
- **Regional health system – Care delivery footprint:** Led a multi-modal care delivery geospatial analysis to determine the impact of health care asset distribution on access to care for a regional, geographically diverse health system.
- **Safety net health system – Clinical operations portfolio:** Oversaw the redesign of the end-to-end clinical operations portfolio for a large, safety net health system, including length of stay reduction, provider productivity, OR utilization, ED throughput, and continuity of care, unlocking over \$180m in value across the system.
- **Academic medical center – Workforce transformation:** Led a clinical operations and workforce productivity transformation for a large academic health system focused on inpatient length of stay, OR throughput, and physician engagement, realizing over \$200M in annual value.
- **Academic medical center – Organizational design:** Designed a clean sheet organizational structure for a large academic health system's quality, safety, provider credentialing, and physician advisor functions
- **Regional health system – M&A:** Advised on approaches, advantages and disadvantages for a regional health system to partner or merge with its academic affiliate
- **Regional health system – Operating model:** Led the redesign of the corporate business function operating model for a multi-state regional health system
- **Health system – Operations:** Redesigned the physician advisor peer to peer process using automation and advanced analytics for a large healthcare client, unlocking over \$40m in net new value

Education

Institution	Degree	Year
Washington University - Olin Graduate School of Business	Executive MBA	2021
University of Minnesota	M.D.	2007
Gustavus Adolphus College	B.A., Biology	2003

Work history

Employer	Role	Years
McKinsey & Company	Engagement Manager	2021-present
Washington University in St. Louis	Vice Chair, Department of Medicine	2015-2021
Ann Arbor Veterans Affairs Healthcare System	Chief Hospital Medicine and Associate Chief of Medicine	2010-2015
University of Michigan Medical School	Physician	2010-2015

Yuqi Shi

Expert – Network capabilities and tools

Key expertise and experience

- Over 8 years of experience in healthcare consulting with a focus on improving cost efficiency, quality of care, and innovation in care delivery models
- Deep expertise in medical cost management, value-based payment design, and healthcare data analytics
- Experienced leader of cross-functional teams of analysts, data engineers, and data scientists in building and deploying complex claims and non-claims analytics
- Strong communicator and problem-solver known for translating complex insights into actionable strategies for public and private healthcare organizations



Yuqi is a Solution Manager in McKinsey & Company's New York office. Yuqi served public and private payors, providers, state agencies, and healthcare investors on a variety of topics including network and contracting, innovative payment model, population health management, medical cost management, operational excellence, care management programs, and healthcare analytics capability building.

Yuqi specializes in developing data-driven solutions to support clients in improving health access and equity, quality and outcomes.

Relevant experience

- **Regional payers – Network design and optimization:** Design of high-performance networks and narrow networks to optimize network efficiency, quality, and access across multiple scenarios, markets, and lines of businesses.
- **Regional payer – Home health strategy design:** Initial diagnostics and bottom-up planning based on claims analytics, identified potential savings opportunities of 2-3% total cost of care via expanding home health programs (e.g., hospital at home).
- **National payer – Product strategy design:** Sizing of total addressable and serviceable market based on integrated third-party national claims analytics (incl. CMS Medicare FFS dataset, Merative Marketscan dataset) and pinning down client's competitive advantage over competitors in launching new utilization management and care management products.
- **National dental carrier – Network strategy:** Development of network strategy, actionable initiatives, and dynamic negotiation support models, identified potential savings opportunities of 4-5% dental costs via expanding lease partnership and optimizing discount rates.
- **Regional payer – Performance evaluation and growth strategy design for serving special-needs population (SNP):** ML-based member clustering and claims-based analytics to identify member archetypes with stronger performance to drive data-driven strategy.
- **Suite of analytical products:** Designed products to size opportunities in expanding innovative care delivery models (incl. care at home and virtual health) and site-of-care optimization, which have been deployed at 20+ clients and resulted in potential savings of 2-6% in total cost of care in various client use cases.

- **State Medicaid agency – Medical cost management:** Developed comprehensive top-down medical cost analytics and cross-region benchmarking, identified potential savings opportunities of 3-5% total cost of care across regions within the state.
- **Healthcare service platform – Pricing strategy design:** Development of a dynamic cost-plus pricing models using ML/advanced analytics, improved the accuracy of cost base estimates by 2x.
- **Private equity healthcare investor – Portfolio strategy design:** Market sizing and prediction of future growth for post-acute SNF and Home Health market under multiple potential scenarios based on claims analytics and expert interviews.

Education

Institution	Degree	Year
Columbia University	Master of Public Health (MPH), Epidemiology	2016
Peking University	Bachelor of Science (BS), Biology/Biological Sciences	2014

Work history

Employer	Role	Years
McKinsey & Company	Solution Manager	2016 - present

Ankit Jain

Expert – Data, claims forecasting

Key expertise and experience

- Healthcare analytics leader with more than 14 years of experience in data strategy, financial modeling, and digital transformation across payers and providers
- Deep expertise in claims analytics, cost reduction, and growth strategy for insurers, health systems, and private equity clients
- Proven track record of deploying complex data-driven solutions to support M&A, network optimization, and clinical service modeling
- Combines actuarial rigor with strategic insight to help clients enhance financial performance and operational excellence
- Experienced at translating complex analytics into actionable strategies, balancing innovation with sustainability



Ankit is an Associate Partner with more than 14 years of experience in healthcare analytics and strategy. He specializes in building data-driven solutions to support cost reduction, network strategy, and growth planning across insurers, health systems, and private equity firms. Ankit has led cross-functional teams deploying complex claims analytics to guide M&A, go-to-market strategies, and clinical service modeling. His work balances financial performance and innovation, emphasizing operational excellence and strategic decision-making. With deep actuarial expertise and extensive experience advising top U.S. healthcare firms, Ankit brings strong analytical leadership to complex transformation efforts.

Relevant experience

- **Health insurer – Claims analytics and cost transformation:** Designed and implemented predictive cost models to identify utilization inefficiencies, enabling a 7% reduction in medical loss ratio while maintaining network adequacy and access.
- **Private equity – Healthcare acquisition diligence:** Led data-driven due diligence for multiple platform investments, modeling reimbursement, case-mix, and volume dynamics to evaluate deal value and post-acquisition synergies.
- **National provider network – Performance optimization:** Built and deployed network dashboards combining claims, provider, and financial data to track access, utilization, and profitability across multi-state operations.
- **Fertility clinic – Market entry/geolocation:** Built market entry model using machine learning and geospatial technique for fertility clinic with limited presence in the US market to grow the footprint organically and inorganically.
- **Integrated delivery system – Site-of-care shift analysis:** Modeled outpatient migration trends and developed actionable insights for service-line reconfiguration, improving system-wide operating margins by over \$40M.
- **Healthcare technology firm – Market-entry strategy:** Developed growth analytics to size addressable markets, forecast adoption rates, and guide partnership strategy for a new digital health solution.

Education

Institution	Degree	Year
Shaheed Bhagat Singh College	B.Com. (H)	2020

Work history

Employer	Role	Years
McKinsey & Company	Associate Partner / Senior Capabilities & Insights Analytics Expert	2012-present

Brian Latko

Expert - Medicaid / states

Key qualifications

- 17+ years of experience in Medicaid, public health and human services topics
- Has supported 15+ states in designing Medicaid managed care strategies, leading procurements, strengthening MCO contracts and building performance management capabilities
- Extensive experience embedding innovations that address behavioral health and youth in care priorities through Medicaid financing, program design, performance, technology and operational improvements



Brian Latko is a Partner in McKinsey's Washington, DC office and leads McKinsey's Medicaid Domain. He leads McKinsey's service to states on Medicaid Program Performance topics including managed care strategy, procurement and oversight nationally.

Relevant experience

- **New York – Managed Care Performance Management Transformation:** Supporting comprehensive capability building program focused on a state Medicaid program's MCO Performance Management function, conducting detailed process mapping and baselining, streamlining contract management, MCO accountability and enforcement processes, building new data and analytics products to create transparency into MCO performance, and improving collaboration across ~250 staff covering 60 MCO contracts.
- **Georgia – Managed Care Organization Strategy, Reprocurement and Performance Management:** Supported the State's reprocurement of its Medicaid managed care program, including the program for foster care and justice-involved youth, leading research and analytics into population health and service delivery improvement and drafting inputs for procurement and contract materials based on national and cross-industry best practices. Redesigned MCO Performance Management capability including organizational design, talent, data, analytics and IT platforms, and execution requirements.
- **California – Managed Care Plan Oversight Roadmap Development:** Conducted an agency-wide review of managed care plan oversight activities to develop a roadmap for modernizing processes following California's statewide procurement, comprehensively analyzing MCO performance data and state processes to develop a portfolio of short and long-term opportunities for capability building.
- **Indiana – Data transformation for Medicaid Managed Long-Term Services and Supports (MLTSS) implementation:** Supported the State's transition to managed LTSS by developing a comprehensive plan for tracking and reporting program performance. Defined the programmatic vision and data strategy, created the cloud foundation and developed analytics capabilities and reporting dashboards, scaled master data management, and managed the overall transformation.

- **Illinois – Medicaid Managed Care Transformation:** Supported the strategic realignment of a state Medicaid agency from a FFS organization to a managed care organization. Developed the procurement materials and processes for a \$60B managed care procurement estimated to create \$200-300M in efficiencies in the first year. Created implementation plans for MCO enrollment and auto-assignment for 2M+ beneficiaries.

Education

Institution	Degree	Year
Harvard University	MPA	2014
Stanford University	MBA	2014

Work history

Employer	Role	Years
McKinsey & Company	Partner	2006-2008; 2015-present
Results for Development Institute	Program Officer	2009-2011

Appendix B: Certificate of Compliance Form

McKinsey's signed certificate of compliance is included on the following pages.

CERTIFICATE OF COMPLIANCE

For your bid to be considered valid, this form must be completed in its entirety, executed by a duly authorized representative of the bidder, and submitted as part of the response to the proposal.

- A. **NON COLLUSION:** Bidder hereby certifies that the prices quoted have been arrived at without collusion and that no prior information concerning these prices has been received from or given to a competitive company. If there is sufficient evidence to warrant investigation of the bid/contract process by the Office of the Attorney General, bidder understands that this paragraph might be used as a basis for litigation.
- B. **CONTRACT TERMS:** Bidder hereby acknowledges that is has read, understands and agrees to the terms of this RFP, including Attachment C: Standard State Contract Provisions, and any other contract attachments included with this RFP.
- C. **WORKER CLASSIFICATION COMPLIANCE REQUIREMENT:** In accordance with Section 32 of The Vermont Recovery and Reinvestment Act of 2009 (Act No. 54), the following provisions and requirements apply to Bidder when the amount of its bid exceeds \$250,000.00.

Self-Reporting. Bidder hereby self-reports the following information relating to past violations, convictions, suspensions, and any other information related to past performance relative to coding and classification of workers, that occurred in the previous 12 months.

Summary of Detailed Information	Date of Notification	Outcome

Subcontractor Reporting. Bidder hereby acknowledges and agrees that if it is a successful bidder, prior to execution of any contract resulting from this RFP, Bidder will provide to the State a list of all proposed subcontractors and subcontractors' subcontractors, together with the identity of those subcontractors' workers compensation insurance providers, and additional required or requested information, as applicable, in accordance with Section 32 of

The Vermont Recovery and Reinvestment Act of 2009 (Act No. 54), and Bidder will provide any

update of such list to the State as additional subcontractors are hired. Bidder further acknowledges and agrees that the failure to submit subcontractor reporting in accordance with Section 32 of The Vermont Recovery and Reinvestment Act of 2009 (Act No. 54) will constitute non-compliance and may result in cancellation of contract and/or restriction from bidding on future state contracts.

D. Executive Order 05 – 16: Climate Change Considerations in State Procurements Certification

Bidder certifies to the following (Bidder may attach any desired explanation or substantiation. Please also note that Bidder may be asked to provide documentation for any applicable claims):

1. Bidder owns, leases or utilizes, for business purposes, space that has received:

- ☐ Energy Star® Certification
- ☒ LEED®, Green Globes®, or Living Buildings ChallengeSM Certification
- ☐ Other internationally recognized building certification:

-
2. Bidder has received incentives or rebates from an Energy Efficiency Utility or Energy Efficiency Program in the last five years for energy efficient improvements made at bidder's place of business. Please explain:

No, we haven't received any incentives or rebates from an Energy Efficiency Utility or Energy Efficiency Program in the last five years

3. Please Check all that apply:

- ☒ Bidder can claim on-site renewable power or anaerobic-digester power ("cow-power"). Or bidder consumes renewable electricity through voluntary purchase or offset, provided no such claimed power can be double-claimed by another party.
- ☐ Bidder uses renewable biomass or bio-fuel for the purposes of thermal (heat) energy at its place of business.
- ☒ Bidder's heating system has modern, high-efficiency units (boilers, furnaces, stoves, etc.), having reduced emissions of particulate matter and other air pollutants.
- ☒ Bidder tracks its energy consumption and harmful greenhouse gas emissions. What tool is used to do this? Internal EMS System
- ☒ Bidder promotes the use of plug-in electric vehicles by providing electric vehicle charging, electric fleet vehicles, preferred parking, designated parking, purchase or lease incentives, etc..
- ☐ Bidder offers employees an option for a fossil fuel divestment retirement account.
- ☒ Bidder offers products or services that reduce waste, conserve water, or promote energy efficiency and conservation. Please explain:

Through McKinsey Sustainability, the firm helps organizations move from planning to achieving real results on their sustainability journeys. This includes helping companies reduce emissions, save costs, and create value while meeting the world's demand for food, energy, and materials.

4. Please list any additional practices that promote clean energy and take action to address climate change:

Reducing Waste and Water Usage: McKinsey's offices implement initiatives to reduce waste and water usage. These include sorting and recycling, donating decommissioned technology equipment, reducing single-use plastics, and minimizing food waste.

Energy Efficiency and Conservation: McKinsey has been sourcing 100% renewable electricity since 2023 and has implemented high environmental standards such as LEED Gold or Platinum for new offices and major renovations. Additionally, McKinsey is working to reduce emissions from its car fleet by increasing the share of electric/hybrid cars.

Sustainable Procurement: McKinsey's procurement practices include sourcing goods and services that minimize the use of natural resources and are sustainably produced. The firm is committed to working with suppliers to improve the sustainability of their practice

E. Executive Order 02 – 22: Solidarity with the Ukrainian People

- ☒ By checking this box, Bidder certifies that none of the goods, products, or materials offered in response to this solicitation are Russian-sourced goods or produced by Russian entities. If Bidder is unable to check the box, it shall indicate in the table below which of the applicable offerings are Russian-sourced goods and/or which are produced by Russian entities. An additional column is provided for any note or comment that you may have.

Provided Equipment or Product	Note or Comment

F. Certification Regarding Use of Contract Funds for Lobbying - Applicable to contracts over \$100,000.00 when federal monies are involved - this clause must be included in all subcontracts over \$100,000.00.

1. The prospective contractor certifies, to the best of his or her knowledge and belief, under the penalties of perjury under the laws of the State of Vermont and the United States that on behalf of the person, firm, association, or corporation he or she represents, that:

a. No Federal appropriated funds have been paid or will be paid, by or on behalf of the undersigned, to any person for influencing or attempting to influence an officer or employee of any Federal agency, a Member of Congress, an officer or employee of Congress, or an employee of a Member of Congress in connection with the awarding of any Federal contract, the making of any Federal grant, the making of any Federal loan, the entering into of any cooperative agreement, and the extension, continuation, renewal, amendment, or modification of any Federal contract, grant, loan, or cooperative agreement.

b. If any funds other than Federal appropriated funds have been paid or will be paid to any person for influencing or attempting to influence an officer or employee of any Federal agency, a Member of Congress, an officer or employee of Congress, or an employee of a Member of Congress in connection with this Federal contract, grant, loan, or cooperative

agreement, the undersigned shall complete and submit Standard Form-LLL, "Disclosure Form to Report Lobbying," in accordance with its instructions.

2. This certification is a material representation of fact upon which reliance was placed when this transaction was made or entered into. Submission of this certification is a prerequisite for making or entering into this transaction imposed by 31 U.S.C. 1352. Any person who fails to file the required certification shall be subject to a civil penalty of not less than \$10,000 and not more than \$100,000 for each such failure.

3. The prospective contractor also agrees that they shall require that the language of this certification be included in all lower tier subcontracts, which exceed \$100,000 and that all such recipients shall certify and disclose accordingly.

RFP/PROJECT:

DATE:
Page 4 of 4

Bidder Name: McKinsey and Company, Inc. Washington D.C. Contact Name: David Nuzum

Address: 1200 19th St NW Suite 1000 Fax Number: n/a

Washington, DC 20036 Telephone: 973.549.6309.

E-Mail: David_Nuzum@mckinsey.com

By:  Name: David Nuzum
Signature of Bidder (or Representative) (Type or Print)

END OF CERTIFICATE OF COMPLIANCE